



IT'S ~~ALL~~ ^{Not} YOUR FAULT

— ♦ —
Helping Caregivers with Aging Loved Ones

FILL-IN WORKBOOK · COLOR-CODED

— ♦ —
ROB BRIZZI

For Hope Brizzi, PharmD

*Author of **The Cardinal's Promise***

CARDINAL'S PROMISE SERIES

It's Not Your Fault

Helping Caregivers with Aging Loved Ones

*Caregivers aren't taught how to manage the care of their aging loved ones.
This will be your guide. I created this based on working in healthcare for over
20 years.*

Rob Brizzi, Certified Dementia Practitioner

Free worksheets, state guides & more at thecardinalspromise.com

“Bear ye one another’s burdens.”

— GALATIANS 6:2

How to Read the Colors

This guide is color-coded so families can find the right level of urgency quickly.

RED — Act Today

Crisis, urgent safety, hospital discharge, hospice, last-days decisions, or caregiver collapse.

PURPLE — Watch Closely

Something has changed. Get support soon, ask better questions, and do not ignore the pattern.

BLUE — Plan Ahead

Documents, money, benefits, medication lists, and conversations best handled before a crisis.

IN A CRISIS RIGHT NOW?

Do not read this book. Turn to page **13**, find the line that sounds like your family, and go to the chapter it names. Every phone number you might need is on p. **258**.

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FROM THE AUTHOR

Why I Wrote This Book

My father, Lou Brizzi, died on Christmas Day, 2025. Like most sons, I figured there'd be more time. I figured I'd be at his bedside the way I had been so many times before. I wasn't.

Two weeks later, on January 9, 2026, I was working — because that's what Lou taught me to do. Suit and tie, the same way he wore one for more than forty years. I was sitting in a facility parking lot when a cardinal landed on my driver's-side mirror. It was my seventeenth anniversary of being sober. Lou had pulled me out of a life that nearly ended in a West Palm Beach apartment, and there he was again. Still showing up.

I started journaling to get through the grief, and through the guilt of not being there at the very end. I wrote so much that my wife, Hope, finally told me to turn it into a book.

That book became *The Cardinal's Promise*. It's with an editor now. While I was writing it, I kept circling back to all the decisions that get dropped on families when someone they love starts to decline — and how almost nobody hands you a map. So I wrote one. *The Cardinal's Promise* is the raw version of my story, of being adopted by Lou and finding my way back to a life worth keeping. This is the guide that goes with it. For a lot of families, just knowing where to look is the whole difference between getting the help your parent needs and missing it completely.

This isn't a feel-good memoir. It's a working guide — built to knock down the myths and help you ask sharper questions when it counts.

The Cardinal's Promise is my story. This guide is yours.

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The author is the founder of The Cardinal's Promise and of Cardinal Care Bridge, a care-navigation service referenced in this book. Readers should compare it against independent options and ask any advisor how they are paid.

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The personal experiences and reflections in this book are the author's own.

EMERGENCY

If someone is in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This guide helps you plan. It does not replace emergency care.

Dedication

For **Hope Brizzi, PharmD** — my wife, who took a chance on me.

Thank you for your love, your wisdom, and your unwavering support — and for telling me to turn my journals into this book. I love you.

In honor of **Lou Brizzi** — my father, who inspired it. Thank you for choosing me, for believing in me, and for showing me what love looks like.

How to Use This Guide

This book is built to be useful on your worst day, not read straight through.

If you are caring for a husband, wife, partner, sibling, or friend — this book is yours too. It says "parent" because that is the road I walked; wherever it does, read "the person you love."

Start wherever the pressure is. If you're in a crisis right now, go straight to Start Here — the triage pages right after the Contents — and to whichever chapter matches what you're facing. If you've got a little more breathing room, read Part 1 first to get your bearings, then work into the care-options, financial, documents, and hospice sections as you need them.

Every chapter follows the same pattern, on purpose: a plain-truth explanation, what families usually think, the teaching, and then:

- **What to Do This Week** — concrete steps you can work through and check off.
- **The Tools** — the worksheets, checklists, and fill-in fields for that chapter.
- **Rob's Note and the Chapter Takeaway** — the heart of it, and the single next question to carry forward.

Write in it. The worksheets in each chapter belong to you. Copy them, fill them in, take them to appointments, hand them to a sibling. A guide with notes scribbled in the margins is a guide doing its job. Need a clean copy? Every worksheet in this book can be downloaded and printed for free at **thecardinalspromise.com** — make a fresh set for each parent, each appointment, each sibling.

It gets specific where that matters. Medicaid, aging services, and a lot of benefits are run at the state and county level, which is exactly where generic national advice leaves families stranded. When a state program, office, or rule applies, this book points you to the right door to knock on — your state Medicaid office, your State Health Insurance Assistance Program (SHIP), your Area Agency on Aging, and your county Department of Social Services. **Your State Guide** at the back of this book shows you how to find your state's version of every one of them.

And it doesn't replace the professionals. What it does is help you ask better questions and show up prepared. The real decisions about money, law, health, and benefits still belong with qualified experts in your state. Use this to walk into those rooms already knowing what to ask.

What Are You Facing?

You do not have to read this book from the beginning before you use it. If your family is calm and planning ahead, start at Part 1 and move through the guide in order. If something is happening right now, this page is the map: find the situation closest to yours, go there, and come back for the rest later. And if your situation is bigger than one chapter — a stroke, a diagnosis, a parent who cannot manage alone — the **Find Your Situation** pathways right after these pages sequence the first moves for you.

If This Is Happening, Start Here

IF THIS IS HAPPENING	GO TO
Mom fell, seems weaker, or is suddenly less steady	Chapter 2: Falls, Weakness, and Decline · p. 37
Dad is forgetting medications, bills, appointments, or directions	Chapter 3: Memory Loss and Dementia · p. 44
Your parent is repeating questions or acting confused in new ways	Chapter 3: Memory Loss and Dementia · p. 44
The caregiver is exhausted, angry, resentful, or overwhelmed	Chapter 4: Caregiver Burnout · p. 50
One person is doing everything and the family is not helping	Chapter 4 and Chapter 5: The Family Meeting · p. 50
The family is arguing, avoiding decisions, or blaming each other	Chapter 5: The Family Meeting · p. 58
Your parent is in the hospital right now	Ask today: “Is she admitted as an inpatient, or under observation?” — Chapter 13: Medicare Basics · p. 107, then Chapter 10 · p. 91
Your parent is coming home from the hospital and you feel unprepared	Chapter 6: Home Health — or Chapter 10 if a rehab facility comes first · p. 67
Your parent needs help bathing, dressing, cooking, or staying safe	Chapter 7: Private Duty Care · p. 73
Living alone does not seem safe anymore	Chapter 8: Assisted Living · p. 80

Doctors are talking about comfort, symptoms, or quality of life	Chapter 11: Palliative Care · p. 96
You are wondering if it may be time for hospice	Chapter 12: Hospice · p. 101
Death may be getting close	Part 5: Hospice and the Last Days · p. 181
You are wondering whether Medicare pays for assisted living	Chapter 8: Assisted Living and Chapter 13: Medicare Basics · p. 80
You are trying to understand home health versus hospice	Chapter 6: Home Health and Chapter 12: Hospice · p. 67
You need a sitter or daily help and are not sure that is "home health"	Chapter 7: Private Duty Care (Chapter 6 explains the difference) · p. 73
Prescriptions are getting too expensive	Chapter 17: Prescription Assistance · p. 129
You are confused about Medicare, Medicaid, insurance, or who pays	Part 3: Paying for Care · p. 107
You do not know where the legal, medical, or financial papers are	Part 4: The Documents That Matter · p. 140

Nothing here sounds like your family? **Find Your Worry** on p. 202 lists worries in plain words, each with a page.

EMERGENCY

If you are in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This guide can help you organize decisions. It does not replace emergency care.

If You Are Not Sure Where to Start

Start with these three questions.

1. Is anyone unsafe today? Unsafe can mean falling, wandering, leaving the stove on, missing critical medication, driving dangerously, or being alone when they cannot manage safely. If yes, focus on safety first — see Chapters 2, 3, and 4.

2. Has something changed recently? A change may be physical, mental, emotional, financial, or behavioral: more falls, new confusion, weight loss, missed medications,

unpaid bills, less bathing, more sleeping, new agitation, caregiver exhaustion. If yes, write down what changed — see Chapter 1.

3. Who needs to be involved? Most caregiving crises get worse when one person carries everything alone. Ask who is the main caregiver, who has medical and financial information, who can attend appointments, and who needs to be part of decisions — see Chapter 5.

In the First 24 Hours — Do These

SEVEN ACTIONS, NOT SEVEN CHAPTERS

1. **Handle immediate safety** — the person, the stove, the car keys, the stairs.
2. **Photograph the insurance and Medicare cards**, front and back.
3. **Gather the medication list** — or the bottles, in one bag (Chapter 25).
4. **If a hospital is involved, ask:** "Is my parent admitted as an inpatient, or under observation?" Write down the answer and who gave it — Chapter 13 explains why this one question matters so much.
5. **Find out who is legally authorized to make decisions**, and where the power of attorney papers are (Chapters 19–20).
6. **Start a notes page** — what changed, when it started, who told you what.
7. **Then go to the chapter this page pointed you to.** That is your next right step.

In the next 24–72 hours: read the chapter that matches your situation, hold a short family meeting (Chapter 5), check the caregiver's own limits honestly (Chapter 4), and start the Important Documents Folder (Chapter 19) and the Emergency Contacts and Crisis Plan (Chapter 26).

Finding immediate help: your Area Agency on Aging, your county or state social services office, your state Medicaid office, and your State Health Insurance Assistance Program (SHIP) for Medicare counseling. Your State Guide at the back of this book shows how to find each one, and Numbers That Matter — p. 258 — puts every phone number in one place.

THE CARDINAL RULE

When everything feels urgent, do not try to solve the whole future. Write down: What changed? What is unsafe today? Who needs to know? What is the next call? The next appointment? The next document to find? That is enough for now.

One more thing. You don't have to solve all of this today. Find the most urgent issue, do one thing about it, and then take the next right step. That's it. That's the whole method.

Choose Your Path

Answer each question and follow the arrow — like a story where you pick the page. Start at 1. You will land on your pathway or chapter within a minute.

1 • IS ANYONE IN IMMEDIATE DANGER RIGHT NOW?

Yes → Call **911** now. This book will be here when you get back.

No → Go to 2.

2 • IS YOUR PARENT IN A HOSPITAL RIGHT NOW?

Yes → Ask today: "Inpatient, or under observation?" — then go to **Pathway 1** (next pages) and Chapters 10 and 13.

No → Go to 3.

3 • HAS A DOCTOR MENTIONED HOSPICE, COMFORT CARE, OR THAT TIME MAY BE SHORT?

Yes → **Chapters 11–12** (Palliative Care and Hospice) and **Part 5** (The Last Days). The Hospice chapter tells you what to ask first — and earlier is almost always better than later.

No → Go to 4.

4 • DID SOMETHING CHANGE SUDDENLY — OVER DAYS, NOT MONTHS?

Yes → Call the doctor today. Sudden confusion or weakness is often reversible — infection, dehydration, medications. Chapters 1–3.

No → Go to 5.

5 • IS MEMORY THE MAIN WORRY — OR WAS DEMENTIA JUST DIAGNOSED?

Yes → **Pathway 3** and Chapter 3. If wandering has started, call the Alzheimer's Helpline (1-800-272-3900) today.

No → Go to 6.

6 • CAN YOUR PARENT STILL MANAGE DAILY LIFE ALONE, SAFELY?

Yes, and nothing has really changed → Go to 7.

Yes — but something is slowly slipping: small changes, a messier house, a quieter

parent → **Chapter 1.** Start the change log and tell the doctor what you are seeing. Worry with nothing provable is a real stage, and it has a chapter.

No → **Pathway 2**, then match the help to the gaps: Chapters 6–9.

7 · IS THE WORRY MONEY, COVERAGE, OR PROTECTING ASSETS?

Yes → **Pathways 4 and 5** and Part 3 (Paying for Care).

No → Go to 8.

8 · IS THE CAREGIVER AT THE EDGE — EXHAUSTED, ANGRY, NOT SLEEPING?

Yes → Chapter 4, today. The self-check takes five minutes. If you are having thoughts of harming yourself, call or text **988** any hour.

No → Go to 9.

9 · NOTHING IS ON FIRE?

Then you have the gift of time → start the Important Documents Folder (Chapter 19), get the Powers of Attorney and advance directive signed (Chapters 20–22), and read For the Future Planners. Future-you says thank you.

Find Your Situation

Wherever you are, this book can meet you there. Some problems are too big for one chapter. They need steps from a few chapters, in the right order, and often fast. These five pathways cover the situations families face most. Each one is two pages: the first steps to take, the mistake that costs families the most, and which chapters to turn to next. If your situation is here, start here. If it is not, the "What Are You Facing?" pages just before this will point you the right way — and the Choose Your Path questions on the page before walk you here, step by step.



Start where you are. The next page begins your path.

YOUR SITUATION

"Mom had a stroke — she's in the hospital and they say she needs a skilled nursing facility."

THE ONE THING THAT PROTECTS EVERYTHING ELSE

Before anything else, ask the hospital — today, and get it in writing: **"Is my mother admitted as an inpatient, or under observation?"** Medicare generally covers the SNF stay only after a **3-day inpatient** hospital stay — observation days and the discharge day do not count. If she is on observation status, ask the doctor whether inpatient admission is appropriate. (Many Medicare Advantage plans — and some Medicare programs — waive this rule; call the plan and ask.) → *Chapters 10 and 13, pp. 91, 107*

Your First Five Moves

1. **Confirm inpatient status** (above) and write down who told you, and when.
2. **Meet the discharge planner** — ask for a list of SNFs with open beds, then check each one on [medicare.gov/care-compare](https://www.medicare.gov/care-compare) before you agree. You may say: "We would like to understand our choices." → *Chapter 10, p. 91*
3. **Get the medication list** and the names of her hospital doctors before the transfer — hand-offs are where medication errors happen. → *Chapter 25, p. 171*
4. **Know the money clock:** with a qualifying stay, Medicare generally pays days 1–20 in full, days 21–100 with a daily coinsurance (\$217 in 2026 — confirm at [medicare.gov](https://www.medicare.gov)), and nothing after 100. Rehab rarely runs that long — but know the numbers before the bills arrive. → *Chapters 13–15, pp. 107–119*
5. **Attend the SNF care conference** (ask for one in week one). Ask: "What are the therapy goals, and what does discharge planning look like from here?"

THE TRAPDOOR TO WATCH

If the SNF says therapy will end because she has "plateaued" or is "not making progress" — **that is not the rule.** Medicare covers skilled care to maintain function too (the *Jimmo* settlement). You can appeal the cut-off notice through your Medicare QIO — the fast appeal costs nothing to file, and the clock is short: call the same day. → *Chapter 10, p. 91.*

While She Recovers — Two Quiet Tasks

- Learn the stroke warning signs for next time — **BE-FAST**: balance, eyes, face drooping, arm weakness, speech trouble → note the time, call 911. → *Chapters 1 and 26, pp. 32, 176*
- If her paperwork is not done and she can still make decisions, this is the week: Healthcare Power of Attorney and advance directive. → *Chapters 20–21, pp. 144–149*

PATHWAY TAKEAWAY

Confirm **inpatient** status today, pick the SNF from real inspection data, and never accept "plateaued" as the end of therapy without an appeal.

YOUR SITUATION

"Dad has been declining — he can't take care of himself anymore."

FIRST, RULE OUT THE REVERSIBLE

If the change is **sudden** — days, not months — treat it as medical until proven otherwise: infections, dehydration, and medications cause confusion and weakness that can be reversed. Call his doctor today and describe what changed, with dates. → *Chapters 1–3, pp. 32–44*

Your First Five Moves

1. **Write the change log:** what he can no longer do safely — bathing, meals, medications, stairs, driving — with examples and dates. Facts move doctors. → *Chapter 1, p. 32*
2. **Book a full evaluation** with his doctor. Bring the log and ask: "Is any of this reversible, and does he meet criteria for a home health evaluation?" → *Chapter 6, p. 67*
3. **Match help to the real gaps** — not the brochure: daily hands-on help is private duty or family (Ch. 7); therapy and nursing at home is home health (Ch. 6); rising supervision needs point to assisted living or memory care (Ch. 8–9).
4. **Get the documents signed while he can:** Healthcare and Financial Power of Attorney and an advance directive require capacity — waiting can cost the family a guardianship case later. → *Chapters 20–22, pp. 144–155*
5. **Call the family meeting** before the crisis picks the date for you — who does what, who pays what, who decides what. → *Chapter 5, p. 58*

THE TRAPDOOR TO WATCH

Medicare does not pay for long-term custodial care — not at home, not in assisted living, not in a nursing home. If his money may run short, start the Medicaid conversation early: the **five-year look-back** (in most states) can penalize money moved late, and an elder-law attorney before any transfer is cheaper than one after. → *Chapters 14–15, pp. 113–119*

And One Thing for You

- ☐ If you are the one absorbing the decline, read the burnout chapter before you think you need it — the scored self-check takes five minutes, and asking for help is planning, not failure. → *Chapter 4, p. 50*

PATHWAY TAKEAWAY

First, check for problems that can be fixed. Then get help that fits what he really needs, sign the legal papers while he still can, and start the money talk now — before a crisis starts it for you.

YOUR SITUATION

"Mom was just diagnosed with dementia — and Dad can't watch her every minute."

FIRST, STEADY YOURSELF

A dementia diagnosis changes the timeline, not the person — today she is still who she was yesterday. Two things matter more this month than everything else combined: **use the capacity window** (documents she can still legally sign) and **build the safety net before the first crisis**, not after. → *Chapters 3, 20–22, p. 44*

Your First Five Moves

1. **Ask the doctor what kind and what stage** — and whether anything reversible is mixed in (medications, thyroid, B12, depression can mimic or worsen dementia). → *Chapter 3, p. 44*
2. **Documents this month, while she can sign:** Healthcare and Financial Power of Attorney and an advance directive. Waiting risks a guardianship case later. → *Chapters 20–22, pp. 144–155*
3. **Call the Alzheimer's Association 24/7 Helpline — 1-800-272-3900** — for local programs, caregiver classes, and support groups; ask about a wandering-response or locator program now, before you need one. → *Chapter 3, p. 44*
4. **Make the house honest:** medication lockbox or organizer, stove and driving plan, door alerts if wandering starts, and a current photo for emergencies. → *Chapters 3, 26, pp. 44, 176*
5. **Decide how Dad gets breaks** — respite is not a luxury; a plan that burns out the well spouse fails both of them. → *Chapter 4, p. 50*

THE TRAPDOOR TO WATCH

Sudden confusion is not "the dementia getting worse." A fast change — hours or days — is delirium until proven otherwise: infection, dehydration, or a new medication. It is often reversible. Call the doctor the day it happens, not after the weekend. → *Chapter 3, p. 44*

When Home Stops Being Enough

- ☐ Watch for the tipping points: wandering Dad cannot catch, nighttime agitation nobody sleeps through, aggression, or care needs beyond two hands. Touring memory care *before* the crisis keeps the choice yours. → *Chapter 9, p. 86*

- On tours, ask the question most families miss: *"How do you handle agitation, fear, aggression, or refusal of bathing and medication — what non-drug approaches do you try first, and what is your approach to antipsychotic medications?"* → Chapter 9, p. 86

PATHWAY TAKEAWAY

Sign the legal papers while he still can, keep the crisis Helpline handy, and put safety steps in place before he wanders the first time. And build in real breaks — this is a long road, and no one can walk it alone.

YOUR SITUATION

"I lost Mom years ago. Now Dad is declining — how do I protect his assets and make sure he gets care?"

THE HONEST ANSWER FIRST

"Protect all the assets" and "qualify for help paying for care" pull against each other, and the law knows it: Medicaid looks back **five years in most states** at money and property moved for less than value, and late transfers can create penalty months when he needs care most. The single highest-value move on this page: **see an elder-law attorney before anyone moves a dollar** — the consult costs a fraction of one look-back mistake. → *Chapters 14–15, pp. 113–119*

Your First Five Moves

1. **Documents while he has capacity:** durable Financial Power of Attorney, Healthcare Power of Attorney, advance directive. Without them, you may need a guardianship case just to help him. → *Chapters 20–22, pp. 144–155*
2. **Build the inventory:** accounts, deed, income, insurance policies — including any **long-term care insurance** he may have forgotten he owns. One folder. → *Chapters 16, 19, pp. 124, 140*
3. **Elder-law consult** (look for CELA certification or a NAELA member): what is exempt, what spend-down would look like, whether any planning still makes sense at his timeline — *before* any transfer, "adding names" to accounts, or signing the house over.
4. **Match care to today's gaps** — home health evaluation, private duty hours, adult day, or a residential move. → *Chapters 6–9, pp. 67–86*
5. **If he ever served:** ask about VA pension with Aid and Attendance — and note the VA has its own **36-month look-back**. → *Chapter 18, p. 135*

THE TRAPDOOR TO WATCH

The neighbor's advice — **"just put the house in your name"** — is how families create Medicaid penalty periods and surprise tax bills at the same time. Widowed parents managing money alone are also prime targets for exploitation: set up account alerts, and treat any new "friend" with financial interest as a red flag. → *Chapter 26, p. 176*

PATHWAY TAKEAWAY

Get the documents done first, then take stock of his money and property, and see an elder-law attorney before moving any assets. Match the help to what he needs now — not to the crisis you're afraid of later.

YOUR SITUATION

"Mom is deciding between Original Medicare and Medicare Advantage."

THE TRADE IN ONE PARAGRAPH

Original Medicare (usually with a Medigap supplement and a Part D drug plan) costs more per month, but nearly any doctor or hospital that takes Medicare, with few prior authorizations. **Medicare Advantage** replaces that with one private plan — lower premium, often dental/vision extras — in exchange for networks, prior authorization, and copays that stack up in a bad year. Neither is "wrong." One question decides most cases: *which would she rather have in the year she gets really sick?* → Chapter 13, p. 107

The Five Questions to Answer Before Choosing

1. Are **her doctors and preferred hospital** in the Advantage plan's network — and what happens if they leave it?
2. Are **her medications** on the plan's formulary, and at what tier? (Run them through the Medicare Plan Finder at [medicare.gov](https://www.medicare.gov).)
3. Can she **afford the Medigap premium now**? Predictable premiums versus unpredictable copays is the real math.
4. Does she **travel or split the year** between states? Networks travel poorly; Original Medicare goes everywhere.
5. How would she feel about **prior authorization** for rehab or home health during a serious illness?

THE TRAPDOOR TO WATCH

The choice **can get you stuck** more than you might think. Leaving Advantage later may mean buying Medigap *with medical underwriting* — in most states the insurer can charge more or say no because of her health, exactly when she most wants back in. Ask SHIP about guaranteed-issue windows and trial rights *before* choosing, not after.

Free, Unbiased Help

- **SHIP** — free, unbiased Medicare counseling: 1-877-839-2675 · shiphelp.org (they will sit with her and run her exact doctors and drugs)
- **Open Enrollment** runs October 15 – December 7 each year; plans change their networks and formularies annually — recheck even if she is happy

PATHWAY TAKEAWAY

Check that her own doctors and medicines are covered by the plan. Look at what a bad year would really cost, not just the monthly premium. And learn how the Medigap health questions work before she picks a plan — not after.

For the Future Planners

If nothing is urgent yet, this is where you start.

A calm stretch may be the best time to use this book.

A rule of thumb worth keeping: 40/70. When the children reach 40 or the parents reach 70 — whichever comes first — it is time to start these conversations. Not because anything is wrong, but because nothing is: everyone still has their full say, and a plan made around a kitchen table beats one made in a hospital hallway every time.

Most families wait until the fall, the hospital stay, the diagnosis, the discharge meeting, or the late-night phone call. By then, every decision feels heavier because it has to be made fast. Planning ahead does not mean you are expecting the worst. It means you are protecting the people you love from having to guess later.

Start with the blue chapters: Medicare Basics, Medicaid Basics, Private Pay, Long-Term Care Insurance, Prescription Assistance, VA Benefits, The Important Documents Folder, Healthcare Power of Attorney, Living Will / Advance Directive, Financial Power of Attorney, and Medication and Doctor Lists. Then build the Emergency Contacts and Crisis Plan (Chapter 26) — it wears red because of *when* you will need it, but the best time to build it is now, while nothing is burning.

Your goal is simple: know where the documents are, understand who can make decisions, talk about money before panic enters the room, and write down the medical information no one wants to hunt for during an emergency.

A future planner does not need to solve everything. Start with three questions: Who can speak for my parent if they cannot speak for themselves? Where are the documents, passwords, insurance cards, and medication lists? What would my parent want if life changed quickly?

That is not fear. Not having a plan is what costs families — sometimes thousands, sometimes millions.

■ **Worksheet — Future Planner Snapshot** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

You Are Not Alone

Before you learn anything in this book, put this down first: you are not alone, and you are not failing.

If you are reading this, someone you love needs more than they used to. Maybe it happened overnight — a fall, a phone call, a hospital hallway. Maybe it crept in so slowly you only see it now, looking back. Either way, you did not sign up for this, nobody trained you for it, and you are somehow supposed to know what to do. Of course you feel overwhelmed. Anyone standing where you are standing would.

So take one breath before the work begins. You do not have to fix everything today. You do not have to have the answer to a question you were handed an hour ago. You only have to take the next right step — and this book exists to show you what that step is, one page at a time.

You will feel tired. You will feel guilt you did not earn. You will wonder if you are doing it right. That doubt is not a sign you are the wrong person for this. It is a sign you love them. The ones who never wonder are rarely the ones sitting where you are sitting.

One more truth, so you stop blaming yourself for it: when a baby is born, the neighborhood shows up — casseroles, visits, advice nobody asked for. When a diagnosis comes, the phone goes quieter. It is not that people stopped caring. It is that nobody ever taught them what to say to this. There is no baby shower for caregiving, no card aisle for it, no script. This book is, among other things, the script nobody handed any of us.

There are millions of us — the sons and daughters and spouses quietly carrying someone through the hardest season of their life. You have joined us. We are glad you are here, and we are sorry you needed to come. Whatever brought you to this page, you do not have to walk it alone. Turn the page when you are ready.

PART 1 · SOMETHING HAS CHANGED

CHAPTER 1

Something Has Changed

Part 1 — Something Has Changed. This part is for the moment when your gut knows something is different, but you do not yet know what it means. A parent may seem weaker. More forgetful. Less steady. More withdrawn. Or nothing dramatic has happened yet, but the daily rhythm is off. Many caregiving journeys begin before the crisis, with small signs that are easy to explain away. You do not need to panic. You do need to pay attention.

“The best time to plant a tree was twenty years ago. The second best time is now.”

— PROVERB

THIS CHAPTER IN 30 SECONDS

- **The rule:** a sudden change should always be taken seriously. The question is not “Is this normal aging?” but: what changed, when, and is it affecting safety?
- **The myth:** that you must be certain before you act. Honest attention is enough.
- **First move:** write down the change with facts and dates (“got lost driving to church on March 3”).
- **The script:** raise it gently — *“I’ve noticed the stairs seem harder lately — can we figure out something together?”*
- **Watch for:** sudden confusion — infection, dehydration, medication changes, pain, poor sleep, and low oxygen can cause it, and may be treatable. If the change is sudden, call the doctor; if anyone is in immediate danger, call 911.

WHAT FAMILIES USUALLY THINK

Families wait until they can prove something is wrong, because naming it makes it real.

You are not accusing your parent of failing. You are noticing that life may require more support than it used to. That difference matters.

What You May Be Seeing

Physical changes.

- ☐ More falls or near falls
- ☐ Trouble standing or walking
- ☐ Weight loss, less interest in food, or poor hygiene
- ☐ Shortness of breath or new incontinence

Memory and thinking changes.

- ☐ Repeating the same question
- ☐ Getting lost in familiar places
- ☐ Missing appointments or medications
- ☐ Trouble managing bills or falling for scams

Home safety changes. Spoiled food, burned pans, unpaid bills, scattered medication bottles, loose rugs, or trouble using the shower. The home often tells the truth before the person does. Look gently. Do not shame.

Emotional and behavioral changes. Withdrawal, irritability, depression, new anger, suspicion, refusing help, or more confusion in the evening. Behavior is information — grief, pain, depression, infection, dementia, medication side effects, or loneliness. The point is not to label it but to write it down and ask better questions.

What Is Urgent and What Can Wait

Call the doctor when a change is new, worsening, or affecting daily life. A sudden change can have a medical cause — do not assume it is "just aging."

CALL 911 FOR

Chest pain, trouble breathing, signs of stroke, serious injury from a fall, uncontrolled bleeding, loss of consciousness, sudden severe confusion, or a person who cannot be moved safely. **Signs of stroke are sudden:** face drooping, arm weakness, slurred speech, vision loss, trouble walking. Note the time it started and call 911 immediately — stroke treatment is time-limited.

Call the doctor soon for: a fall without obvious injury, new confusion or weakness, medication errors, weight loss, eating or drinking less, or a caregiver who can no longer manage safely. (One exception: a head strike in someone on a blood thinner is an emergency-department visit the same day — see the red box in Chapter 2.)

Schedule a planning conversation for: gradual decline, difficulty managing the home, unclear documents, no emergency plan, or concerns about driving or living alone. Planning early is how families avoid decisions forced by crisis.

How to Raise It

Raise it with your parent first, and gently. "I've noticed the stairs seem harder lately — can we figure out something together?" lands far better than "You can't manage anymore." People resist being managed and respond to being respected. Then bring it to the physician. Before the visit, write down specific examples: "got lost driving to church on March 3." Vague worry is easy for a busy doctor to dismiss; concrete observations get taken seriously.

WHAT TO DO THIS WEEK

1. **Write down the change.** Use facts and dates. Do not rely on memory during stress.
2. **Check immediate safety.** Bathroom, medications, food, stove, driving, and the caregiver's safety too.
3. **Call the doctor** if the change is new or concerning.
4. **Tell the key family.** Awareness, not panic.
5. **Choose the next right section of this guide.** Physical change → Chapter 2, p. 37.
Memory → Chapter 3, p. 44. Caregiver overwhelmed → Chapter 4, p. 50. Decisions needed → Chapter 5, p. 58.

■ MINI-TOOL — THE "SOMETHING HAS CHANGED" NOTE

Use this when you are worried but not sure what to do next.

What changed? _____

When did it start? _____

Is it getting better, worse, or the same? _____

Is anyone unsafe today? _____

What have we already tried? _____

Who needs to know? _____

What is the next call? _____

What is the next appointment? _____

What information do we need to find? _____

■ QUESTIONS TO ASK THE DOCTOR

1. Could this change be caused by infection, dehydration, medication, pain, or another treatable issue?

2. Are any medications possibly causing confusion, dizziness, weakness, or falls?
3. Should we check bloodwork, urine, oxygen, or blood pressure?
4. Is it safe for my parent to live alone right now?
5. Should my parent be driving?
6. Would home health, therapy, or nursing support help?
7. Should we consider a memory evaluation?
8. What changes should make us call you again — and what should send us to the hospital?

ROB'S NOTE

Families usually know more than they think.

They may not know the diagnosis. They may not know the benefit rules. But they know when something feels different.

I have watched families talk themselves out of that feeling because they did not want to upset anyone. I understand that. No one wants to be the person who says, "Mom is not safe anymore."

But noticing a change is not betrayal. It is care. You are not taking over. You are paying attention. And sometimes, paying attention is the first loving thing you can do. It can turn panic into a plan.

CHAPTER 1 TAKEAWAY

You do not need to diagnose the problem today. The next right question is: **what changed, and is anyone unsafe today?**

CARDINAL MOMENT



A daughter once told me her mother was "completely fine." Then, almost like it was no big deal, she added that her mom had started keeping the mail in the refrigerator. She said it in a calm, everyday way. For months, she had found a reason for each small thing. On its own, each one was easy to explain away.

But when she listed them out loud, all in a row, she finally heard it. She cried. Nothing new had happened. She had just let herself see what she already knew. Then she wiped her face and said, "Okay. What do I do first?"

That question is where everything starts. Noticing is not being disloyal. It is a loving thing to do for someone who can no longer take care of everything alone.

A composite drawn from years at the bedside. Names and details changed.

Falls, Weakness, and Decline

THIS CHAPTER IN 30 SECONDS

- **The rule:** a fall is not just a fall. Ask why it happened and what has to change.
- **The myth:** "She just tripped." Caution is not a care plan. Your job is not to blame — it is to investigate.
- **Red line:** a head strike on a blood thinner — even daily aspirin — means the emergency department the same day.
- **The script:** ask for "*an evaluation to determine whether home health is appropriate.*"
- **Watch for:** near falls — the smoke alarm before the fire. Log every one.

WHAT FAMILIES USUALLY THINK

Families often say, "She just tripped." Sometimes that is true. But most older adults are not falling because they are careless. Something has changed — the body, the home, the medications, the disease, or the support they need.

Why Falls Matter

A fall is rarely a single bad moment. For many older adults it is the hinge the rest of life turns on: a broken hip leads to surgery, surgery to a hospital stay, and days in bed cost strength a frail body may never rebuild.

Even without injury, many become afraid, move less, and weaken — which sets up the next fall. Doctors call the strength lost from lying still *deconditioning*, and it happens fast. So the goal is never just to treat the injury. It is to protect strength and independence — starting the same week, with movement and therapy, not bed rest.

After a Fall: First Questions

Start with safety. Are they hurt? Did they hit their head? Are they bleeding? Can they move normally? Are they confused?

ACT NOW

If there is serious injury, a possible head injury, sudden confusion, chest pain, trouble breathing, one-sided weakness, severe pain, or they cannot be safely moved, **call 911**. Do not force someone up if you are not sure they are safe to move.

While they are on the floor: check for injury first, keep them warm and calm, and if they are uninjured but cannot get up, call 911 or the non-emergency line and ask for a **lift assist**. And know the term "long lie": an older adult who has been down for an hour or more needs medical evaluation even without visible injury.

When to Call the Doctor After a Fall

Call after a fall if:

- ☐ It is new or repeated
- ☐ They hit their head
- ☐ They are on blood thinners
- ☐ They have new pain or weakness
- ☐ They are dizzy or confused
- ☐ They do not remember falling
- ☐ They are walking differently
- ☐ They are afraid to walk
- ☐ They are eating or drinking less
- ☐ They recently changed medication
- ☐ You are not sure why the fall happened

Do not assume a fall is normal aging. A doctor may review medications, check blood pressure and hydration, screen for infection, or order therapy.

HEAD INJURY + BLOOD THINNERS

If your parent takes a blood thinner — anticoagulants like warfarin, Eliquis, or Xarelto, antiplatelet drugs like Plavix, or even daily aspirin — and hits their head, do not wait for symptoms. **Go to the emergency department the same day, even if they seem fine.**

Bleeding inside the head can show up hours or days later. After any head strike, watch for worsening headache, vomiting, new confusion, or unusual sleepiness over the next 24–72 hours.

Weakness Is Information

Weakness may look like trouble standing from a chair, holding walls while walking, shuffling steps, or refusing a needed walker. Write down when it started, whether it came

on suddenly, whether it followed a hospital stay or a medication change, and whether there is pain, shortness of breath, or dizziness.

Decline After the Hospital

An older adult can decline sharply after a hospital stay, so the discharge plan matters. Before discharge, ask: Can they walk safely? Can they get to the bathroom? What equipment do they need? Home health or rehab? Who manages medications, and who do we call if things get worse?

When Home Health May Help

After a fall, new weakness, illness, or a hospital stay, ask whether home health may be appropriate.

IMPORTANT

A family cannot simply request home health over the phone and receive it. For Medicare-covered home health, a clinician must determine that the person meets the requirements — including a skilled need, homebound status, and the required face-to-face documentation. So the better request is: **"Can we schedule an evaluation to discuss this fall or decline and determine whether home health is appropriate?"**

Home Health vs. Private Duty Care

HOME HEALTH	PRIVATE DUTY CARE
Usually short-term and medical	Usually non-medical daily support
Ordered by a clinician; criteria apply	Arranged by the family; often paid privately
Nursing, PT, OT, speech, social work, limited aide	Bathing, dressing, meals, housekeeping, companionship
Not there all day	Can cover longer blocks, including overnight
Helps treat, teach, assess, and strengthen	Helps with daily support and supervision

A family may need both. The question is: **what does my parent actually need at home?**

Home Safety After a Fall

A home that was fine for a healthy adult can be full of hazards for someone with reduced balance, vision, or cognition:

- ☐ Loose rugs and clutter
- ☐ Poor lighting, and no nightlight from bed to bathroom
- ☐ Stairs without railings; chairs that are hard to rise from
- ☐ A slippery bathroom: no grab bars, shower chair, handheld shower head, non-slip mat, or raised seat
- ☐ No bedside commode at night; no phone or call device nearby

Fixing these extends independence rather than restricting it.

Near Falls Count Too

Families remember the big fall but not the near falls — grabbing the counter, stumbling but catching themselves, "I just got dizzy for a second." Several near falls deserve to be taken as seriously as a fall.

When Falls May Mean More Help Is Needed

A fall does not automatically mean assisted living or memory care. But repeated falls may mean the current setup is no longer safe — especially if your parent lives alone, cannot call for help, forgets the walker, falls at night, cannot bathe safely, is on blood thinners, or has dementia. The goal is not to keep someone home at all costs — it is to keep them safe, supported, and dignified.

WHAT TO DO THIS WEEK

1. **Write down every fall and near fall** — including whether they hit their head.
2. **Call the doctor** if the fall is new, repeated, or unexplained.
3. **Ask for a home health evaluation.**
4. **Review medications** for fall risk — do not stop any on your own.
5. **Make the home safer**, starting with the path from bed to bathroom.
6. **Ask whether more support is needed** — therapy, private duty care, rehab, or a care-setting change.

■ MINI-TOOL — THREE QUESTIONS AFTER A FALL

1. **What happened?** Where was the person? What were they doing? Did they trip, slip, faint, or feel dizzy? Did anyone see it?
2. **What changed?** New pain? New confusion or weakness? New fear of walking? New medication? Less eating or drinking?

3. What needs to change now? Call the doctor? Home safety changes? Therapy? Home health? More supervision? Safe alone tonight?

Fall Log — record every fall and near fall. Bring this to appointments.

DATE	TIME	LOCATION	WHAT HAPPENED	INJURY / NEXT STEP

■ QUESTIONS TO ASK THE DOCTOR

1. What could be causing the falls or weakness?
2. Could medications be contributing?
3. Should blood pressure be checked sitting and standing?
4. Should we check for infection, dehydration, anemia, or pain?
5. Is physical or occupational therapy appropriate?
6. Would an evaluation for home health be appropriate?
7. Does my parent need a cane, walker, wheelchair, shower chair, or bedside commode?
8. Is it safe for my parent to live alone, bathe alone, or drive?
9. What signs should make us call again — and what signs mean call 911?

ROB'S NOTE

Families often tell me about "the fall."

But after I ask a few more questions, I usually hear about the near falls that came before it. The stumble in the hallway. The hand grabbing the kitchen counter. The bad turn in the bathroom. The moment Dad said, "I'm fine," then had to sit down for twenty minutes.

A fall is not always the beginning. Sometimes it is the first thing the family can no longer ignore.

That is why I want families to write things down early. Not to build a case against the person they love. To build a plan around what is really happening. Asking about home health care is not overreacting. It is smart. A nurse or therapist who comes to the home may notice things the family misses. The family is too close, too tired, or too used to getting by.

CHAPTER 2 TAKEAWAY

Falls, weakness, and decline are information. The next right question is: **why did this happen, and what needs to change so it does not happen again?**

CARDINAL MOMENT



A man told me his wife had "just slipped in the kitchen." That morning, she was walking on her own. A few weeks later, when I met them, she was in a wheelchair. It was not from the broken hip. That had healed. It was from all the weeks she spent lying still while it healed.

He kept saying, "It was only one fall." But that was not the part he blamed himself for. No one had told them the real danger. It was not the broken bone. It was staying still for so long after it. Once therapy started and she began to move again, some of her strength came back. Not all of it. But enough to show them both that moving sooner would have helped.

If someone you love falls, treat the days that follow like they matter. Because they do.

A composite drawn from years at the bedside. Names and details changed.

Memory Loss and Dementia

THIS CHAPTER IN 30 SECONDS

- **The rule:** memory loss is not always dementia. The first question is safety.
- **The myth:** a parent can sound fine on the phone and still be unsafe alone.
- **First move:** write down examples with dates. Not "Dad is getting forgetful" but "Dad got lost driving home from church and paid the same bill three times."
- **The script:** ask *"Is there anything simple and reversible we might be missing here?"*
- **Watch for:** sudden confusion is delirium, a medical urgency, not dementia's slow slope.

WHAT FAMILIES USUALLY THINK

Families think dementia means someone who does not know who or where they are. But many still tell old stories and sound fine on the phone.

Safety Comes First

The first question is not "Is this dementia?" It is **"Is this safe?"** Memory loss becomes urgent when it touches medication, driving, cooking, finances, or being alone. Safety protects dignity from a preventable crisis. Watch for:

- ☐ Repeating questions
- ☐ Forgetting medications or taking them twice
- ☐ Trouble with driving, the stove, or the bills

One mistake may not mean dementia. A pattern deserves attention.

Medication is the clearest signal: a full pillbox when it should be empty, duplicate bottles, morning pills taken at night. Do not remind your parent to be careful. Change the system: a pill organizer, blister packs from the pharmacy, phone alarms, a medication review, or home health nursing.

Driving Safety

Driving means independence and identity. Warning signs:

- ☐ Getting lost on familiar roads

- ☐ New dents, drifting from the lane, or missing stop signs
- ☐ Family feeling afraid as passengers

Ask the doctor, "Is it safe for my parent to continue driving?" and ask about a formal driving evaluation if available. Let the physician deliver the verdict — when the doctor says the driving is over, the family stays family instead of becoming the enemy. If driving must stop, build the transportation plan *before* the keys leave — scheduled rides from family, ride-share, senior transit, grocery delivery — for appointments, groceries, church, and social connection. Removing keys without a plan creates anger and isolation. When driving ends, support must replace what is lost.

Money, Cooking, and Wandering

Money and scams. Unopened mail, unusual withdrawals, new "friends" asking for money. Ask whether there is a financial power of attorney and whether bills can go on autopay. Financial decline is a symptom, not a character flaw.

Cooking and the kitchen. Burned pans, a stove left on, spoiled food. If the stove is unsafe, act: meal delivery, stove shut-off devices, or removing the knobs.

Wandering and getting lost. Leaving the house at night, trying to "go home" while already home. Ask whether the person can safely be alone and whether doors are secured. Program the Alzheimer's Association 24/7 Helpline (1-800-272-3900) into your phone and ask about a wanderer locator program (MedicAlert, Project Lifesaver, or similar), and tell trusted neighbors.

Mood and Behavior Changes

Dementia can bring anxiety, depression, suspicion, hallucinations, agitation, sundowning, or refusing care. Behavior is still information: pain, infection, medication side effects, poor sleep, and dehydration all worsen confusion. Do not assume every change is "just dementia."

When they cannot see it: anosognosia. Some people with dementia genuinely cannot perceive their own decline — the same brain changes that cause the illness also hide it from them. It has a name: anosognosia. Your mother is not being stubborn when she insists she is fine; her brain is reporting that she is. This is why rational safety arguments so often fail — and why the next section matters: when logic cannot land, connection still can.

Entering Their Reality

As dementia progresses, the rules of conversation change. The goal shifts from exchanging accurate information to preserving connection and dignity. When a person with dementia believes something that isn't true — that they need to go to a job they retired from decades ago, that a long-dead spouse is coming home — arguing or correcting rarely helps and often causes real pain. Respond to the feeling, not the fact. "You're worried about getting to work on time" acknowledges the emotion without a confrontation that neither of you can win.

Keep it simple and calm. Use short sentences and one idea at a time. Ask one question, not several. Give simple choices — "Would you like the blue sweater or the green one?" — rather than open-ended ones. Your tone and body language carry more than your words; a calm, warm presence reassures even when the words don't land.

A Predictable Routine Helps

Routine lowers anxiety. Anchor the day: consistent wake and sleep times, regular meals, medication at the same times. Put bathing and appointments at your parent's best hour, often the morning, and keep late afternoon and evening calm, when sundowning arrives. Leave room for a walk, music, a visit.

Rule Out the Reversible Causes

Some conditions look exactly like dementia and are treatable: a urinary tract infection, dehydration, a thyroid problem, a vitamin B12 deficiency, depression, or a bad combination of medications. Sudden confusion is **delirium**, coming on over hours to days. It is a medical urgency, unlike dementia's slow slope over months to years, and using the word with the care team gets it taken seriously. Insist the doctor rule these out: **"Is there anything simple and reversible we might be missing here?"**

Call the doctor when changes are new, worsening, sudden, or affecting safety: sudden confusion, medication mistakes, falls, or wandering.

When Home Health or Memory Care May Help

Home health may help when memory loss comes with a medical need: falls, weakness, medication concerns, or hospitalization. It is not long-term supervision or a sitter service. Medicare-covered home health requires a clinician to evaluate the person and confirm skilled need, homebound status, and face-to-face documentation.

Memory care may be needed when dementia creates risks that cannot be managed at home: ongoing wandering, medication mistakes, being unsafe alone, or a caregiver who

cannot sleep. It is not a punishment. It is a care setting. Do not wait for a collapse or an injury to have the conversation.

WHAT TO DO THIS WEEK

1. **Write down examples with dates** — medication, driving, wandering, behavior.
2. **Check safety** — medications, driving, cooking, being alone, money.
3. **Call the doctor** and ask for a cognitive evaluation.
4. **Review medical causes** — infection, dehydration, pain, medication side effects.
5. **Get documents in order early** — healthcare and financial power of attorney, advance directive, medication and doctor lists, emergency contacts.
6. **Decide what support is needed now** — family check-ins, home health, private duty care, adult day care, assisted living, or memory care.

■ MINI-TOOL — MEMORY AND SAFETY SNAPSHOT

Use this when you are worried about memory loss but unsure what to do next.

What changed? _____

When did it start? Is it getting worse? _____

Is medication safe? _____

Is driving safe? _____

Is cooking safe? _____

Is the person safe alone? _____

Are bills and money safe? _____

Is the caregiver safe and able to continue? _____

Who needs to know? What is the next call? _____

■ DEMENTIA SAFETY CHECKLIST

Check any concern that applies. If several are checked, do not wait — call the doctor and begin a family planning conversation.

- | | |
|---|--|
| ☐ Repeats questions often | ☐ Misses bills or falls for scams |
| ☐ Forgets or misuses medications | ☐ Misplaces items or accuses others of stealing |
| ☐ Gets lost driving or has new driving concerns | ☐ Wanders or tries to leave |
| ☐ Leaves stove or appliances on | ☐ Is confused at night |
| ☐ Forgets to eat or has spoiled food in the home | ☐ Has unsafe anger or agitation |

- ☐ Has poor hygiene
- ☐ Caregiver is exhausted or unsafe
- ☐ Cannot call for help reliably

■ QUESTIONS TO ASK THE DOCTOR

1. Could this be dementia, delirium, depression, medication side effects, infection, dehydration, or pain?
2. Should we do a memory screening and review medications?
3. Should we check labs or urine?
4. Is my parent safe to live alone, drive, cook, and manage medications?
5. Would an evaluation for home health, nursing, therapy, or social work help?
6. Should we see a neurologist, geriatrician, or memory clinic?
7. What changes should make us call again — and what should be treated as urgent?

ROB'S NOTE

Dementia can be very hard. Families lose the person a little at a time.

One day they can still tell stories from forty years ago. The next day they cannot remember if they ate lunch. They may know who you are one minute, then blame you the next. They may sound fine on the phone, then leave the stove on after dinner.

That back-and-forth confuses families. It can also split them apart. The person who visits for an hour may think everything is fine. The person who handles the pills, the bills, and the late-night calls knows it is not.

If that is you, write things down. Not to prove everyone else wrong. To help the whole family see clearly. Memory loss needs kindness. It also needs a plan. It can need both at once.

CHAPTER 3 TAKEAWAY

Memory loss is about safety, judgment, and daily life — not only forgetting. The next right question is: **what is no longer safe to leave to memory alone?**

CARDINAL MOMENT



A woman kept asking to go home. But she was already standing in the house where she had lived for forty years. She did not mean this home. She meant a porch, a dog, her mother's cooking. She meant a place from long ago. Her son used to correct her. "Mom, you ARE home." Every time, she got more scared. She felt more sure she had been taken somewhere strange.

One tired evening, he stopped correcting her. Instead he asked, "Tell me about home. What's it like there?" She talked for twenty minutes. As she talked, she calmed down. He had not lied to her. He had just stopped arguing about a place he could no longer follow her to.

You cannot win an argument against a disease. But you can almost always answer the feeling behind the words.

A composite drawn from years at the bedside. Names and details changed.

Caregiver Burnout

THIS CHAPTER IN 30 SECONDS

- **The truth:** burnout is a warning sign, not weakness. When a caregiver burns out, everyone becomes less safe.
- **The myth:** "I should be able to do this." Love does not remove exhaustion.
- **The check:** take the self-check, then ask whether the caregiver and the parent are both safe under the current plan.
- **This week:** write down the actual workload, then ask by task — *"Can you take Mom Thursday at 10?"*
- **If you are at the edge:** call or text 988 any hour, and ask about respite — the Eldercare Locator is 1-800-677-1116.

WHAT FAMILIES USUALLY THINK

Families think caregiving should be manageable because it is done out of love. "She's my mother." "I should be able to do this." But love does not lift a person safely from the bathroom floor, make dementia predictable, or turn one daughter into a care team.

What Burnout Looks Like

Burnout can look like sadness, anger, control, or silence — a daughter who snaps at everyone, a son who stops answering calls, a spouse who cries in the shower. Watch for constant exhaustion, irritability, trouble sleeping, ignoring your own health, and dreading the phone. Sometimes "I'm fine" means "I have no room left to fall apart."

IF YOU ARE AT THE EDGE

If you are having thoughts of harming yourself, or you are afraid of what exhaustion might make you do, call or text **988** — the Suicide & Crisis Lifeline — any hour. Tell your doctor what caregiving is costing you: caregiver depression is treatable, not a character flaw. Ask the doctor, a social worker, or the Eldercare Locator (1-800-677-1116) about respite today.

It Has a Name

Burnout is real, and it has a name. Herbert Freudenberger named it in the 1970s; in 2019 the World Health Organization formally recognized it — deep exhaustion from chronic stress that was never given a chance to ease.

Two things are worth knowing about it. First, burnout is not the same as stress. Stress is carrying too much; burnout is carrying too much, for too long, with no relief and no backup — pressure plus isolation. That is why a weekend off does not fix it, but a support system often does. Second, keep this in mind the next time you feel unprepared: hospitals give new nurses months of training and a mentor before their first shift alone — and many still leave the work within the first year. Families get a diagnosis, a discharge packet, and a parking ticket. If trained professionals buckle under this work *with* support, feeling overwhelmed without any is not weakness. It is arithmetic.

It rarely arrives in a single day. Freudenberger and Gail North described a slow slide of about a dozen stages — the drive to prove you can handle it alone, neglecting your own sleep and your own doctor, denying the strain, withdrawing from the people who would help, and then emptiness, depression, collapse.

The Hidden Load

Caregiving is not only the visible tasks. It is the hidden load: refills, appointments, watching for decline, insurance and bills, keeping peace between siblings, worrying at night. This is why one caregiver may seem "too emotional" to relatives who are less involved. They carry the whole weather system.

One more thing to name: anticipatory grief. You may be mourning someone who is still alive — grieving the conversations you used to have, the father who remembered your birthday, the mother who was the one you called for advice. It feels forbidden, so most caregivers bury it and call it guilt. It is not guilt, and it is not disloyalty. It has a name — anticipatory grief — and it is one of the most normal experiences in caregiving. You are allowed to miss someone you are still holding.

The Promise That Traps Families

Many families carry a promise: "I promised Mom I would never put her in a nursing home." Those promises come from love, but they are made before anyone understands what care may require — before wandering, before falls at night, before bathing is unsafe. The deeper promise is not "I will keep you in this exact place." It is "I will make sure you are cared for. I will not abandon you." Sometimes keeping it means changing the plan. That is not betrayal.

And here is the part nobody says out loud: when the physical work of care grows past what one exhausted person can give, handing that work to professionals is often the only way to save the relationship itself. Care done at the edge of collapse gets worse, not better — for everyone. Let someone else carry the lifting and the medication schedule, and you get to stop being the worn-out nurse and go back to being the daughter, the son, the husband. That trade is not giving up. It is choosing the part of the job only you can do.

Resentment Is Information

Caregivers are ashamed of resentment. But resentment is information. It may mean the workload is unfair, there is no relief, or the plan no longer works. Instead of "I'm a terrible person for feeling this way," try "What is this resentment telling me about the care plan?"

The Family Pattern Problem

Caregiving does not create family patterns. It reveals them. The responsible sibling becomes more responsible, the avoidant sibling more avoidant, and old wounds walk into the family meeting and sit down. You may not be able to fix the family system, but you can make the next decision clearer: facts first, **one point person**, specific tasks and dates.

When to Ask for Help

Ask for help before the caregiver breaks. Help may come from:

- Family, friends, faith or community groups
- The doctor, a social worker, or a counselor
- Home health (if there is a skilled need and criteria are met), private duty care, adult day care, respite care
- Assisted living, memory care, palliative care, hospice if appropriate

And find one person who is a year ahead of you — a friend who walked this road with their own mother, a support group, another family from the waiting room. Nurses stay in the profession when someone mentors them through the first year; caregivers are no different. One person who can say "that's normal — here's what helped us" is worth a shelf of advice.

When you ask, name the task. Not "I need help with Mom," but "Can you take Mom to her appointment Thursday at 10?" Specific requests are harder to ignore, and they show who is willing to help.

When the Parent Refuses Help

Many caregivers are trapped by one sentence: "They won't let me." Refusal does not mean the current plan is safe. Ask whether the parent understands the risk, whether memory loss is affecting judgment, and whether a doctor should evaluate capacity or safety. Sometimes help gets in with different words — "Someone is coming to help with laundry and lunch" rather than "You need a caregiver." The goal is not to win the argument. It is to lower the risk.

PERMISSION TO REST

A nurse, Linda Liner, RN, shared something worth passing on. Over the years, she has heard family members say the same thing: *"I wish someone had given me permission to go home and get some rest."*

So here it is, in writing: you have permission. Go home. Sleep. Rest is not giving up on them — it is how you keep showing up.

Finding Your Way Back

Burnout's opposite is not happiness — it is feeling like yourself again. The sociologist Corey Keyes named the gray middle so many caregivers live in: *languishing* — flat, not depressed exactly, but not flourishing. The way back is small, deliberate steps:

- **Name it.** Say out loud, "I am languishing." Naming a feeling loosens its grip.
- **Protect one hour that is yours,** guarded like an appointment.
- **Stay connected.** One person who knows is a lifeline, not a luxury.
- **If the flatness does not lift,** tell your doctor. Languishing can slide into depression, and depression is treatable.

WHAT TO DO THIS WEEK

1. **Write down the actual workload.** The family cannot divide work it has not named.
2. **Take the burnout self-check** on the next page. Do not argue with the score.
3. **Hold a family meeting** (Chapter 5) with names, tasks, and dates.
4. **Add one form of relief this week** — meals, help with bills, paid care, adult day care, or respite.
5. **Recheck the care setting** — is home still safe, is one caregiver enough, is nighttime care breaking the caregiver?

■ MINI-TOOL — CAREGIVER BURNOUT SELF-CHECK

This is not a diagnosis. It is a warning-light checklist. Use it when the caregiver says, "I'm fine," but everything around them suggests otherwise. Check any statement that feels true most days, then total each section.

Physical exhaustion (___ / 8)

- | | |
|---|---|
| ☐ Tired even after sleeping | ☐ Skipping my own appointments |
| ☐ Not sleeping enough | ☐ Eating worse than usual |
| ☐ Waking at night over caregiving | ☐ Getting sick more often |
| ☐ More headaches, tension, or pain | ☐ Physically worn down |

Emotional strain (___ / 8)

- | | |
|---|---|
| ☐ Anxious much of the time | ☐ Guilty when I want a break |
| ☐ Sad, numb, or hopeless | ☐ Feeling trapped |
| ☐ Crying more easily | ☐ Resentful that others don't help |
| ☐ Angry or irritated often | ☐ Dreading calls and new problems |

Caregiving load (___ / 8)

- | | |
|---|--|
| ☐ Managing most appointments | ☐ Doing bathing, meals, or daily care |
| ☐ Managing medications | ☐ The main person everyone calls |
| ☐ Managing bills and insurance | ☐ No reliable backup |
| ☐ Doing most transportation | ☐ If I stop, everything falls apart |

Safety concerns (___ / 8)

- | | |
|--|--|
| ☐ Parent should not be left alone long | ☐ I've lifted my parent when it felt unsafe |
| ☐ Parent has fallen or nearly fallen | ☐ Afraid something bad happens if I leave |
| ☐ Parent is confused about medication | ☐ Making more mistakes because I'm tired |
| ☐ Parent is unsafe cooking, driving, or walking | ☐ The current plan does not feel safe |

Total score: _____ / 32

SCORE	WHAT IT MAY MEAN, AND THE NEXT STEP
0–7 · Watch & Plan	Managing for now. Keep a written care list, name one backup, review emergency contacts, organize documents.
8–15 · Stress Building	Hold a family meeting. Assign tasks. Ask what can come off the caregiver's plate. Consider private duty, adult day support, or a home health evaluation.
16–23 · High Risk	Schedule a family meeting soon. Ask for concrete help. Consider paid support. Review whether the parent is safe alone.
24–32 · Crisis	Do not wait. Call a family meeting. Contact the doctor, social worker, or care team. Consider urgent respite or a care-setting change. If anyone is in immediate danger, call 911.

The two most important questions: Is the caregiver safe? Is the person receiving care safe under the current plan? If the answer to either is no, the plan needs to change — not because anyone failed, but because the need has outgrown the plan.

■ CAREGIVER LOAD LIST

Bring this to the family meeting. The goal is to make the invisible work visible, then assign a helper and a start date to each task.

TASK	CURRENTLY DONE BY	NEW HELPER / DATE
Appointments & doctor calls		
Medications & refills		
Bathing, dressing, daily care		
Meals, laundry, housekeeping		
Transportation		
Bills, mail, insurance		
Overnight supervision		
Emotional support & family updates		

What must come off the caregiver's plate this week? _____

ROB'S NOTE

I have met caregivers who could tell me every pill, every diagnosis, every appointment, and every change in their parent's health.

Then I would ask, "How are you?" And the room would go quiet. Not because they did not know the answer. Because no one had asked them in a long time.

Caregivers often end up doing it all. The schedule. The pills. The late nights. The worrying. But everyone has a limit. When the caregiver breaks down, that is not a small, private problem. It is an emergency for the whole family.

If you are the caregiver, please hear this. You are allowed to ask for help before you break down. You are allowed to say the plan is not working. You are allowed to love someone and still be worn out.

CHAPTER 4 TAKEAWAY

Caregiver burnout is not weakness. It is information. The next right question is: **is the care plan protecting both the parent and the caregiver?**

CARDINAL MOMENT



A son had slept in a recliner outside his mother's bedroom door for eight months. That way, he would hear her if she got up in the night. He never chose to become the night shift. It just happened one night, and then it never stopped.

He told me he was fine. But his hands were shaking as he said it. We set up paid help for two nights a week. The morning after the first night, he slept until ten. He woke up crying. Part of it was relief. Part of it was sadness that he had waited so long to let anyone help.

Needing rest never made him a worse son. But believing he could not have any had almost made him sick.

A composite drawn from years at the bedside. Names and details changed.

The Family Meeting

THIS CHAPTER IN 30 SECONDS

- **The point:** a meeting moves the family from reacting to deciding.
- **The structure:** written facts, the parent in the room, one point person, tasks with names and dates, a next check-in.
- **The script (the Velvet Hammer):** *"Thank you for sharing your thoughts. What part of the plan can you take responsibility for this week?"*

WHAT FAMILIES USUALLY THINK

"We don't want drama." "My brother will never agree." Avoiding the conversation does not remove the problem. A meeting is not about everyone feeling the same way. It is about making the facts visible: what changed, what is unsafe, what the caregiver needs, and who does what next.

When a Family Meeting Is Needed

Hold a family meeting when:

- ☐ A parent has fallen
- ☐ Memory loss is affecting safety
- ☐ Medications are being missed
- ☐ One person is carrying almost everything
- ☐ Hospice or palliative care has been mentioned
- ☐ Living alone or driving may not be safe
- ☐ A hospital or rehab discharge is coming

Do not wait until everyone agrees. The person carrying the most usually sees the need first. If they say, "We need to talk," believe them.

The Conversation Nobody Wants to Start

Most families wait too long to start this conversation — not because they do not care, but because saying "Dad can't keep living alone" out loud feels like a betrayal. So everyone stays quiet, and the risk grows in the silence.

This is a partnership, not a promotion. You are not becoming your parent's parent — nobody wants that job, least of all them. Your parent has run their own life for fifty or sixty years, and the fastest way to end a conversation is to sound like you are taking that away. So ask instead of announce. "What would you want if the stairs got hard?" opens a door. "We think you should move" closes one.

And when a parent pushes back with criticism — of your house, your cooking, your plan — try to hear it for what it usually is: fear wearing an old costume. A person losing control criticizes the things they can still reach. Taking it personally starts a fight; hearing the fear underneath it keeps the conversation alive.

One frame that keeps everyone's dignity intact: your parent stays the **CEO of their own care**, and you are the chief of staff — you gather the options, they make the calls, for as long as they possibly can. And the chief of staff's question is never "Here's what we've decided." It is "*What do you need to feel safe here?*" — then you build around the answer.

An easier way in: borrow someone else's story. "Something happened to my friend's dad — he fell, and nobody knew where anything was. It made me realize we've never talked about this." A third-party story lets everyone look at the problem side-on instead of head-on. Nobody is accused, and nobody has to admit decline. You are not saying something is wrong with Dad — you are saying you don't want to be caught the way your friend was.

One honest sentence opens the door: "I'm worried about Mom, and I don't want to figure this out alone."

Who Should Be There

Invite the people who make care decisions: the parent, the main caregiver, adult children, the healthcare and financial powers of attorney, and any professional already involved — care manager, social worker, clergy, hospice or palliative team, facility representative.

Whenever possible the person receiving care is in the room. They are not a project. A parent who cannot manage every detail can still answer: Where do you feel safest? What are you afraid of? What do you not want?

Separate Roles From Opinions

Every care plan needs one point person — not to do all the work, but to coordinate the medication list, the doctors, the appointments, the documents. Not necessarily the oldest child or the loudest voice.

Everyone has an opinion. Not everyone has a role. "I think Mom should stay home" is an opinion. "I'll take Mom to the doctor Tuesday" is a role. Opinions without roles exhaust the caregiver.

THE VELVET HAMMER

Use this sentence: **"Thank you for sharing your thoughts. What part of the plan can you take responsibility for this week?"** Soft handle, real impact.

The Family Meeting Agenda

1. **What changed?**
2. **What is unsafe today?**
3. **What matters most to the person receiving care?**
4. **What support is needed?** Family help, home health, private duty, adult day care, assisted living, memory care, palliative care, hospice, or respite.
5. **Who is responsible for what?** Names. Dates. Deadlines. No fog.
6. **When will we meet again?**

Ground Rules, Siblings, Distance, and Money

Ground rules: facts not accusations, no interrupting the main caregiver, stay on the next decision, and if you have a strong opinion, take a role.

When siblings disagree, return to facts: "Mom fell Monday and Friday." "Dad missed medication four times this week."

The long-distance family member often underestimates the situation after a cheerful five-minute call. Ask harder questions — Has she fallen? Is the caregiver sleeping? — and offer specific help: bills, scheduling, respite, a weekend.

When money enters the conversation, start with clarity: what income, savings, and insurance exist; whether there is long-term care insurance or VA eligibility; and who has authority to access financial information.

"One parent can take care of eight children, but eight children cannot take care of one parent."

— An old proverb, often quoted by consumer advocate Clark Howard

When One Child Does Everything

One person — the closest, or the daughter, or simply the one who cannot look away — becomes the appointments, the medications, the nights. Because that person keeps managing, the family assumes it is handled. The cost is resentment on one side and guilt on the other.

If you are that one child, you are allowed to say, "I can't keep doing this alone." If you are the sibling at a distance, "Let me know if you need anything" puts the delegating onto the person already drowning. Take something and own it: "I'll handle the medications and the pharmacy." "I'll pay for two mornings of help a week."

DIVIDE BY STRENGTHS, NOT GUILT

Match the work to what each person is good at, not to who feels guiltiest or lives closest. The one calm with numbers takes the bills, insurance, and benefit applications. The one steady in a doctor's office takes the medical calls. Roles assigned by strength get done.

What If No One Agrees?

The meeting will probably get emotional. That does not mean it failed — these conversations involve loss, fear, duty, money, old history, and love. When emotion rises: "What needs to happen in the next seven days?"

Sometimes it ends with the family still split. Two anchors outrank any single vote: **what is safe** and **what the person receiving care actually wants**. A preference to "stay home" does not survive a parent who falls weekly and cannot call for help.

If your parent still has capacity, the choice is theirs. If not, the named healthcare or financial power of attorney carries the decision — that is why those documents exist (Chapters 20 and 22). When the conflict runs deeper than the kitchen table can settle, a neutral third party can break the logjam: a doctor, a social worker, a geriatric care manager, a mediator, or clergy.

WHAT TO DO THIS WEEK

1. **Name the reason** in one sentence: "Mom has fallen twice."
2. **Invite the decision-makers.** Bring written facts — logs, medication lists, discharge papers.
3. **Choose one point person,** assign tasks with names and deadlines, set the next check-in.

■ **Worksheet — The Family Meeting** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

ROB'S NOTE

Family meetings are hard. Families are people, not numbers on a list.

Every family has a past. People fall back into old roles. Old hurts come back up. The one who always fixes things will try to fix it all. The one who hates conflict may go quiet or leave. The person who gives the most care may seem upset. That's because they are the closest to the pain.

That doesn't mean the meeting is a waste. It means the meeting needs a plan. Facts matter. Names matter. Dates matter.

The goal is not to win a fight with your family. The goal is to protect the person who needs care. It's also to keep one person from doing all the work alone. When you feel stuck, ask one simple question: what needs to get done in the next seven days?

CHAPTER 5 TAKEAWAY

A family meeting makes the facts visible and protects both the person receiving care and the caregiver. The next right question is: **who is doing what by when?**

CARDINAL MOMENT



Three grown-up siblings sat in a small room. They would not look at each other. One of them had done everything for two years. The other two lived in far-off cities. They had watched from a distance, and they had opinions about all of it.

We did not fix their past that day. We just made a list — who does what, and by when. The sister who had done it all alone finally took a deep breath. The brother lived four states away and had felt useless. Now he was in charge of the medicine and the pharmacy. For the first time in two years, he felt like a son again, not just someone watching from the side.

The meeting did not turn them into a perfect family. It made them a team for the one job that mattered most. Sometimes that is the most love a family can give at one time. And it is enough.

A composite drawn from years at the bedside. Names and details changed.

There's a story behind this guide. Rob tells it in his memoir, *The Cardinal's Promise*. Read more at thecardinalspromise.com.

Aging Whispers Before It Shouts

Nobody picks the day they become a caregiver.

It starts with a fall. Or a diagnosis. Or a hospital stay. Or a discharge planner on the phone asking where your parent is going next, like you're supposed to already know. One day you're a son or a daughter. The next you're making decisions nobody prepared you for.

I know how that sneaks up on you, because looking back, my parents had been aging for years before I let myself see it.

In 2019, my mother and I were walking through Central Park. She was sixty-eight. She stepped toward a curb and went down. It didn't seem like much at the time — people trip, people fall. We got her up and went on with our day. But it stuck with me. It wasn't that she fell. It was *how* she fell. I remember thinking, my mom is getting old. Then life moved on and I let it. I didn't ask questions. I didn't look closer. I didn't connect anything.

Three years later she was visiting us in Pensacola, standing in our kitchen, and I noticed her hand shaking. She looked down at it and said, almost offhand, "I need to get in with an acupuncturist."

That night I described it to Hope, my wife, a pharmacist. She didn't hesitate. "Rob, your mom has Parkinson's."

I started crying. Not because a doctor had confirmed anything — none had, yet — but because somewhere in me I already knew she was right. A neurologist confirmed it later.

Here's what I understand now: my silence those three years came from love and fear and denial, all tangled together. I wasn't afraid of being wrong. I was afraid of being right. Knowing something and being able to look straight at it are two different things, and when it's your own parent, looking straight at it is the hardest thing there is.

Aging almost never announces itself all at once. It whispers before it shouts. The curb in Central Park was a whisper. The shaking hand in my kitchen was another. I only heard them once I looked back.

The whispers aren't only physical. Consumer advocate Clark Howard has spent years urging families to stay almost nosily present in an aging parent's money — the bills, the accounts, the paperwork — long before anything looks wrong. There's wisdom in that for hearing the whisper: a checkbook often says what a face still hides. A missed payment, a withdrawal your parent can't explain, a scam that finally gets through — these are rarely just money problems. They are often the first quiet word that something is changing.

So hear this before anything else: you're not behind. Nobody teaches this. You're walking into the middle of the storm, same as the rest of us did. You don't have to know everything today. You just have to know the next right step. Keep this book close. It's meant to make sense even on the worst day.

Home Health

Part 2 — Care Options. The question after something changes: what kind of help do we need? These chapters cover home health, private duty care, assisted living, memory care, skilled nursing and rehab, palliative care, and hospice — each the same way: what it is, what it is not, and when it may help.

“If you want to go fast, go alone. If you want to go far, go together.”

— AFRICAN PROVERB

THIS CHAPTER IN 30 SECONDS

- **What it is:** medical care at home — nursing, PT, OT, speech, social work, limited aide visits — ordered by a clinician for people who meet the requirements.
- **What it is not:** all-day supervision, housekeeping, meals, or a sitter — that is private duty care (Chapter 7).
- **When to ask:** after a fall, hospitalization, surgery, new weakness, new wound, or medication change.
- **How to ask:** not “order home health” — “Can we schedule a provider evaluation to determine whether home health is appropriate?”
- **If the answer is no:** ask what criteria are not met and what they recommend instead.

“Home health is like raising your hand in the classroom. It takes humility to ask for help, but those who do often learn faster, recover stronger, and reach their goals sooner. Sometimes, asking for help is the smartest step you can take.”

— Nick Adams, PTA

Follow the Insurance

When my father was referred to home health, a social worker asked what insurance he had. Because he was still working, Medicare was not his primary insurance. That detail mattered.

Plans do not all reimburse providers the same way. I once overheard a provider say of a referral: "I'll take this private-insurance patient if you send me your next two Medicare patients." Behind every referral is a business reality.

Do not become cynical; become informed. Why was this agency selected? Does it accept my insurance? Do I have a choice?

WHAT FAMILIES USUALLY THINK

Families often think home health means someone will stay with Mom, that Medicare will send an aide every day, or that the doctor can order it over the phone. They imagine a safety net large enough to catch everything. It is a specific service — valuable, but not every kind of help.

What Home Health May Include

Skilled nursing — medication and disease teaching, wound care, symptom monitoring, caregiver instruction, doctor communication.

Physical therapy — strength, balance, walking, transfers, fall risk, stairs, safe walker or cane use.

Occupational therapy — bathing, dressing, toileting, kitchen safety, equipment, home setup.

Speech therapy — swallowing, communication, cognition, memory strategies.

Medical social work — community resources, long-term planning, caregiver stress, advance care planning.

Home health aide — limited personal care, such as bathing help, when it is part of the skilled plan of care. Visits are limited — not daily private duty caregiving.

What Home Health Usually Does Not Provide

Home health usually does not provide all-day supervision, overnight or long shifts, housekeeping, meals, transportation, companionship, routine medication reminders

without a skilled need, ongoing bathing help as the only need, or twenty-four-hour care. It is not meant to replace family care, private duty care, assisted living, memory care, or long-term supervision — it is rarely the whole plan.

HOME HEALTH	PRIVATE DUTY CARE
Medical, intermittent, clinician-ordered	Non-medical daily help, often paid privately
Nursing, therapy, social work, limited aide	Bathing, meals, companionship, supervision
After illness, wounds, weakness, or decline	When the person cannot safely be alone
Not long blocks of supervision	Can cover longer blocks and overnight

When Home Health May Help

Ask about a home health evaluation after:

- ☐ A fall
- ☐ New weakness or trouble walking
- ☐ Surgery, hospitalization, or rehab discharge
- ☐ A new wound
- ☐ Medication changes or a new diagnosis
- ☐ Shortness of breath or new confusion with a medical concern
- ☐ A decline in bathing, dressing, or transfers

Home Health Requires Evaluation and Documentation

HOW TO ASK, THE RIGHT WAY

For Medicare-covered home health, a clinician must determine whether the person meets the requirements — a skilled need, homebound status, and the required face-to-face encounter documentation. The better request is not "Can you just order home health?" It is: **"Can we schedule a provider evaluation to discuss this recent change and determine whether home health is appropriate?"**

What "Homebound" Means in Plain English

Families hear "homebound" and think it means never leaving the house. In plain language, it means leaving home is difficult, taxing, unsafe, or requires help, equipment, or considerable effort. A person may still leave for medical appointments and be considered

homebound if they otherwise meet the requirements. Describe what is happening and let the clinician evaluate it.

When Home Health Is Not Enough — or Not Ordered

Home health alone may not be enough when the person cannot safely be alone, dementia causes wandering, medications require daily supervision, the caregiver cannot sleep, or falls continue despite therapy. The need may then be private duty care, adult day care, assisted living, memory care, skilled nursing, or hospice — not a failure of home health, but a need bigger than intermittent skilled visits.

If a clinician says home health is not appropriate — no skilled need, not homebound, or a different kind of care — ask: "What criteria are not being met?" and "What support do you recommend instead?" The answer should point somewhere: outpatient therapy, a medication review, a specialist, a care manager, or a palliative or hospice evaluation.

WHAT TO DO THIS WEEK

1. **Write down what changed** — facts: "Mom fell twice in two weeks. He gets short of breath walking to the bathroom."
2. **Decide whether there may be a skilled need** — fall risk, weakness, wounds, therapy, teaching, home safety.
3. **Ask for a provider evaluation** — not "order home health."
4. **Bring your notes** — fall log, medication list, discharge papers, symptoms, safety concerns.
5. **Make the home ready** — clear walkways, gather medications, show the clinician the bathroom, stairs, and walking paths.

■ **Worksheet — Home Health Evaluation** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE PROVIDER

1. Does my parent meet criteria for home health, and is there a skilled need?
2. Is my parent considered homebound, and was the required face-to-face encounter completed or scheduled?
3. Would nursing, physical therapy, occupational therapy, speech therapy, or social work help?
4. Would a home health aide be part of the care plan?
5. What documentation is needed?

6. What should we do if home health is not appropriate — what criteria are not being met?
7. What support is needed beyond home health?

■ HOME HEALTH SCRIPT

Calling the provider's office: "My parent has had a recent change in function and safety. We are seeing concerns with _____. Can we schedule a provider evaluation to determine whether home health is appropriate and whether the required documentation can be completed?"

ROB'S NOTE

Home health can be one of the best services a family gets. But it only helps when the family knows what it is.

I have seen families feel let down. They thought someone would stay for hours, cook meals, watch over a person with dementia, and take over the whole day. That is not what home health does.

I have also seen families miss how much home health can help. They didn't know how much a nurse, therapist, or social worker could see once inside the home. The doctor's office sees a patient sit in a chair for fifteen minutes. Home health sees more. It sees the narrow hallway. The bathroom with no grab bars. The pill bottles on the counter. A caregiver lifting the wrong way. Stairs that are now too hard to climb.

A home can look fine at first. But it can be unsafe for someone who is weak, confused, short of breath, or healing. Home health is not the whole answer. But it can be the first trained person to spot the problem. That helps a family figure out what to do next.

CHAPTER 6 TAKEAWAY

Home health is medical care in the home for people who meet the requirements. The next question: **can we schedule a provider evaluation to determine whether home health is appropriate, and what support is needed if it is not?**

CARDINAL MOMENT



A daughter thought she had just two choices. She could quit her job to care for her dad. Or she could move him into a place he did not want to go. In her head, she was already writing her goodbye letter to work. No one had told her something important. After his time in the hospital, Medicare would send a nurse and a physical therapist to his house for a while. And it would cost them nothing.

Home health gave her six weeks. It was not forever, and it was never meant to be. But it gave her time. She got to keep her paycheck. And her dad got to stay in his own kitchen a little longer.

Sometimes the help you need most is the help no one thinks to mention. So ask the hospital in plain words: "Does he qualify for home health?" Make them answer before he goes home.

A composite drawn from years at the bedside. Names and details changed.

Private Duty Care

THIS CHAPTER IN 30 SECONDS

- **What it is:** non-medical help in the home — bathing, dressing, meals, housekeeping, companionship, transportation, errands.
- **What it is not:** medical care. Home health is medical and intermittent, and requires eligibility (Chapter 6). Some families need both.
- **Who pays:** usually private pay; long-term care insurance, VA benefits, or certain Medicaid programs may help. Medicare generally does not pay for long-term custodial care.
- **How to introduce it:** time-limited — *"Let's try it for two weeks and see what helps."*
- **The real question:** what tasks are no longer safe or realistic for the family to handle alone?

WHAT FAMILIES USUALLY THINK

"We're not ready for strangers in the house." "Medicare should cover this." But help at home is not failure. Sometimes it is what makes staying home possible.

The Yardstick: Activities of Daily Living

Professionals measure need using "activities of daily living" (ADLs) — bathing, dressing, eating, toileting, transferring in and out of a bed or chair, continence — and "instrumental" activities (IADLs): medications, money, cooking, shopping, the phone, getting around. Decline shows up first in the instrumental tasks; a steady rise in help needed signals a higher level of care may be coming. Help only as much as needed — doing too much accelerates decline and wounds dignity.

Private Duty Care vs. Home Health

Families mix these up (Chapter 6 draws the full distinction); a family may need both.

HOME HEALTH	PRIVATE DUTY CARE
Medical and intermittent	Non-medical daily help and supervision

Nursing, PT, OT, speech, social work, limited aide	Bathing, meals, errands, companionship, overnight care
Requires eligibility and clinical documentation	Often private pay; can be scheduled for longer blocks
Not long blocks of daily supervision	Can cover mornings, evenings, overnight, and respite

Medication reminders depend on state rules and agency policy. Private duty care does not include skilled medical care unless the agency provides licensed nursing: many caregivers cannot give injections, manage complex wounds, or adjust medications. Ask directly.

When Private Duty Care May Help

It may help when the person needs help with bathing, dressing, or meals; needs supervision or transportation; is at risk for falls; has dementia or wanders; forgets medications; needs overnight support; is coming home from a hospital stay; or has a caregiver who needs relief.

Families often underestimate, asking "Can someone come once a week?" when the real need is daily. Match the schedule to the risk — morning (bathing, dressing, medications), midday (missed meals, wandering, falls), evening (confusion, unsafe cooking), and overnight (getting up, toileting).

Cost and Who Pays

Private duty care gets expensive. Ask about:

- ☐ the hourly rate
- ☐ minimum shift length
- ☐ weekend and holiday rates
- ☐ overnight billing
- ☐ deposits and fees
- ☐ cancellation policy
- ☐ what happens if a caregiver calls out

It is often paid privately — savings, retirement income, family contributions, long-term care insurance, certain VA benefits, or some Medicaid waiver or state programs depending on eligibility. **Medicare generally does not pay for long-term custodial private duty care**, so do not assume it will cover daily supervision, bathing, and meals.

Choosing an Agency — or Hiring Privately

A good agency is more than a warm body in the house. Ask about:

- ☐ licensing
- ☐ background checks
- ☐ training
- ☐ dementia and fall-prevention experience
- ☐ caregiver supervision
- ☐ backup coverage
- ☐ care plans
- ☐ documentation
- ☐ medication and transportation policies
- ☐ minimum hours
- ☐ rates
- ☐ how caregiver matching and replacement work

The cheapest option is not always the safest.

Hiring privately may lower cost and add flexibility, but it also means no backup if the caregiver is sick, payroll and tax responsibilities, liability concerns, and less supervision. When in doubt, ask an elder law attorney, accountant, or care manager what risks to consider.

Introducing Help When a Parent Refuses

Many parents refuse at first — "I don't need a babysitter." Do not lead with the word "caregiver" if it creates resistance. Try "Someone is coming Tuesday to help with lunch and laundry," or "Let's try it for two weeks." The first goal may not be acceptance. It may be getting through the door.

When Private Duty Care Is Not Enough

It may not be enough when the person needs two-person transfers, is unsafe even with scheduled visits, has severe dementia behaviors or wandering, needs twenty-four-hour supervision that is not financially sustainable at home, or when the caregiver cannot continue. Then consider assisted living, memory care, skilled nursing, palliative care, or hospice.

WHAT TO DO THIS WEEK

1. **List the daily tasks that are no longer safe** — bathing, meals, supervision, overnight safety, respite.
2. **Identify the highest-risk times of day.**
3. **Decide how many hours to try first.**

4. **Call at least three agencies** and compare rates, minimums, training, and backup coverage.
5. **Decide who will manage the schedule** and name a backup.
6. **Review after two weeks** — safer parent? rested caregiver? more or fewer hours?

■ **Worksheet — Private Duty Care Planning** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK A PRIVATE DUTY AGENCY

1. Are you licensed or registered as required in this state, and are caregivers employees or contractors?
2. Do you perform background checks, and what training do caregivers receive (including dementia)?
3. Can caregivers help with bathing, toileting, transportation, and medication reminders — and what are they not allowed to do?
4. What is the minimum shift length, the hourly rate, and are nights, weekends, or holidays billed differently?
5. What happens if the caregiver calls out, and can we request a different caregiver if needed?
6. Who supervises the caregiver, and how do you communicate with the family?
7. Do you help with long-term care insurance paperwork, and how quickly can care start?

■ HOME SETUP NOTES FOR THE CAREGIVER

Emergency contact _____

Primary doctor / preferred hospital _____

Pharmacy _____

Allergies _____

Mobility & bathroom safety notes _____

Meal preferences _____

Dementia / behavior notes _____

Things that calm the person _____

Things that upset the person _____

House rules _____

First-week care plan

TASK	PERSON RESPONSIBLE	DATE
List care needs & call agencies		
Compare costs & review budget		
Choose agency & schedule first shift		
Prepare home notes & update family		
Review after first week		

Two-week review

Is the current plan working? Yes ☐ No ☐ Somewhat ☐ Unsure ☐

What needs to change?

- | | |
|--|---|
| ☐ More hours | ☐ More family help or supervision |
| ☐ Fewer hours | ☐ Assisted living should be explored |
| ☐ Different caregiver or agency | ☐ Memory care should be explored |

Final decision — agency chosen & start date _____

Schedule _____

Main goal of private duty care _____

Next review date _____

■ INTRODUCTION SCRIPT — WHEN A PARENT RESISTS

"We are going to try having someone come in to help with _____. This is not about taking over. It is about making the day easier and safer. We'll try it for _____ and then see what is working."

If they say, "I don't need help," try: "I understand. This is also to help me — I need some support so I can keep doing this well."

If they say, "I don't want a stranger," try: "That makes sense. We'll meet them together, and if it's not a good fit, we'll speak up."

ROB'S NOTE

Private duty care can be hard for families. It can feel like letting a stranger in too close.

Before this, the family may have done all the care. Then all at once, someone from outside is helping. They help with showers, meals, laundry, and the private parts of daily life. That can feel strange.

But I have seen private duty care give families a break. The daughter who never sleeps gets a morning off. The husband or wife who is scared to leave can go to the doctor. The parent who kept skipping meals starts eating lunch again.

It is not always perfect. The first caregiver may not be the right fit. The cost may be more than you thought. But asking for help is not a failure to love. It can keep love from turning into burnout. The right question is not, "Should we be able to do this on our own?" It is, "What help would make this safer for everyone?"

CHAPTER 7 TAKEAWAY

Private duty care is daily, non-medical support in the home. The next right question is: **what tasks are no longer safe or realistic for the family to carry alone?**

CARDINAL MOMENT



A family hired four hours of help each day. They felt a quiet shame about it. They thought their love should have been enough by itself. For weeks, they kept saying sorry to each other for needing help.

The aide's name was Denise. She learned that their father liked his coffee with two sugars. She learned he liked the ballgame on low. Within a month, he called her by name. He even told her the same three stories he only told people he trusted.

Bringing in a stranger did not replace the family. It gave the family back to him. Now they were rested. Now they were glad to see him again, instead of worn out by him.

A composite drawn from years at the bedside. Names and details changed.

Assisted Living

THIS CHAPTER IN 30 SECONDS

- **What it is:** residential care — meals, medications, bathing, safety, staff around the clock. Not a nursing home; many have a memory care wing (Chapter 9).
- **Who pays:** almost never Medicare. Generally private pay; long-term care insurance, VA benefits, and some state programs can help (Part 3).
- **The question that decides it:** "Who is with them at the key times?"
- **The money trap:** the care level cost, not the base rent — get it in writing before you fall in love with a place.

Margaret's daughter didn't decide anything the day of the fall. She decided three weeks later, standing in her mother's kitchen at 7 a.m., counting pill bottles and realizing she'd driven over before work every day that month. Her mother hadn't eaten a real dinner in days. The neighbor had found her outside at dusk, looking for a dog that died in 2019. The daughter wasn't failing. She was doing the work of three people, and there was still only one of her. *(A composite, drawn from years around senior living — details changed.)*

That morning is what this chapter is for. Not the fall. The kitchen, three weeks after.

What You Need to Understand

Assisted living is a residential care setting — not a nursing home, not home health. Meals, medications, bathing, laundry, safety checks, staff around the clock. Many communities also have a separate, secured memory care wing if dementia is part of the picture (Chapter 9).

Two beliefs stop families at the door, and both are usually wrong. The first is that a move means giving up. For many families it means more safety, more connection, staff at 2 a.m. — and it gives the family back its relationship. The daughter gets to go back to being a daughter instead of a full-time nurse.

The second is that Medicare pays for it. It almost never does. Assisted living is generally private pay, though long-term care insurance, certain VA benefits, and some state programs can help (Part 3 shows you how families actually pay).

The question that tells you whether it's time isn't on any brochure: **“Who is with them at the key times?”** If your family can't answer that, the answer is the conversation.

What Families Commonly Miss

Do your research. The tour shows you the lobby — but the staffing, the medication process, and the fall response are what you're actually buying, and none of them are on the tour unless you ask.

And the price on the website is not the price. The base rent is where the cost *starts*. Care is priced in levels on top of it, and the level can change after move-in — usually at the first care reassessment.

\$ THIS COULD SAVE YOU THOUSANDS

The base rent gets all the attention. The **care level cost** is what breaks budgets. Before you sign anything, ask three questions and get the answers in writing: What level is she today, and what does it cost? What would move her to the next level? What does *that* cost? A community quoting \$4,200 a month at “Level 1” can honestly become \$5,600 at Level 3 — and the family that never asked finds out at the reassessment, after the furniture is moved in. Get the numbers in writing before you fall in love with a place.

The Solution

Work in this order, and the decision gets simpler.

Needs first, tours second. Be honest about bathing, transfers, medication, nighttime, memory. Assisted living meets many needs, not all — two-person transfers, frequent wandering, or complex medical care may mean memory care or skilled nursing instead. Ask each community what they *cannot* manage and what would force a move-out later. A community that answers that honestly is telling you something good about itself.

Tour three if you can, with your eyes open. Watch how staff treat residents. Ask to see care areas, not just the dining room. Notice whether call buttons get answered. Red flags: vague answers about staffing or cost, pressure to sign quickly. Green flags: staff who greet residents by name, costs explained in writing, honest talk about limits.

BRING THIS ON THE TOUR

Ask: What levels of care do you provide, and how do you decide what someone needs? · How often are needs reassessed, and what happens if they increase? · What would require a move-out or transfer? · Can you support bathing, toileting, incontinence, and two-person transfers? · How many caregivers are on days, evenings, and overnight? · Is a nurse on site or on call, and who manages medications? · How quickly are call buttons answered? · How are falls handled and reported to family? · What is the base rent, what is included, what costs extra, and how often can rates increase? · Who is the family contact, and when do you call the family vs. 911?

Red flags: rushed or dismissive staff · residents ignored · strong odors · vague answers on staffing or cost · pressure to sign quickly. **Green flags:** staff greet residents by name · costs in writing · honest talk about limits · a clear family-communication process.

Then bring it to the family (Chapter 5), compare in writing, and decide the next step — not the whole future.

WHAT NOBODY TELLS YOU

Tour at mealtime or shift change — that's when you see the real staffing, not the brochure's version. And more is negotiable than families assume: the one-time community fee and the first months' rate often move, especially when a building has open apartments. You will never get a discount you didn't ask about.

ROB'S NOTE

Families often tour a senior community and love the fancy lobby.

But your parent will not live in the lobby. They will live in the hallway, the dining room, and the shower. They will live in the moment they press a call button at 2 a.m.

So watch the hallway. Watch how the staff talk to people in wheelchairs. See how long the call light stays on before someone comes. Ask the hard questions about falls, medicine, and what happens when your parent needs more care. Then notice how they answer. Do they tell you the truth? Or do they change the subject back to the nice features?

The best communities will tell you what they cannot do. That kind of honesty is worth more than any fancy lobby. You are not just picking a building. You are picking the people who will be there for your parent when you can't be in the room.

WHO TO CALL

- **Not sure what level of care fits** — start with your Area Agency on Aging (Eldercare Locator, 1-800-677-1116), or a local placement advisor. Ask any advisor one question first: “*Who pays you?*” Most are paid by the communities they refer families to. That is not disqualifying, but you should know it before you take anyone’s shortlist. **Cardinal Care Bridge** at TheCardinalsPromise.com is the care-navigation service I run — free to families, and you should ask me the same question.
- **Paying for it** — an elder law attorney (protecting assets, Medicaid timelines — Chapter 14); your SHIP office for insurance questions (Chapter 13)
- **Veteran or surviving spouse** — a Veterans Service Officer about Aid & Attendance (Chapter 18)
- **Suspect it’s more than assisted living can handle** — ask the doctor for a care-needs evaluation before you tour

Need the comparison sheet? The one-page tour worksheet — five questions, three communities, the care-level trap built in — is in **The Workbook** at the back of this book, p. 209 — printable copies at TheCardinalsPromise.com/resources.

WHAT TO DO THIS WEEK

1. **Answer one question honestly tonight:** who is with them at the key times — mornings, medications, nights? If the answer is “no one” or “only me,” that is your answer.
2. **Write down what is no longer safe** — bathing, nights, medications, meals, leaving the house.
3. **Ask the doctor for a care-needs evaluation** before you tour anything.
4. **Call one community and book a tour.** Not a commitment. A look.
5. **Get the care-level pricing in writing** before a second visit.

**KEEP GOING**

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CHAPTER 8 TAKEAWAY

Assisted living is residential care — meals, medications, bathing, and staff in the building around the clock — and Medicare almost never pays for it. The next right question is: **who is with them at the key times, and what does the price become when the care level changes?**

CARDINAL MOMENT



For years, a mother swore she would never go into "one of those places." Her daughter went and toured seven of them anyway, quietly. She kept looking until she found one that did not smell like anything bad. It even had a piano that someone actually sat down and played.

Her mother cried on move-in day. Her daughter drove home sure she had made a terrible mistake. A month later, her mother cried again. But this time it was because she had made a friend at dinner. She had not expected to have a life again.

The fear is almost always about the idea, not the real place. So go tour a real one. The right building is not an ending. Sometimes it is the first company she has had in years.

A composite drawn from years at the bedside. Names and details changed.

Memory Care

THIS CHAPTER IN 30 SECONDS

- **What it is:** a residential setting for people with dementia or Alzheimer's who need more structure, supervision, and safety than assisted living provides.
- **What it is not:** a punishment, "locking someone away," or giving up.
- **When to consider it:** a pattern of risk, not one bad day. The one question: is this person unsafe because they cannot remember how to stay safe?
- **How to say it:** not "We have to put you in memory care" but *"a place with more support and a safer routine."*

WHAT FAMILIES USUALLY THINK

"Mom is not that bad." "Dad still knows who I am." But a person can recognize loved ones and still be unsafe with medication, cooking, wandering, or nighttime confusion. The question is whether the current setting can support them when clarity disappears.

Memory Care vs. Assisted Living

The two may share a building, but not a level of support. Assisted living may work when a person needs help with meals, bathing, dressing, and medications but is still safe in a less secure setting. Memory care adds what dementia risk requires: secured or monitored exits, dementia-trained staff, more frequent cueing, support for wandering, and calm redirection for agitation. Ask one question: **is this person unsafe because they cannot remember how to stay safe?**

Consider memory care when the person:

- ☐ wanders or tries to leave
- ☐ gets lost
- ☐ is unsafe with the stove or medication
- ☐ falls or forgets the walker
- ☐ cannot reliably call for help
- ☐ is confused or frightened at night
- ☐ becomes suspicious or agitated

- ☐ refuses bathing or medication
- ☐ needs supervision throughout the day
- ☐ or when home care is no longer enough and the caregiver cannot safely continue

Write down the pattern.

The Wandering Risk

Wandering is one of the clearest reasons to consider memory care. It rarely looks dramatic at first:

- ☐ walking outside "just for a minute"
- ☐ trying to go home while already home
- ☐ leaving at night
- ☐ looking for a deceased spouse
- ☐ exiting during appointments
- ☐ becoming restless in the evening

A person who wanders may not understand danger, and instructions may not be remembered ten minutes later. Ask whether they can safely be alone, whether the caregiver can sleep, and whether a secured setting is needed.

Nighttime, Behavior, and Caregiver Collapse

Nighttime often reveals how unsafe the plan has become — getting up repeatedly, falling on the way to the bathroom, or the caregiver no longer sleeping. Sudden behavior changes can have treatable causes — pain, infection, constipation, dehydration, poor sleep — so report them to a clinician. And memory care is sometimes needed not because the person is declining but because the caregiver is. A plan depends on one exhausted caregiver staying awake forever is not a plan. It is a fuse.

How to Talk About It

Instead of "You cannot live alone anymore," try "We are worried the current setup is not keeping you safe." One conversation may not settle it — expect repetition and reassurance. The goal is care, not winning the argument.

Tour Beyond the Locked Door

A secured door is the easy part. What matters is the care inside it. On a tour, ask how they manage wandering and exit-seeking, agitation and refusal of care, medications, falls, and family updates, and what would require a transfer out. Watch how staff speak to residents:

do they rush, correct harshly, or redirect calmly? Green flags include warm, name-by-name greetings, a calm environment, a safe supervised outdoor space, dementia-adapted activities, and staff who can explain how they handle the hard days. A beautiful building does not guarantee good dementia care. The care culture does.

WHAT TO DO THIS WEEK

1. **Write down the pattern of safety risks.**
2. **Ask whether assisted living is enough** or whether the risks need a secured setting.
3. **Tour more than one community** and ask about the hard days, not the lobby.
4. **Talk with the doctor** about sudden changes.
5. **Hold a family meeting** (Chapter 5) on safety, caregiver capacity, and cost.
6. **Decide the next step** — more supervision, private duty, adult day care, assisted living, or memory care.

■ **Worksheet — Memory Care Decision** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK MEMORY CARE

1. How do you manage wandering or exit-seeking, and are exits secured or monitored?
2. What dementia training do staff receive, and how many are on during the day, evening, and overnight?
3. How do you handle agitation, fear, aggression, or refusal of bathing and medication — what non-drug approaches do you try first, and what is your approach to antipsychotic medications?
4. How are medications and falls managed, and how quickly is family notified of a change?
5. How are activities adapted for different stages of dementia?
6. How do you learn the resident's routines, history, triggers, and preferences?
7. What happens if care needs increase, and what would require a transfer or discharge?

Decision summary

Main reasons memory care may be needed _____

Main concerns _____

Questions still unanswered _____

Next step / person responsible / due date _____

■ CONVERSATION SCRIPT

With family: "We need to talk about memory care not because we are giving up, but because dementia is creating safety concerns. The question is whether the current setting can still protect safety, dignity, and the caregiver."

With the person, if appropriate: "We are looking at a place with more support and a safer routine. The goal is not to take your life away. It is to make sure your days are safer, calmer, and better supported."

ROB'S NOTE

Memory care is one of the hardest talks a family can have. It feels like a door that only opens one way.

Families often wait, because they want to respect the person they love. I get that. But dementia changes the rules bit by bit. At first, the family helps with small things. Then they manage the pills. Then they hide the car keys. Then they sleep with one ear open. Then they are scared to shower, because the person might walk out the door.

No one planned for this. Memory care should never be treated lightly. But it is not a failure either.

Sometimes the most loving thing a family can say is, "This is bigger than what we can safely handle at home." That sentence hurts. It can also keep everyone safe. The goal is not to save the old plan. The goal is to care for the person in front of you right now.

CHAPTER 9 TAKEAWAY

Memory care may be needed when dementia creates safety risks that cannot be managed at home or in assisted living. The next right question: **is the current setting safe during the moments when memory cannot protect them?**

CARDINAL MOMENT



A husband kept his wife at home two years longer than was safe. Moving her felt like breaking the promise he had made to her. He propped chairs against the doors and slept in his clothes.

One night in February, she got out anyway. He found her shaking on the neighbor's steps. Something in him finally let go. Memory care did the one thing his love could not do alone. It locked the doors, so he could go back to being her husband instead of her guard.

Choosing more help is not choosing less love. Sometimes it is the most loving thing left to do.

A composite drawn from years at the bedside. Names and details changed.

Skilled Nursing and Rehab

THIS CHAPTER IN 30 SECONDS

- **What it is:** short-term medical care, nursing, and therapy after a hospital stay when someone is not ready to go home safely — a decision point, not just a place to "get stronger."
- **Inpatient vs. observation:** ask whether the hospital stay was inpatient or observation — it can decide Medicare coverage (Chapter 13).
- **The money clock:** with a qualifying stay, Medicare pays on a clock:
 - **Days 1–20:** Medicare pays in full.
 - **Days 21–100:** a daily coinsurance (\$217 in 2026 — confirm at [medicare.gov](https://www.medicare.gov)).
 - **After day 100:** Medicare pays nothing.

Start the "what happens after" conversation early (Chapters 13–15).

- **The "plateaued" trap:** under the Jimmo settlement, coverage does not legally require the person to keep improving. Ask for the written notice and how to appeal.
- **The fast appeal:** if coverage or the stay is ending and it is not safe, Medicare patients have a formal fast-appeal right through the Quality Improvement Organization (QIO) — the number is on the written notice and the clock is short, so act the same day. No notice? Find your QIO at qioprogram.org ("Locate Your QIO") or call 1-800-MEDICARE.

WHAT FAMILIES USUALLY THINK

Families often think, "Rehab will get Mom back to normal," "Dad will stay until he is ready," or "They cannot discharge her if we are not ready." Sometimes rehab helps. But older adults lose strength quickly, and a person may improve and still need more help than before. Rehab answers important questions — but not always the way the family hoped.

What Rehab May Provide — and What It Does Not Mean

A skilled nursing or rehab facility may provide:

- nursing care, medication management, and wound care
- physical, occupational, and speech therapy
- help with bathing, dressing, toileting, and transfers

- discharge planning, social work, and caregiver training
- equipment recommendations

Ask what is actually being provided — do not assume.

Rehab does not guarantee the person returns home independent, or daily progress, a long stay, or that insurance keeps paying. It does not solve dementia, frailty, advanced illness, caregiver burnout, or unsafe housing.

Short-Term Rehab vs. Long-Term Care

Short-term rehab is focused on recovery after a hospital stay, surgery, illness, or fall, and may be covered by insurance if requirements are met. One protection to know: **Medicare coverage does not legally require the person to keep improving.** Under the *Jimmo* settlement, skilled care to maintain function or to prevent or slow decline also qualifies. If you are told coverage is ending because the person has "plateaued," ask for the written notice and ask how to appeal. Long-term care is ongoing custodial care for someone who cannot safely live alone; it is paid differently, and **Medicare generally does not pay for long-term custodial nursing home care.**

Two more protections. Ask whether the hospital stay was **inpatient or observation** — it can decide Medicare coverage (Chapter 13 explains the trapdoor). And if coverage or the stay is ending and it is not safe, use the Medicare fast appeal through your Quality Improvement Organization (QIO); the written notice the facility hands you carries the phone number, and the clock is short.

Therapy and Nursing: What to Ask

Ask the therapy team how far the person can walk and with how much help — independent, supervision, or hands-on; whether they need a walker, cane, or wheelchair; whether they can get out of bed, get on and off the toilet, manage stairs, and bathe and dress safely; and what would make home unsafe. Ask for plain language — not "She is min assist," but **"What does that mean at home at 2 a.m. when one person is helping?"**

Ask the nursing team what medical needs are being managed, whether medications are stable, whether there is wound care, pain, confusion, incontinence, poor eating, or shortness of breath, and what must be watched after discharge.

Social Work, Discharge Planning, and the Care Conference

Rehab is a handoff, not a holding place — discharge planning begins almost immediately. Use the stay to learn what the person can do now and whether home, home health, private

duty care, assisted living, memory care, or hospice fits best. The discharge planner or social worker is your key contact. Ask about:

- ☐ the expected discharge date
- ☐ the recommended next level of care
- ☐ what equipment is needed
- ☐ whether home health or private duty care will be arranged
- ☐ what the options are if home is not safe — including what happens if the family disagrees with the discharge plan

Do not wait — ask for a care conference.

A care conference brings the rehab team — nursing, therapy, social work, sometimes a dietitian — together with the family. Ask for one early. Use it to learn what has and has not improved, what help the person still needs, the discharge goal and date, what setting is safest next, and what training the family needs. It should produce a plan, not just reassurance.

When Going Home May Not Be Safe

Going home may not be safe if the person cannot transfer safely or get to the bathroom, is at high risk for falls, needs more help than one caregiver can provide, has dementia that creates supervision needs no one can meet at home, needs skilled care for wounds or medications the family cannot manage, is not eating or drinking safely, or has a caregiver who cannot safely continue. If home is not safe, that is not a failure — it is information: the next setting has to match the care the person needs now.

WHAT TO DO THIS WEEK

1. **Ask whether this is short-term rehab or long-term care** and how it is paid for.
2. **Request a care conference early** — do not wait for the discharge call.
3. **Get plain-language answers** from therapy and nursing about walking, transfers, toileting, bathing, and what home would require.
4. **Ask what happens after discharge** — home health, private duty care, and equipment.
5. **Compare the settings honestly** — home, assisted living, memory care, or long-term care.
6. **Hold a family meeting** (Chapter 5) before discharge, not after.

■ **Worksheet — Rehab & Discharge Planning** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK BEFORE DISCHARGE

1. Is this stay short-term rehab or long-term care, and how is it paid for?
2. Can my parent transfer, toilet, walk, and bathe safely enough for the planned setting?
3. What would make home unsafe, and what would we need to make it work?
4. Will home health be evaluated, and what skilled needs qualify?
5. What medications, wounds, or conditions must be monitored after discharge?
6. What should we watch for, and who do we call if things get worse?
7. What happens if we do not feel the discharge is safe?

ROB'S NOTE

Rehab is one of the most misread moments in the whole journey.

Families hear the word "rehab" and picture their mother walking back through the front door the way she left. Sometimes that happens. Often it does not.

I have sat with families on day three of a rehab stay who were already being asked about going home. They had not even caught their breath from the hospital. That is why I tell people: rehab is not a pause. It is a countdown. Ask for the care meeting early. Ask the therapists what "min assist" means at 2 a.m. Ask the nurses what the nights are like.

The goal is not to rush the person home to prove the crisis is over. The goal is to send them to a place that can actually keep them safe. Sometimes that is home with help. Sometimes it is not. Either way, you want to be the family that asked the hard questions early. You do not want to be the one handed a go-home date with nowhere safe to go.

CHAPTER 10 TAKEAWAY

Rehab is a decision point, not just a place to get stronger. The next right question: **what level of care will actually be safe after rehab ends?**

CARDINAL MOMENT



A man went to rehab after his stroke "for a week or two." The whole family thought he would be home by the end of the month. But progress came slower than the movies had promised. Everyone, including him, started to lose hope.

What turned it around was not pushing harder. It was showing up. They sat in on therapy. They learned the exercises. They cheered the small wins out loud: a spoon lifted, a word that came back, a step without the rail.

Rehab is not a machine you drop someone into and pick them up from later. It works best when the family is in the room, counting each inch right beside them.

A composite drawn from years at the bedside. Names and details changed.

Palliative Care

THIS CHAPTER IN 30 SECONDS

- **What it is:** specialized support for serious illness — symptoms, stress, goals of care, and quality of life — that can often begin earlier and be used alongside treatment.
- **What it is not:** not hospice, not stopping treatment, not a signal that death is close.
- **How to ask:** *"Would palliative care be appropriate to help with comfort, quality of life, goals of care, and family support?"*

WHAT FAMILIES USUALLY THINK

Families hear "palliative care" and think it means hospice, that death is close, that the doctor is giving up. That confusion makes families turn down help they could use. It is an extra layer of support for serious illness — someone to ask: What is the goal now? What matters most if the illness gets worse?

What Palliative Care May Provide

Services vary by setting and program, but may include:

- symptom management (pain, shortness of breath, nausea, fatigue)
- emotional, social work, and spiritual support
- goals-of-care conversations, family meetings, and advance care planning
- medication review and care planning
- help understanding treatment options and coordination with other doctors

It may be provided in hospitals, clinics, nursing facilities, assisted living communities, or at home — ask whether a home-based program serves your area.

Palliative Care vs. Hospice

Palliative care does not mean the person must stop treatment, or that hospice is starting. Hospice is generally for people nearing the end of life who are no longer pursuing curative treatment for the terminal condition. Both care about comfort, dignity, and support — what differs is timing and eligibility.

PALLIATIVE CARE	HOSPICE
Can often begin earlier in a serious illness	For those in the last stage who meet eligibility
May be used alongside treatment	Comfort-focused when cure is no longer the goal
Focuses on symptoms, stress, quality of life, goals	A full team and a specific benefit structure

IF THE FAMILY IS UNSURE

Ask: **"Are we talking about palliative care for serious illness support, or hospice care because the illness may be in its final stage?"** That question can clear the room.

When Palliative Care May Help

Consider asking about palliative care with:

- ☐ advanced heart, lung, kidney, or liver disease
- ☐ cancer, neurologic illness, or dementia with increasing complications
- ☐ repeated hospitalizations or ER visits
- ☐ uncontrolled pain or shortness of breath, weight loss, or declining function
- ☐ difficult treatment decisions, family disagreement about goals, or confusion about prognosis
- ☐ treatments becoming harder than the illness

It may be especially helpful when everyone is working hard but no one is asking what the patient actually wants most.

The Goals-of-Care Conversation

A goals-of-care conversation is not only about medical treatment — it is about what kind of life the person wants as illness changes. What is the person hoping for, and what are they afraid of? If time is shorter than the family hoped, what would make the days better?

WHAT TO DO THIS WEEK

1. **Write down current symptoms** and what helps or worsens each.
2. **List hospital and ER visits** in the last six months and what keeps sending you back.
3. **Ask about palliative care** using the script in this chapter, and clarify whether palliative or hospice is being recommended, and why.
4. **Review advance care documents** — power of attorney, advance directive, DNR/POLST if applicable.

5. Invite key family members in, and hold a family meeting if needed.

■ **Worksheet — Palliative Care Conversation** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK THE PALLIATIVE CARE TEAM

1. What symptoms can you help manage, and how can we improve comfort and quality of life?
2. What should we understand about this illness, and what changes should we expect next?
3. Are current treatments helping, or becoming too burdensome?
4. How can we reduce unnecessary crisis trips to the hospital, and who do we call after hours?
5. What support is available for the caregiver, and can you help with advance care planning or a family meeting?
6. Is hospice appropriate now, or should we continue palliative care — and what would tell us it is time to consider hospice?

Advance care planning — documents to find or update

- | | |
|---|--|
| ☐ Healthcare Power of Attorney | ☐ Medication list & doctor list |
| ☐ Living Will / Advance Directive | ☐ Emergency contacts |
| ☐ DNR / MOST / POLST (if applicable) | ☐ Funeral home information (if known) |

■ PALLIATIVE CARE REQUEST SCRIPT

With a doctor or care team: "We are trying to better manage symptoms, stress, and decisions related to this serious illness. Would palliative care be appropriate to help with comfort, quality of life, goals of care, and family support?"

If they say it is not needed: "What support would you recommend for symptom management, caregiver stress, and planning?"

If hospice is mentioned: "Can you explain whether you are recommending hospice now, or palliative care while treatment continues?"

ROB'S NOTE

Palliative care is one of the most misunderstood words in medicine.

Families hear it and brace themselves, like they have just been told the worst. But palliative care is not the end of the road. It is someone walking beside you on the road. They ask the questions no one else has slowed down to ask.

I have watched a palliative care team change a family's whole experience. They did not add another treatment. They finally asked, "What matters most to you now?" Then they built the plan around the answer.

You do not have to be dying to deserve comfort. You do not have to give up to want less pain. Palliative care means taking your symptoms, your stress, and your dignity seriously, while there is still a lot of living to do.

CHAPTER 11 TAKEAWAY

Palliative care is comfort and clarity alongside treatment, not instead of it. The next right question is: **would extra support help us manage symptoms, stress, and decisions?**

CARDINAL MOMENT



A daughter said no to palliative care for months. She was sure it meant giving up, and her mother was still going to every chemo appointment. She thought that accepting comfort was the same as accepting defeat.

When she finally said yes, the palliative nurse just asked what was making the days so hard. The nausea. The sleepless nights. The pain no one had fully treated. Then the nurse fixed what could be fixed. Her mother kept fighting her cancer. And for the first time in months, she also felt like a person again.

Palliative care is not the opposite of hope. It is comfort walking next to the fight, so the fight stays bearable.

A composite drawn from years at the bedside. Names and details changed.

Hospice

THIS CHAPTER IN 30 SECONDS

- **What it is:** comfort-focused care for eligible people nearing the end of life — a full team (nurse, aide, social worker, chaplain, volunteers, medical director) wherever the person lives. It is not giving up.
- **The trade-off:** Medicare generally stops paying for treatment intended to cure the terminal illness; care for other conditions and comfort care continue, and you can leave hospice at any time and return to regular Medicare.
- **What it is not:** usually not a full-time bedside caregiver — families still provide much of the hands-on daily care.
- **Cost and the clock:** under the Medicare Hospice Benefit families usually pay almost nothing (often just a copay of up to \$5 per comfort prescription); the roughly six-month prognosis is renewable with recertification, though room and board in a facility is billed separately.

WHAT FAMILIES USUALLY THINK

Families often think hospice means death is days away, that all treatment stops, that they are abandoning their loved one, or that they have failed. None of that is true. Hospice continues care — it changes the goal from cure to comfort. One thing does change: **Medicare generally stops paying for treatment intended to cure the terminal illness.** Care for other conditions and comfort treatment continue, and the person can leave hospice at any time. Hospice does not shorten the time; it makes the time better — the pain controlled, the fear lowered, the family finally able to be family again instead of a frightened medical crew.

What Hospice Provides

Hospice provides a team and a plan of care:

- regular nurse visits for symptom and comfort management
- aide visits for personal care
- a social worker for support and planning
- chaplain or spiritual care if wanted, and volunteers
- bereavement support for the family — typically for about 13 months after death

It typically covers medications for the terminal condition, equipment such as a hospital bed or oxygen, and supplies. Hospice works wherever the person lives — home, assisted living, memory care, or skilled nursing — and inpatient hospice may be available when symptoms require it.

What hospice usually does not provide is a full-time bedside caregiver — the family, and sometimes private duty caregivers, still provide much of the hands-on daily care, so plan that coverage early. And if someone improves or wants treatment again, they can leave hospice.

Two Things Families Get Backwards

First, families fear hospice will be a financial burden. Under the Medicare Hospice Benefit it is usually the opposite: the team, the medications and supplies, short-term inpatient and respite care, and grief support are covered, and most families pay almost nothing — often just a small copay of up to \$5 per comfort prescription. The one real gap is room and board if the person lives in a facility, which is billed separately. Confirm the specifics for your plan — but do not let the fear of cost keep you away.

Second, families think hospice is a one-way door with a hard six-month clock.

Hospice is generally for a prognosis of about six months or less, but it is reversible and renewable: a person can leave hospice and return, and the benefit can continue past six months with recertification if they remain eligible. There is also a level most families never hear about — continuous or "crisis" care, where a nurse can stay at the bedside for hours when symptoms are hard. If symptoms spike, ask for it by name. It is short-term and criteria-based, so ask the hospice what the criteria are.

When to Ask About Hospice

Ask for an evaluation when a serious illness is progressing and you see a pattern of decline:

- ☐ more hospitalizations or ER visits
- ☐ more weakness, sleeping, and time in bed; less eating and drinking; weight loss
- ☐ more falls, infections, confusion, or shortness of breath
- ☐ wounds that will not heal, difficulty swallowing, or treatment that is no longer helping or has become too hard on the person

Also consider it when a doctor has mentioned comfort care, when a palliative team has raised it, or when the family finds itself quietly asking, "What are we doing this for?" Patterns, not single bad days, tell the real story.

The Question Families Are Afraid to Ask

THE WORDS THAT HELP

If someone is afraid that hospice means giving up, the words that help are simple: **"We are not trying to give up. We are trying to understand whether comfort-focused support would help now."**

An evaluation commits no one to anything, and the family always chooses. Choosing hospice is not abandoning the person — it is deciding that their comfort, their dignity, and their time matter more than one more hard treatment that may not help.

WHAT TO DO THIS WEEK

1. **Write down the decline with specific examples, then ask the provider for a hospice evaluation** — it commits you to nothing.
2. **Call more than one agency** and compare team, visits, after-hours support, and services.
3. **Plan the hands-on caregiving coverage.**
4. **Gather the documents** — power of attorney, advance directive, DNR/POLST, medication and doctor lists.
5. **Name a family point person** and how everyone will be kept updated.

■ **Worksheet — Hospice Questions & Decision** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ HOSPICE EVALUATION & AGENCY QUESTIONS

1. Does my loved one appear hospice appropriate, and for what diagnosis?
2. What decline and symptoms are you seeing, and what would hospice help with most?
3. What treatments would continue, and what medications, equipment, and supplies would hospice provide?
4. How often would the nurse and aide visit, and what support is available after hours?
5. Who do we call in a crisis at night, and what would make inpatient hospice appropriate?
6. What hands-on care is the family still responsible for, and would private duty care still be needed?
7. Can hospice support care at home, assisted living, memory care, or skilled nursing?
8. What happens if my loved one improves, or if we change our mind?

■ HOSPICE REQUEST SCRIPT

With a doctor, facility nurse, or hospice agency: "We are seeing decline and are wondering whether hospice would be appropriate. Can we talk honestly about whether the focus should shift toward comfort, dignity, and support?"

Or simply: "Given the changes we are seeing, would a hospice evaluation be appropriate?"

ROB'S NOTE

I have walked into a lot of rooms after the word "hospice" was finally said out loud.

Almost always, the fear in the room is bigger than the truth. Families think they are signing a surrender. What they are really doing is finally letting someone help.

I have watched pain come down. I have watched a daughter stop being a nurse and go back to being a daughter, holding her father's hand instead of his medication chart. I have watched families say the things they were afraid they would never get to say.

Hospice did not take the time away. It gave the time back — softer, closer, more honest. So if you are standing at that door, afraid to open it, hear me. Choosing hospice is not choosing to stop loving someone. It is choosing to love them all the way to the end, on their terms, with their comfort at the center. That is not giving up. That is showing up.

CHAPTER 12 TAKEAWAY

Hospice is not the family choosing death. It is the family choosing support when the illness is already changing life. The next right question is: **would comfort-focused care help us care better now?**

CARDINAL MOMENT



A family fought the word hospice for weeks. Saying yes felt like signing something they could never take back. So they waited. And their father spent those days gripping the bed rails in pain.

When they finally called, the nurse came that same night. The pain was under control by morning. For the first time in a week, their father slept. Their only regret, they told me later, was every day they had waited.

Hospice is not the day you stop fighting. It is the day comfort becomes the goal. And most families say afterward that they wish they had called sooner.

A composite drawn from years at the bedside. Names and details changed.

MORE TOOLS

Free worksheets, printable state guides, and checklists are waiting at
thecardinalspromise.com.

OUR MISSION

Mission

To help families avoid unnecessary suffering by understanding the healthcare system before a crisis forces them to.

If this book helps one family make one better decision, it has done its job.

Medicare Basics

Part 3 — Paying for Care. Care decisions often become money decisions. Families may know what help is needed but not who pays for it, and that confusion creates panic, arguments, and false expectations. These chapters are not meant to turn you into a Medicare expert, Medicaid planner, or elder law attorney. They are meant to help you ask better questions. Rules can change, and coverage depends on the person's situation, plan, medical need, eligibility, provider participation, and documentation. When in doubt, ask the plan, provider, social worker, or a qualified professional — and do not build a care plan on assumptions.

“An investment in knowledge pays the best interest.”

— ATTRIBUTED TO BENJAMIN FRANKLIN

THIS CHAPTER IN 30 SECONDS

- **The one sentence:** Medicare is health insurance, not long-term care insurance — it generally does not pay for long-term custodial care.
- **Know the plan:** Original Medicare (Parts A and B, often with Part D and a Medigap supplement) follows Medicare rules; Medicare Advantage adds networks, referrals, and prior authorization.
- **The trapdoor question:** ask early and more than once — *“Is my parent admitted as an inpatient, or under observation status?”* — and write down the answer and who gave it.

WHAT FAMILIES USUALLY THINK

Families often think Medicare pays for nursing homes, assisted living, someone to stay with Dad, or private duty care. It usually does not — so planning gets delayed and caregivers burn out waiting for help that is not coming.

The Main Parts of Medicare

PART	WHAT IT GENERALLY HELPS COVER
Part A	Hospital insurance — inpatient hospital care, skilled nursing facility care under certain conditions, hospice, and some home health.

Part B	Medical insurance — doctor visits, outpatient care, preventive services, durable medical equipment, and some home health.
Part C	Medicare Advantage — private plans through which you receive your Medicare benefits. You are still in Medicare, but the plan's networks, prior authorizations, and copays apply. Call the plan directly.
Part D	Prescription drug coverage — depends on the plan, formulary, pharmacy, and cost-sharing.

Original Medicare vs. Medicare Advantage

	ORIGINAL MEDICARE + MEDIGAP	MEDICARE ADVANTAGE
Doctors & hospitals	Any provider that takes Medicare, anywhere in the U.S.	The plan's network; referrals and prior authorization are common
Monthly cost	Higher premiums (Part B + Medigap + Part D)	Often low or \$0 premium on top of Part B
Cost when sick	Predictable — Medigap fills most of the gaps	Copays add up visit by visit, up to the plan's yearly out-of-pocket maximum
Drug coverage	Separate Part D plan	Usually built in
Dental, vision, hearing	Not included	Often included, with limits — read them
Travel & snowbirds	Works in every state	Networks are local — check coverage away from home
Changing your mind	Can switch to Advantage any fall	Going back may mean health questions (underwriting) for Medigap
Tends to fit	Complex health needs, wants any doctor, values predictable bills	Healthy now, budget-tight, comfortable inside a network

One more caution: leaving an Advantage plan to return to Original Medicare later may mean medical underwriting to buy a Medigap supplement — a person with health problems can be turned down or charged more. Ask before switching, not after.

What Medicare Usually Does Not Cover

THE NEED CAN BE REAL AND STILL NOT COVERED

Medicare generally does not cover long-term custodial care when that is the only care needed — ongoing help with bathing, dressing, toileting, meals, housekeeping, supervision, long-term dementia care, assisted living or memory care room and board, custodial nursing home care, or private duty care. Other paths may exist (Medicaid waivers, VA benefits, long-term care insurance, state resources), but do not assume Medicare pays for daily care.

Home Health, Skilled Nursing, and the Inpatient Trapdoor

Home health. Medicare may cover home health — skilled nursing, therapy, medical social services, and part-time or intermittent aide care in the plan of care — when requirements are met: a skilled need, homebound status, a clinician's evaluation, and documentation. Ask, "Can we schedule a provider evaluation to determine whether home health is appropriate?"

Skilled nursing facility care. Medicare Part A may cover a limited stay if requirements are met: Part A days available, a qualifying inpatient hospital stay, a Medicare-certified facility, and a need for skilled care or therapy. The qualifying stay generally means at least three consecutive days as an admitted inpatient — observation days and the discharge day do not count — though many Medicare Advantage plans and some programs waive this. Ask how many covered days are available and what you could owe. In 2026: days 1–20 generally paid in full, days 21–100 with a \$217-per-day coinsurance, and nothing after day 100 — confirm current amounts at [medicare.gov](https://www.medicare.gov).

Inpatient vs. observation status. A person may spend the night in the hospital and still be considered outpatient under observation status, which can affect whether Medicare covers a later skilled nursing stay. Ask early and more than once: "**Is my parent admitted as an inpatient, or under observation status?**" Write down the answer and who gave it.

Hospice, Assisted Living, and Equipment

Hospice. When eligibility requirements are met, Medicare covers hospice — team support, medications related to the hospice plan, equipment, supplies, and bereavement support. Room and board in a facility is generally not covered unless the team determines short-term inpatient care is needed for symptom management.

Assisted living and memory care. Medicare usually does not pay the monthly residential room and board, though it may still cover medical services there — doctor visits, covered therapy, home health, hospice, equipment, or medications. That cost is usually private funds, long-term care insurance, VA benefits, a Medicaid waiver in some states, or family money.

Durable medical equipment. Medicare may cover a walker, wheelchair, hospital bed, oxygen, or bedside commode when medically necessary, prescribed, and supplied by a Medicare-approved supplier. Ask:

- ☐ whether it is covered and what you will owe
- ☐ who orders it and which supplier to use
- ☐ whether prior authorization is needed
- ☐ who teaches you to use it

WHAT TO DO THIS WEEK

1. **Identify the coverage** — Original Medicare or Medicare Advantage — and find the plan name and number.
2. **Name the exact care needed** — hospital, rehab, home health, hospice, equipment, or residential care.
3. **Ask whether that care is skilled or custodial** — this decides most coverage.
4. **Ask who can help** — case manager, social worker, SHIP counselor, or benefits counselor.

■ **Worksheet — Medicare Coverage Questions** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK WHENEVER SOMEONE SAYS "MEDICARE SHOULD COVER IT"

1. Which part of Medicare — Original or Advantage — and is the provider in network?
2. Is prior authorization required, and what requirements must be met?
3. Is there a skilled need, and is this short-term or long-term care?
4. What does Medicare pay, and what do we owe?
5. What happens if coverage ends, and can we appeal a denial or discharge?
6. Who can explain this in writing?

ROB'S NOTE

Medicare confusion can make families feel foolish. They are not foolish.

The system is complicated. The words sound like they should mean more than they do. Skilled care. Custodial care. Observation. Inpatient. Benefit period. Prior authorization. Advantage plan. Supplement. It's a lot of hard words. And most families are trying to make sense of them while someone they love is falling, getting worse, or being sent home.

So don't try to remember it all. Ask better questions. Write down names and dates. Ask what is covered, what is not, and what happens next.

Medicare can be a big help. But it is not a free pass for aging. The clearer the family gets about that, the fewer surprises they will face.

CHAPTER 13 TAKEAWAY

Medicare is health insurance, not long-term care insurance. It may cover skilled or medical services, but not long-term daily custodial care. The next right question: **what exact care is needed, and does Medicare cover it in this situation?**

CARDINAL MOMENT



A son was sure Medicare would pay for his mother's long-term care. He found out it would not the way most families do. A bill showed up months in, and it scared him.

What Medicare did cover, it covered well. It paid for the hospital, the rehab after, the home-health nurse, and later, hospice. If he had known where that line was before the crisis, he could have skipped a month of worry.

Medicare pays for a lot. But it does not pay for the everyday, long-term care most families think it does. Learn where the line is before you are standing on it.

A composite drawn from years at the bedside. Names and details changed.

Medicaid Basics

THIS CHAPTER IN 30 SECONDS

- **What it is:** a needs-based program, administered by states, that may help pay for long-term care — nursing facility or home and community-based services — for people who meet both financial and functional or medical requirements. It is not Medicare, not automatic, and not the same everywhere.
- **Before moving money:** transfers and gifts may be reviewed during a look-back period — currently five years (60 months) in most states — so get advice from an elder law attorney or Medicaid planner before you move anything.
- **Also ask:** what spousal impoverishment protections let the spouse at home keep, and whether estate recovery applies — get it in writing.

WHAT FAMILIES USUALLY THINK

Families think Medicaid is Medicare, that it only pays for the poor, that it takes the house right away, or that they should wait until the money is gone. Most of that is too simple. One family needs nursing facility Medicaid, another home and community-based services, another a waiver or help paying Medicare premiums (ask for "Medicare Savings Programs" by name). Find the right doorway.

Medicaid vs. Medicare

A person can have both (often called "dual eligible"), but neither one makes the other available.

MEDICARE	MEDICAID
Health insurance	Needs-based program
Hospital, doctor, short-term SNF, home health, hospice	May help pay for long-term services and supports
Generally does not pay for long-term custodial care alone	May cover nursing facility or home/community services if you qualify
Same federal rules nationwide	Administered by states — rules vary by state

Nursing Facility, Home & Community, Assisted Living, and Memory Care

Medicaid is the nation's primary payer for long-term services and supports, at home or in a facility.

Nursing facility Medicaid may help pay for that level of care, but qualifying involves a detailed financial review and clinical eligibility. Not every facility accepts Medicaid, and not every bed is a Medicaid bed — ask early.

Home and community-based services (HCBS) let some people receive care at home instead of an institution, often through state waiver programs. Waiting lists can exist.

Assisted living and memory care coverage varies widely. In some states a waiver may pay for certain services but not room and board; in others these settings stay private pay. Ask the community and the state Medicaid office before move-in, not after the money is gone.

Spend-Down, Transfers, and the Look-Back Period

DO NOT MOVE MONEY WITHOUT ADVICE

"Spend-down" generally means reducing countable income or assets to meet Medicaid rules — not giving money away or making random purchases. For long-term care Medicaid, transfers and gifts may be reviewed during a look-back period — currently five years (60 months) in most states — and can create penalties or eligibility problems. Before spending down, transferring assets, changing titles, or making gifts, ask a qualified elder law attorney or Medicaid planner. Good planning can protect care options. Bad guessing can create expensive delays.

WHAT NOBODY TELLS YOU

The conversion trap. Families are often stunned to find that a parent with a year of savings left — who will then need Medicaid — gets quietly turned away by nursing facilities. Admissions offices weigh how long a resident can pay privately before converting, and nobody says that out loud. Three things change the odds. First, *present the runway as an asset*: apply to Medicaid-certified facilities with a clear written plan — "twelve months private pay, Medicaid application filed at month nine, elder law attorney already engaged." That is a profile many buildings accept. Second, know your rights once admitted: a Medicaid-certified facility cannot evict a resident for converting from private pay to Medicaid, and it is illegal for one to demand a promise to stay private-pay or to waive Medicaid rights — the fight is only ever about admission. Third, if a hospital stay happens, choose the rehab facility with the long-term plan in mind — a Medicare-paid rehab stay is often the door into a building for the long term, so tell the discharge planner exactly that. If a nursing facility asks you to promise to stay private-pay, or to waive Medicaid rights as a condition of admission, say no and put the request in writing — for a Medicaid-certified facility that condition is not enforceable, and a written record matters later. Know what the Long-Term Care Ombudsman can and cannot do: the office is a powerful advocate for someone already living in a facility, and a limited one for someone still trying to get in. Assisted living communities that are not Medicaid-certified can legally require a private-pay period. This is exactly the moment for an elder law attorney and a placement advisor who knows which local buildings say yes.

The Spouse at Home and Estate Recovery

The spouse at home. When one spouse needs long-term care and the other stays home, spousal impoverishment protections may let the spouse at home keep certain income or assets within program rules. Do not assume both spouses must go broke, and do not assume everything is protected.

Estate recovery. State Medicaid programs are required to seek recovery from certain enrollees' estates. For people age 55 or older, states must do so for nursing facility services, home and community-based services, and related hospital and prescription drug services. Rules, exemptions (for a spouse, disabled child, caregiver child, and others), deferrals, and hardship waivers vary. Ask whether it applies here.

When to Ask for Help

Ask for help early if:

- ☐ long-term care may be needed, or private funds are running low
- ☐ a spouse remains at home, the person owns a home, or there is a trust
- ☐ assets were transferred or gifted
- ☐ discharge is close with no payment plan

Helpers: the state Medicaid office, the local Department of Social Services, a hospital or rehab social worker, the Area Agency on Aging, a SHIP counselor, an elder law attorney, and a veterans service officer if VA benefits may apply.

WHAT TO DO THIS WEEK

1. **Name the program question** — nursing facility, HCBS, waiver, assisted living, memory care, or help with Medicare costs — and the level of care needed.
2. **Gather financial documents** — bank and income statements, insurance cards, property info, power of attorney.
3. **Ask about timing** — application length, waiting lists, retroactive coverage, and what to pay while pending.
4. **Do not transfer assets without advice**, and ask about estate recovery in writing.

■ **Worksheet — Medicaid Planning** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ KEY QUESTIONS TO ASK

1. Which Medicaid program may apply, and what are the income and asset limits?
2. What medical or functional level of care is required, and is there a waiting list?
3. What documents are needed, how long does the application take, and can coverage be retroactive?
4. What does the person pay from monthly income, and what happens to the spouse at home?
5. How is the home treated, and do transfers or gifts create problems?
6. Does estate recovery apply, and what protections or hardship waivers exist?
7. Does the facility accept Medicaid, are there limited slots, and what happens if private funds run out?

ROB'S NOTE

Medicaid talks make families nervous. People worry about the house, the bank account, the spouse still at home, the look-back, and the application. They fear that one wrong move will ruin everything.

That fear makes sense. But fear is not a plan.

The best thing a family can do is slow down. Gather your papers. Ask questions about your own state's rules. Get good help before you make any money moves. Medicaid may become the bridge that keeps care going when private money runs out. But it is a bridge with rules. Don't cross it blindfolded.

Ask early. Write everything down. And before you move money, change names on titles, or give gifts, get advice.

CHAPTER 14 TAKEAWAY

Medicaid may help pay for long-term care for people who meet financial and functional requirements, but the rules vary by state and program. The next right question is: **which program might apply, what are this state's rules, and who can help us apply correctly?**

CARDINAL MOMENT



A daughter was scared that applying for Medicaid meant her mother would lose the house she had lived in her whole life. She almost didn't apply at all, all because of a rumor.

A good elder-law lawyer walked her through the rules. They were far kinder than the stories she had heard. The home was often protected. The healthy spouse was not left with nothing. The paperwork was hard. But the disaster she feared never came.

Medicaid is a maze, not a trap. Get a guide who knows the way, and start before the money is gone, not after.

A composite drawn from years at the bedside. Names and details changed.

Private Pay

THIS CHAPTER IN 30 SECONDS

- **What it is:** the family or the person receiving care pays directly from personal funds — savings, retirement income, pensions, Social Security, investments, home sale proceeds, or family contributions.
- **What it buys:** what insurance does not — private duty care, assisted living, memory care, adult day care, respite, transportation, home modifications. Money creates options; it cannot buy unlimited time.
- **The key number:** the monthly burn rate — a number, not a vague guess. Divide available funds by the monthly gap (costs minus income) to see how long the plan can last.
- **Protect two people:** do not spend down without knowing how the spouse at home will live, and put family contributions in writing — who pays, how much, gift or loan, and the backup plan.

WHAT FAMILIES USUALLY THINK

Families often think Medicare will cover more than it does, that they should not have to use savings, or that Dad wanted to leave something to the kids. Those thoughts are dangerous when they delay planning. Money should not make every decision — but it should be named clearly enough that the family can plan honestly.

What Private Pay Covers — and What It Doesn't Solve

Private pay can cover what insurance does not:

- private duty caregivers
- assisted living and memory care fees
- adult day care and respite
- care managers and transportation
- meal delivery, supplies, and home safety modifications

But money cannot guarantee a perfect caregiver, a safe home, family agreement, or no future decline. It creates options, and it can disappear quickly without a plan.

The Monthly Burn Rate

The monthly burn rate is the amount spent on care each month. Families need this as a **number, not a vague guess**. Include:

- housing or facility base rate
- care level fees
- private duty care
- medications
- supplies
- transportation
- insurance premiums
- food
- home maintenance
- utilities
- copays
- equipment
- care manager fees

Then compare it to monthly income and available savings. The plan may prove sustainable, work only for a limited period, or point toward Medicaid planning, long-term care insurance, VA benefits, a home sale, or a less expensive setting. Numbers turn fog into a map.

Home, Assisted Living, and Memory Care Costs

At home, a few hours a week may be manageable, but daily help costs more, overnight costs more, and twenty-four-hour care can become extremely expensive. Ask how many hours are needed, the hourly rate, minimum shifts, and night and weekend rates. The question is not what is emotionally preferred — it is whether home is safe, supported, and financially sustainable.

Assisted living bills may include base rent, care level fees, medication management, community or move-in fees, supply and transportation charges, and rate increases over time. Ask for a written cost breakdown, not the brochure number.

Memory care costs more because it provides more supervision and dementia-specific support. Ask what needs would require added private duty care or a move, and whether Medicaid options exist in that state. It can drain funds quickly if not planned carefully.

Family Contributions and the Spouse at Home

Family contributions should be written down, not built on vague promises: who is contributing, how much, how often, for how long, gift or loan, who pays the bill, who tracks payments, and the backup plan if someone stops. A written plan keeps the care plan from collapsing because everyone "thought someone else was handling it."

WATCH CLOSELY — THE SPOUSE AT HOME

The spouse at home needs extra care in private-pay planning. Do not spend down money without understanding how the spouse at home will live — what income they need, what bills, housing, and insurance costs remain, what assets are available, and whether Medicaid planning is likely later. Care planning should not protect one person by making the other financially unsafe. Both lives matter.

Ask for professional guidance when care costs are high, funds may run out within one to two years, a spouse remains at home, a home may need to be sold, or Medicaid may be needed later. An elder law attorney, financial advisor, Medicaid planner, veterans benefits counselor, or care manager can help.

WHAT TO DO THIS WEEK

1. **Write down the current care costs** — every recurring cost. Do not guess.
2. **Identify monthly income** — Social Security, pension, retirement distributions, other income.
3. **Estimate available savings** — list accounts and approximate values. Do not make changes yet.
4. **Calculate the monthly gap** — care cost minus income.
5. **Ask how long the current plan can last** — divide available funds by the monthly gap.
6. **Decide when another plan is needed** — review insurance and VA benefits, ask about Medicaid planning, or compare settings before the money is gone.

■ **Worksheet — Private Pay Planning** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ COST QUESTIONS TO ASK A FACILITY OR AGENCY

1. What is the base monthly cost, what is included, and what costs extra?
2. How are care levels priced, and how often can rates increase?
3. Are there move-in fees or deposits, and are supplies, medication, and transportation included?

4. What happens if care needs increase, or if private funds run out?
5. Do you accept long-term care insurance, or Medicaid / Medicaid waiver if applicable?
6. What is the move-out policy, and can we get all costs in writing?

ROB'S NOTE

Talking about paying out of pocket can feel cold, because it puts numbers next to love. But the numbers are not the enemy. The numbers are there so the family can protect the care plan.

I have seen families put off the cost talk until the crisis hits. Then everyone is shocked, rushed, and angry. That is when bad choices happen.

Talking about money does not mean you care less. It means you are trying to make care last. It means you are trying to protect the parent, the caregiver, the spouse at home, and the whole family from a surprise.

Don't let guilt write the budget. Don't let fear hide the numbers. Put the costs on paper. Then decide what the next right step should be.

CHAPTER 15 TAKEAWAY

Private pay means using personal or family funds for care that insurance or public benefits may not cover. The next right question is: **what does the care plan cost each month, and how long can we safely sustain it?**

CARDINAL MOMENT



A family paid for care out of their own pocket. They watched their savings drop, faster every month. The not-knowing was its own kind of fear. Every time they took money out, it felt like the last safe time.

What helped them was a single page. It showed what the care cost, how many months the money would last at that rate, and exactly what came next when it ran low. The number was scary. But not knowing the number had been worse.

Paying out of pocket is not failure. It is a stage, and it has a way out. Map how long the money lasts and what comes next — before an empty account, not a plan, makes the choice for you.

A composite drawn from years at the bedside. Names and details changed.

Long-Term Care Insurance

THIS CHAPTER IN 30 SECONDS

- **The starting line:** you either have a policy or you do not — coverage usually has to be purchased before care is needed, so do not try to create it in the middle of a crisis. (Planning ahead for yourself? Traditional and hybrid life/LTC policies are typically bought in one's 50s or 60s — premiums rise with age and health changes can close the door — so compare options with a fee-only advisor well before care is on the horizon.)
- **First job:** find the policy — check documents, bank statements for premium drafts, employer or retirement benefit files, life insurance policies with long-term care riders, hybrid policies, and financial advisor records.
- **If no policy:** move on to the next payment option — no policy is not a moral failure; it is just information.
- **If a policy exists:** call the company and ask about covered settings and providers, the benefit amount, the elimination period, reimbursement or cash benefit, and what claim documentation is required.
- **Then:** get the benefit details in writing, match the provider or setting to the policy requirements before choosing care, and assign one person to manage the claim.

WHAT FAMILIES USUALLY THINK

Families often think, "I'm sure Dad has something," or "Mom was organized, so there must be a policy," or "We'll just buy a policy now." Maybe. Maybe not. The first job is not to guess — it is to find the policy.

What to Look For — and Where

Search for:

- ☐ a long-term care insurance policy
- ☐ premium notices
- ☐ annual statements
- ☐ insurance company letters
- ☐ bank drafts for premiums
- ☐ employer or retirement benefit documents

- ☐ life insurance policies with long-term care riders
- ☐ hybrid life/long-term care policies
- ☐ insurer emails
- ☐ financial advisor records
- ☐ an agent's contact information

Check:

- ☐ the important-documents folder
- ☐ filing cabinet
- ☐ safe
- ☐ desk drawers
- ☐ bank statements
- ☐ email
- ☐ financial advisor portal
- ☐ tax files
- ☐ insurance binder
- ☐ mail pile
- ☐ power of attorney files

If There Is No Policy

If there is no policy, do not get stuck here. Move to the next payment questions: Can care be paid privately? Are VA benefits possible? Could Medicaid become part of the plan? Is there a state program or waiver? Does the care setting offer financial options? No policy means no long-term care insurance benefit to activate. That is not a moral failure. It is just information.

If There Is a Policy

If a policy exists, call the insurance company. Ask what is covered and how to start a claim — and do not assume every setting or service qualifies. Ask specifically about:

- ☐ home care, assisted living, memory care, adult day care, and nursing facility care
- ☐ caregiver and licensed-agency requirements
- ☐ the daily or monthly benefit amount
- ☐ the elimination period
- ☐ the maximum benefit period
- ☐ inflation protection
- ☐ required assessments and documentation
- ☐ whether it is reimbursement or a cash benefit

- ☐ who can submit claims

Key Terms Families Need to Know

Benefit amount — how much the policy may pay, listed as a daily amount, monthly amount, or pool of money.

Elimination period — the waiting period before benefits begin, during which the family may need to pay privately. Ask whether it is counted in calendar days, service days, or another rule.

Covered setting — not every policy covers every type of care; ask whether it covers the setting you are considering.

Covered provider — some policies require a licensed agency or approved facility, and privately hired caregivers may not qualify.

Claim documentation — the insurer may require care notes, invoices, assessments, or provider forms. Without the right paperwork, payment may be delayed.

WHAT TO DO THIS WEEK

1. **Find out whether a policy exists** — check documents, bank statements, financial records, and advisor files.
2. **If no policy exists, move on** to the next payment option. Do not linger.
3. **If a policy exists, call the company** and ask how to start a claim and what documents are needed.
4. **Get the benefit details in writing** — ask for a benefits summary.
5. **Match the policy to the care plan** — make sure the provider or setting meets the policy requirements before choosing care.
6. **Assign one person to manage the claim** — claims involve ongoing invoices, forms, and follow-up.

■ **Worksheet — Long-Term Care Insurance Policy Finder** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ STEP 6: CLAIM REQUIREMENTS TO ASK THE INSURER

1. How do we start a claim, and what forms are needed?
2. Is a provider assessment or doctor statement required?
3. Are invoices or care notes required, and must the provider be licensed?

4. How long does approval usually take?
5. Who can speak on behalf of the policyholder, and what power of attorney paperwork is needed?

Final decision

Where this lands: No policy — move to other payment options ☐ Policy exists, start a claim ☐
 Policy exists but current care may not qualify ☐ More information needed ☐

Next step _____

Person responsible _____

Due date _____

ROB'S NOTE

Long-term care insurance is not something families should spend three weeks arguing about during a crisis.

Find the policy. If it exists, call and start the process. If it does not exist, move on.

That may sound blunt, but it is kind to the family's energy. When care is needed now, the family does not need pretend coverage. They need real choices.

Paper first. Facts second. Decision third.

CHAPTER 16 TAKEAWAY

Long-term care insurance starts with one question: does a policy already exist? If yes, find out what it covers and how to start a claim. If no, move to the next payment option.

CARDINAL MOMENT



A man found out, only after his father moved into care, that the old man had quietly paid for long-term-care insurance for twenty years. The policy was in a drawer no one had thought to open.

It covered most of what he needed. But first the family had to learn how to file the claim, log the hours of care, and wait out the waiting period before the payments began. The coverage was real. The paperwork almost buried it.

If a policy might exist, find it early and read it twice. The coverage is only ever as good as the claim someone actually files.

A composite drawn from years at the bedside. Names and details changed.

Prescription Assistance

THIS CHAPTER IN 30 SECONDS

- **The core rule:** *"Never disqualify yourself. Ask the question. Complete the application. Let the program tell you no."*
- **The cap:** Medicare caps yearly out-of-pocket costs for covered Part D drugs — \$2,100 in 2026, adjusted annually — and the Medicare Prescription Payment Plan spreads them into monthly bills.
- **Doors to try:** Extra Help, manufacturer patient assistance programs, State Pharmaceutical Assistance Programs, disease-specific foundations.
- **Ask the pharmacist:** the counter price is a starting point, not a verdict — check the cash price, another pharmacy, a 90-day supply.
- **Ask the prescriber:** *"Is there a lower-cost alternative that is safe and appropriate?"* — and ask for a medication review.

The Million-Dollar House and the Free Medication

Years ago, working for a large pharmaceutical company, I helped a physician's office find a way for a patient to afford a medication that cost more than \$850 per month. The office said she would never qualify: "She lives in a multi-million-dollar home."

I asked a different question: "What is her income?" Many pharmaceutical assistance programs focus primarily on income, not the value of a person's home. The patient applied anyway. She was approved. Her medication was provided at no cost.

Families assume they do not qualify before they ever ask. **Never disqualify yourself. Ask the question. Complete the application. Let the program tell you no.**

\$ THIS COULD SAVE YOU THOUSANDS

Never disqualify yourself — let the program tell you no. Five moves, no lawyer, no fee: check **Medicare Extra Help** (worth thousands a year; apply free at ssa.gov); search **manufacturer patient assistance programs** for every expensive brand-name drug — many look at income, not the house; ask the pharmacist to run the **cash price** against the copay; **recompare the drug plan every fall** during open enrollment; and ask for a **medication review**. The \$850-a-month medication in the story above went to zero with one application — more than \$10,000 a year, saved by asking.

WHAT FAMILIES USUALLY THINK

Families think the pharmacy price is the only price, that Medicare covers all prescriptions, or that they make too much to qualify. Do not guess — prescription assistance is a question tree: one door closes, you try the next.

Medicare Part D and Extra Help

Part D and drug coverage. What a drug costs depends on the specific Part D or Medicare Advantage plan — formulary, tier, preferred pharmacy, deductible, prior authorization, step therapy. Recompare plans every fall at open enrollment.

The yearly cap on drug costs. As of this writing, Medicare caps what a person pays out of pocket for covered Part D drugs each year — \$2,100 in 2026, adjusted annually. After that, covered drugs cost nothing for the rest of the year. Ask also about the **Medicare Prescription Payment Plan**, which spreads those costs into predictable monthly bills.

Extra Help. Extra Help is a Medicare program that helps people with limited income and resources pay Part D premiums, deductibles, and copays. Some people qualify automatically; others apply, before or after enrolling in Part D. Do not assume income is too high — check.

Manufacturer, State, and Foundation Programs

Manufacturer patient assistance programs (PAPs). Some drug companies provide financial assistance or free medication, sometimes for people with Part D and sometimes only for those without insurance. Medicare has a tool to search assistance programs by drug.

State Pharmaceutical Assistance Programs (SPAPs). Some states help with drug costs. Ask whether this state has a program, who qualifies, and whether it works with Part D.

Disease-specific foundations. Some nonprofits help with medication costs for specific diagnoses — certain cancers, rare diseases, heart, lung, or kidney conditions. Funding opens and closes, so check current availability and income limits.

Ask About Alternatives, Coupons, and a Medication Review

Alternatives. The easiest savings often come from one question: "Is there a lower-cost alternative that is safe and appropriate?" Ask about:

- ☐ a generic
- ☐ a therapeutic alternative
- ☐ a covered formulary option
- ☐ a 90-day supply
- ☐ whether the medication is still needed

Do not stop or change medications without clinical guidance — but do ask.

Coupons and discount cards. Discount programs may lower costs, but they may not apply with Medicare or Medicaid, and using one can mean the cost does not count toward the plan's deductible or out-of-pocket tracking. Sometimes the cash price is lower than the copay; ask the pharmacist for both.

Medication review. Ask for a medication review if there are many medications, duplicates, multiple prescribers, or if the person is falling, confused, dizzy, sleepy, or nearing end of life. A medication that made sense five years ago may not make sense now.

WHAT TO DO THIS WEEK

1. **Make a complete medication list** — name, dose, prescriber, pharmacy, and monthly cost.
2. **Ask the pharmacist for a cost review** of the most expensive medications.
3. **Ask the prescriber about alternatives** — a generic, formulary option, or safe substitute.
4. **Check assistance programs** — Extra Help, plus manufacturer and state programs using Medicare's tool.
5. **Assign one person to track medication costs** and follow up.

■ **Worksheet — Prescription Assistance** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE PHARMACIST

1. Which medication is costing the most, and is it running through the correct insurance?
2. Is this a preferred pharmacy — and would another pharmacy, mail order, or a 90-day supply be cheaper?
3. Is there a generic version or a discount option?
4. If we use a discount card, does it affect insurance tracking?
5. Should we ask the doctor about an alternative?

■ QUESTIONS FOR THE PRESCRIBER

1. Is this medication still needed, and is there a generic or lower-cost/formulary alternative?
2. Can the dose or formulation be changed safely to lower cost, and are any medications duplicated?
3. Could any medications be increasing falls, confusion, dizziness, or sleepiness?
4. Would a medication review be appropriate?
5. Is there a manufacturer assistance program, and can your office help complete the forms?

Assistance checks

Medicare Extra Help — could it apply? Yes ☐ No ☐ Unsure ☐ — who will check/apply?

Also check: Manufacturer patient assistance ☐ State Pharmaceutical Assistance Program ☐
Disease-specific foundation ☐

Documents often needed for manufacturer programs: prescription information, proof of income, insurance/Part D card, a doctor form, patient signature, and denial or prior-authorization information.

Medication safety review (check any concern)

- | | |
|--|--|
| ☐ Too many or duplicate medications | ☐ Multiple prescribers |
| ☐ Missed or double doses | ☐ Old medications still in the home |
| ☐ Dizziness, falls, or sleepiness | ☐ Hospice or palliative care involved |
| ☐ New confusion or poor appetite | |

HOPE'S NOTE — FROM THE PHARMACIST

Two things I wish every family knew. First, your pharmacist can see what your doctor often cannot: every prescription, from every prescriber, on one screen — and we are the easiest person in healthcare to reach. No appointment needed; just ask for a minute at the counter.

Second, never let the price at the register be the end of the conversation. Ask us to check the cash price, a different pharmacy, a 90-day fill, and whether the prescriber should hear about an alternative. The counter price is a starting point, not a verdict. And ask us to run a discount program — GoodRx is the one most people know. Patients are often unsure about these, but the discount price is sometimes lower than your insurance copay. The trade-off: a discount purchase usually does not count toward your deductible or out-of-pocket cap, so ask us which way wins for each drug.

ROB'S NOTE

Drug costs can make families feel ashamed. People don't always want to say, "We can't afford this medicine."

Say it anyway. Tell the pharmacist. Tell the doctor. Tell the social worker. Say it before pills get skipped, doses get stretched, or refills get put off.

A medicine can't help if it's stuck behind a price the family can't pay. There isn't always an answer, but there's almost always a better question. Ask what is covered. Ask what is cheaper. Ask what is still needed. Ask what help is out there.

A quiet drug bill can turn into a loud crisis if no one looks at it.

CHAPTER 17 TAKEAWAY

Prescription assistance starts with one practical step: make the medication list, find the expensive drugs, and ask the pharmacist and prescriber what can be done safely.

CARDINAL MOMENT



A widow was quietly cutting her heart pills in half to make them last all month. She was sure nothing could be done about the cost, and she was too proud to say so.

When her daughter finally told the pharmacist, he knew of a program that helped patients pay. It brought the price down to almost nothing. For most of a year, she had been going without her own medicine, and no one knew.

No one should have to choose between medicine and food, alone and unheard. Ask the pharmacist, the doctor, the drug maker. The help is there, but only for the family that asks.

A composite drawn from years at the bedside. Names and details changed.

VA Benefits

THIS CHAPTER IN 30 SECONDS

- **The first question:** "*did they serve?*" — if the person is a Veteran, a surviving spouse, or a dependent, VA benefits may matter.
- **Several doors, not one bucket:** VA health care, VA pension, Aid and Attendance, Housebound benefits, survivor benefits, long-term services and supports, caregiver support, and burial benefits.
- **Aid and Attendance is not automatic:** qualify for VA Pension or Survivors Pension first; the doctor documents the need on VA Form 21-2680.
- **Do not move money here either:** VA pension has a net worth limit and a 36-month look-back, with penalties for gifts made to qualify.
- **How to start:** find the DD214, gather care cost records, call a Veterans Service Officer — their help is free — and for claims, ask whether the helper is VA-accredited. Never pay anyone a fee or a share of benefits to file a claim.

WHAT FAMILIES USUALLY THINK

Families think, "Dad was in the military, so VA will pay," or "He never used VA before, so it is too late." Most of that is too simple. VA benefits are not one bucket — they are several doors. Your first job is to find the right one.

First Question: Did They Serve?

Did the person receiving care serve in the U.S. military? If yes or unsure, gather what you can:

- ☐ branch, dates of service, and discharge status
- ☐ DD214 or separation papers
- ☐ any VA claim number, health care enrollment, or prior VA benefits
- ☐ any service-connected disability rating
- ☐ surviving spouse status if applicable

Aid and Attendance, Housebound, and Pension

Aid and Attendance / Housebound. Monthly payments added to a VA pension for qualified Veterans and survivors who need help with daily activities or are housebound. They are not automatic — the person must first qualify for VA Pension or Survivors Pension. The doctor completes VA Form 21-2680 to document the need; it accompanies the pension application (VA Form 21P-527EZ, or 21P-534EZ for survivors).

VA Pension and Survivors Pension. VA Pension may provide monthly payments to eligible wartime Veterans who meet age or disability requirements and have limited income and net worth; Survivors Pension may help eligible surviving spouses or children. Ask a VA-accredited representative or Veterans Service Officer whether the wartime service period, discharge status, income, net worth, and unreimbursed medical expenses fit the rules.

DO NOT MOVE MONEY HERE EITHER

VA pension has its own net worth limit and its own look-back period — currently 36 months — on asset transfers, with penalty periods for gifts made to qualify. Do not move money to qualify without advice from a VA-accredited representative, and be wary of anyone selling financial products to "help you qualify."

VA Health Care, Long-Term Services, and Caregiver Support

Health care and long-term services. VA health care may provide access to geriatric and extended care programs — home-based primary care, homemaker and home health aide services, adult day health care, respite, skilled home health, community living centers, contract nursing home care, state Veterans homes, geriatric evaluation, palliative care, and hospice-related support. Eligibility varies by location and clinical need.

Caregiver support. VA has caregiver support programs for caregivers of eligible Veterans — education, training, resources, respite, and support groups, plus the Program of Comprehensive Assistance for Family Caregivers for those who qualify. Ask whether there is a caregiver support coordinator, or contact the VA Caregiver Support Line.

Care settings. VA does not simply pay for assisted living or memory care, but Aid and Attendance can add monthly pension support toward care, and some state Veterans homes or community programs may be relevant. For hospice or palliative care, ask whether VA support is available and how it coordinates with Medicare hospice. Many Veterans also have Medicare, Medicaid, or private insurance; understand how they work together. Do not rely on a brochure or a hallway rumor. Verify.

Documents and Who Can Help

Start gathering:

- ☐ DD214 or separation papers, and discharge status
- ☐ Social Security number, any VA claim number, and VA health care enrollment info
- ☐ marriage and death certificates if a survivor benefit is involved
- ☐ medical records, a doctor statement, and VA Form 21-2680 if applicable
- ☐ income and asset information, and care expense records
- ☐ power of attorney documents

For help, consider a local Veterans Service Officer, county Veterans Services office, VA medical center social worker, accredited VA claims agent or attorney, or the state Veterans affairs office. A Veterans Service Officer can often identify which benefits apply and help complete the forms — and for claims, ask whether the helper is VA-accredited.

WHAT TO DO THIS WEEK

1. **Ask the service question** — did they serve, in what branch, and when?
2. **Find the DD214** or other separation documents.
3. **Identify the VA connection** — enrollment, disability rating, or benefits received.
4. **Call a Veterans Service Officer** to review eligibility.
5. **Gather care cost records** — care, medications, and other unreimbursed medical expenses.
6. **Assign one person to manage the VA follow-up.**

■ **Worksheet — VA Benefits Screening** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE VETERANS SERVICE OFFICER OR VA CONTACT

1. Which VA benefits might apply, and is the Veteran enrolled in VA health care?
2. Is VA Pension, Survivors Pension, Aid and Attendance, or Housebound possible?
3. What income, asset, service-record, and medical-form requirements apply — and is VA Form 21-2680 needed?
4. Are unreimbursed medical expenses important to the calculation?
5. Could caregiver support, home-based services, respite, or a state Veterans home apply?
6. Could VA help with palliative care or hospice, and how does it work with Medicare?
7. What forms should be filed first, who can help complete them, and how long may it take?

Final decision

Where this lands: VA benefits do not appear relevant ☐ May apply — explore ☐ Review Aid and Attendance / Housebound ☐ Review VA health care enrollment ☐ Review caregiver support ☐
 More information needed ☐

Next step _____

Person responsible _____

Due date _____

ROB'S NOTE

VA benefits are easy to miss. Families are usually focused on the crisis right in front of them — the fall, the hospital stay, the assisted living bill, the worn-out spouse, the tour of a memory care home.

*But one question can open another door: **did they serve?***

If the answer is yes, don't guess and don't brush it off. Check. The benefit may not apply. The steps may take time. It may not cover everything.

But if a Veteran or a surviving spouse earned the right to help, the family should at least know what is possible. Find the papers. Call the right person. Ask the question.

CHAPTER 18 TAKEAWAY

VA benefits may help some Veterans, surviving spouses, or caregivers, but eligibility depends on the person and the program. The next right question is: **did they serve, and who can help us check?**

CARDINAL MOMENT



A daughter had no idea her father qualified for a VA benefit called Aid and Attendance. It would have paid for years of the care he had gone without. He had never made much of his service. He thought it "wasn't a big deal."

Once the claim was finally filed, it changed what he could afford. The only sad part was the years it could have helped, if someone had known to ask sooner.

If anyone in the family ever wore a uniform, ask about VA benefits. A war fought long ago can still help pay for care today.

A composite drawn from years at the bedside. Names and details changed.

Rob's memoir *The Cardinal's Promise* is the story that started this work — thecardinalspromise.com.

The Important Documents Folder

Part 4 — The Documents That Matter. When a crisis hits, the family that can find the right paper moves; the family that cannot gets stuck. These chapters help you gather the medical, legal, and financial documents that decisions actually depend on — one trusted folder, the words on the key legal documents, and the information a family needs the moment someone can no longer speak for themselves. You do not need to become a lawyer. You need to be able to find the right document quickly.

“The palest ink is better than the best memory.”

— CHINESE PROVERB

THIS CHAPTER IN 30 SECONDS

- **What it is:** one folder — paper, digital, or both — so the right person can put a hand on the right paper in minutes, not hours.
- **Why now:** the hospital will want the medication list, the facility will need to know who has authority, the doctor will ask about advance directives — almost always at the worst possible time.
- **The step:** start with the first five — medication list, doctor list, insurance cards, healthcare decision documents, and emergency contacts. A useful folder beats a perfect folder that never gets built.
- **Watch for:** add a HIPAA release — without one, even a devoted daughter can be told “we can’t discuss that.”
- **Keep it alive:** tell the main caregiver, backup caregiver, and healthcare decision maker where it lives, and review it every three months and after any major change.

WHAT FAMILIES USUALLY THINK

Families often think the paperwork can wait until things settle down, that one person “has it handled,” or that everything is somewhere in the house if they ever need it. But things rarely settle down, the one person who has it handled is not always reachable, and “somewhere in the house” is not a plan at 2 a.m. in an emergency room. Building the folder is not a sign that something bad is coming. It is a gift to the family for the day something does.

What Belongs in the Folder

One folder — paper, digital, or both — should hold the essentials in a few clear categories:

- ☐ emergency and medical contacts
- ☐ the medication list and doctor list
- ☐ insurance and benefit cards
- ☐ healthcare decision documents (power of attorney, advance directive, DNR or POLST if applicable)
- ☐ financial and legal documents (financial power of attorney, will, trust, deeds)
- ☐ care and facility paperwork
- ☐ benefits and military or VA documents
- ☐ end-of-life and funeral information

You do not need every item on day one. You need a folder that exists and grows.

One small document worth adding while you are at it: a **HIPAA release**, which lets doctors and hospitals share medical information with the family members named on it. Without one, even a devoted daughter can be told "we can't discuss that."

Start With the First Five

Do not wait for the whole folder to be perfect. **A useful folder beats a perfect folder that never gets built.** Start with the five documents a family reaches for most: the medication list, the doctor list, the insurance cards, the healthcare decision documents, and the emergency contacts. Once those five are in one place, the folder is already doing its job — and you can keep adding.

Keep It Alive, and Tell People Where It Is

A folder only helps if the right people know where it lives. Tell the main caregiver, the backup caregiver, and the healthcare decision maker, and store it somewhere secure. Make copies of important cards and lists, and ask whether copies of legal documents are acceptable or whether originals are needed. Then review it every three months, and after any hospitalization, medication change, doctor change, insurance change, or move. Documents change; a folder that never gets updated becomes a museum. This folder should be alive.

WHAT TO DO THIS WEEK

1. **Choose one folder location** — paper, digital, or both — and one document point person.
2. **Gather the first five documents** — medication list, doctor list, insurance cards, healthcare decision documents, and emergency contacts.

3. **Make copies of important cards and lists**, and confirm whether legal documents need originals.
4. **Tell the right people where the folder is** — main caregiver, backup caregiver, and healthcare decision maker.
5. **List what is missing** and assign who will find each item and where to look.
6. **Set a review date** — every three months and after any major change.

■ **Worksheet — Important Documents Folder Checklist** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

ROB'S NOTE

The documents folder isn't exciting. No one brags about a binder.

But when the hospital asks for the medication list, when the facility asks who's in charge, when the doctor asks about advance directives, when the family is tired and scared — that binder becomes a real gift.

These papers aren't busywork. When a hard moment comes, a family with the right documents can act — a family without them gets stuck.

Start with the first five. Then keep going.

CHAPTER 19 TAKEAWAY

The important documents folder gives the family one trusted place to find medical, legal, financial, and emergency information. The next right question is: **can the right person find the right document in less than ten minutes?**

CARDINAL MOMENT



When her mother collapsed, a daughter spent the first frantic hour not at the bedside, but on her knees in a closet. She was tearing through drawers, looking for an insurance card and some idea of what medicines her mother took.

Months later, a neighbor showed her a single folder that held all of it — cards, lists, documents, phone numbers — in one place. The next emergency came, because there always is one. This time she was ready in ninety seconds.

The folder is boring right up until the day it is everything. Build it now, on a calm afternoon, so a crisis never has to build it for you.

A composite drawn from years at the bedside. Names and details changed.

Healthcare Power of Attorney

THIS CHAPTER IN 30 SECONDS

- **What it does:** names the healthcare agent — also called a proxy or surrogate; the term varies by state — who makes medical decisions if your parent cannot.
- **What it does not do:** it does not usually cover money (that is a Financial Power of Attorney), and the agent must honor the person's known wishes, not personal preference.
- **Why now:** sign or update it while the person still has decision-making capacity — once capacity is lost, families may face asking a court to appoint a guardian: slower, more expensive, more public.
- **The step:** name a backup agent, have the wishes conversation, and share copies.
- **Watch for:** a move to another state, a changed agent or relationship, a serious diagnosis, or an old form — ask an attorney whether it still works.

WHAT FAMILIES USUALLY THINK

Families often think, "I'm the oldest, so it's me," or "Everyone agrees, so we don't need paperwork." Sometimes the law provides default rules, but crisis has a way of turning soft assumptions into hard arguments. The Healthcare Power of Attorney tells the medical team who should speak when the patient cannot.

What It Does — and Does Not Do

Depending on the document and state law, it may cover decisions about:

- doctors, hospitals, and surgery
- medications, tests, and treatments
- rehab and care-related placement
- palliative care and hospice
- artificial nutrition or hydration
- life-sustaining treatment
- comfort-focused care
- access to medical records

It does not usually give someone control over money — that is a Financial Power of Attorney. And the named person cannot ignore the patient's wishes: the agent's job is to honor the person's known wishes, values, faith, goals, and best interests.

Capacity Matters — Do Not Wait

The best time to complete or update this document is while the person still has decision-making capacity — the ability to understand the decision, understand the consequences, and communicate a choice. If dementia has been diagnosed, do not assume it is automatically too late, but do not delay either — ask the doctor and an attorney what is appropriate. Once a person loses capacity, signing new legal documents may no longer be possible, and families may face harder options — such as asking a court to appoint a guardian — a slower, more expensive, and more public process. The door is easiest to open before it locks.

Choosing the Agent — and a Backup

The best healthcare agent is not always the loudest, closest, or oldest. It is someone who can:

- ☐ stay calm under pressure
- ☐ listen to doctors and ask good questions
- ☐ honor the person's wishes
- ☐ communicate with family
- ☐ be reachable
- ☐ make hard decisions without making themselves the center

Every Healthcare Power of Attorney should name a backup agent if possible; the first person may be sick, traveling, overwhelmed, or unreachable. Make sure each one knows they are named, understands the person's wishes, and has a copy.

The document matters, but so does the conversation. Do not make the agent guess in the hallway outside an ICU room — talk through what the person would and would not want while there is time.

When Family Disagrees

A Healthcare Power of Attorney can reduce conflict, but it may not erase emotion. Family members may still disagree or accuse the agent of giving up. The agent stays grounded in the person's wishes: "I hear that this is hard. My job is to follow what Mom wanted, not what is easiest for any of us."

Give Copies to the Right People, and Review It

Once complete, share copies with the healthcare agent, backup agent, primary doctor, hospital system, any facility, hospice or palliative team, main caregiver, and attorney, and put one in the important documents folder. Keep the original in a safe but accessible place — a document no one can find is a paper ghost. Review it after a move to another state, the death or unsuitability of the named agent, a change in relationships (marriage, divorce, widowhood), changed wishes, a serious diagnosis, progressing dementia, or an old or missing document. Do not assume an old form still does the job — ask an attorney whether it is valid and current.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and hospital records.
2. **Identify who is named** — the agent and the backup.
3. **Make sure the agent knows** and understands the responsibility.
4. **Have the wishes conversation** — what care would be wanted, and what would not.
5. **Update if needed** — if the document is missing or outdated, ask an attorney.

■ **Worksheet — Healthcare Power of Attorney** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ THE WISHES CONVERSATION (ASK WHILE THE PERSON CAN STILL ANSWER)

What matters most to you if you become seriously ill? _____

What kind of care would you want — and not want? _____

Where would you prefer to receive care if possible? _____

Who do you want involved in medical decisions? _____

What role should faith or spiritual beliefs play? _____

What would you want your family to remember if decisions get hard?

■ CONVERSATION SCRIPTS

With the person: "Who would you trust to make medical decisions if you cannot speak for yourself, and what would you want that person to know?"

With family: "This is not about control. It is about making sure the doctors know who is legally named to speak."

With the doctor or facility: "We have a Healthcare Power of Attorney. What is the best way to make sure it is added to the medical record?"

ROB'S NOTE

The Healthcare Power of Attorney is one of those documents families don't think about until the room gets serious. Then, all at once, everyone wants a clear answer.

Who can talk to the doctor? Who can make the decision? Who knew what Mom wanted? Who has the paper? Who is the backup?

Don't leave that to memory. Don't leave it to family fights. Don't leave it to the loudest person in the room. Put it in writing. Talk about it while there is still time.

The best healthcare agent isn't the person who wins the argument. It's the person who can carry out the wishes.

CHAPTER 20 TAKEAWAY

A Healthcare Power of Attorney names the person who can make medical decisions if your parent cannot. The next right question is: **who is legally named to speak for this person if they cannot speak for themselves?**

CARDINAL MOMENT



Three grown children stood in an ICU. A doctor gently asked who was allowed to speak for their father, who was unconscious. No one had ever been named. Scared and grieving, the siblings disagreed about almost everything, and the hospital couldn't just pick one of them.

The days that followed were harder than they needed to be. Not because they loved him less, but because one short form — the kind you could have signed on any ordinary afternoon years earlier — had never been signed.

Naming a healthcare agent isn't planning to die. It's deciding, calmly and ahead of time, who speaks for you on the day you can't speak for yourself.

A composite drawn from years at the bedside. Names and details changed.

Living Will / Advance Directive

THIS CHAPTER IN 30 SECONDS

- **What it does:** says what care a person would or would not want — CPR, ventilators, feeding tubes, comfort care. The Healthcare Power of Attorney names who speaks; have both.
- **What it does not do:** it usually applies only in situations defined by state law — often a terminal condition or permanent unconsciousness certified by physicians. Ask what triggers it.
- **Watch for:** it may state wishes about CPR, but a DNR, MOST, or POLST medical order may also be needed, depending on state and setting.
- **The step:** read it before the crisis, and share copies with the doctor, agent, facility, and care team.
- **How to ask:** *"Can you help us understand whether the advance directive matches the medical situation we are facing now?"*

WHAT FAMILIES USUALLY THINK

Families think, "We know what Mom would want," or "The Healthcare Power of Attorney is enough." The problem is not that families do not care — love does not remember clearly under pressure. One sibling says, "Mom would fight." Another says, "Mom said she never wanted machines." Both sincere, and both guessing if no one wrote it down.

What a Living Will May Cover

The Healthcare Power of Attorney answers **who speaks**; the Living Will or Advance Directive answers **what they would want**. Try to have both. Depending on the state and document, a Living Will may address:

- CPR
- ventilator use
- feeding tube and artificial hydration
- dialysis, surgery, antibiotics
- hospital transfer and intensive care

- comfort-focused care, palliative care, hospice
- organ donation
- spiritual preferences and preferred place of care
- what quality of life means, and what treatments would feel too burdensome

A Living Will usually applies only in specific medical situations defined by state law — often a terminal condition or permanent unconsciousness certified by physicians. Ask the attorney or doctor what triggers it.

CPR and Code Status

On television CPR looks quick and successful. Real life is different, especially for someone frail, seriously ill, or near the end of life. Ask the doctor: What would CPR look like for this person? What is the chance it would help? Would it lead to a ventilator or ICU stay? Would it match the person's goals? A Living Will may express wishes about CPR, but a medical order such as a DNR, MOST, or POLST may also be needed depending on the state and setting. Do not assume one form does everything.

Ventilators, Nutrition, and Hydration

Ventilators and intensive care. A ventilator helps someone breathe. For some it is temporary support during a reversible illness; for others it may prolong dying. Ask whether it is expected to be temporary, what happens if the person cannot come off it, whether they would want ICU care, and what comfort-focused care would look like instead.

Artificial nutrition and hydration. Feeding tubes and IV fluids feel personal — food means love, water means care. But near the end of life the body may not process food and fluids the same way. Ask what problem you are trying to solve, whether it would improve comfort or cause burden, what comfort feeding means, and what hospice or palliative care would recommend.

Comfort-Focused Care

A Living Will can help clarify when the goal shifts from cure to comfort. Comfort-focused care may include:

- pain, breathing, anxiety, and nausea relief
- mouth and skin care, gentle repositioning
- a quiet room, family presence
- spiritual support

- hospice, palliative care, and avoiding burdensome treatments
Comfort care is not "doing nothing." It is active care — with the noise turned down.

The Conversation, Memory, and Disagreement

Talk while the person can still answer: what matters most if they become seriously ill, what would feel like too much, and what treatments they would and would not want. Write the answers down; they do not have to be perfect, only honest.

If memory is changing, this becomes urgent — a person with early dementia may still be able to share wishes and sign, depending on capacity, but waiting can close the door. When family disagrees, the document returns everyone to what the person said.

Give Copies to the Right People, and Review It

Copies should go to the healthcare agent and backup, the primary doctor, the hospital system, any facility, the palliative or hospice team, the important documents folder, and the attorney. Keep the original somewhere safe but accessible. Review it if the person moved to another state, a serious diagnosis occurred, dementia is progressing, wishes or the named agent changed, the care setting changed, hospice was discussed, or the document is old. Ask an attorney or qualified legal resource whether it is valid, current, and appropriate for the state.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and hospital records.
2. **Read it before the crisis** — do not wait until the hospital asks.
3. **Write down what it says** — CPR, ventilators, feeding tubes, comfort care, hospice.
4. **Talk with the healthcare agent**, and share copies with the doctor, agent, facility, and care team.
5. **Update if needed** — if it is missing, outdated, or unclear, ask an attorney or legal resource.

■ **Worksheet — Living Will / Advance Directive** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE DOCTOR

CPR / code status

1. What would CPR likely look like for this person, and what is the chance it would help?
2. Would CPR likely lead to ICU care or a ventilator, and would it match the person's goals?
3. Is a DNR or medical order (MOST / POLST) appropriate, and does the current document match the situation?

Ventilator / feeding tube

1. Would a ventilator be temporary or likely long-term, and what happens if the person cannot come off it?
2. What problem would artificial nutrition or hydration solve, and would it improve comfort or cause burden?
3. What would comfort-focused care look like instead?

Comfort-focused care — what matters most (check all that apply)

- | | |
|--|---|
| ☐ Pain relief | ☐ Avoiding hospital transfer |
| ☐ Breathing comfort | ☐ Staying at home if possible |
| ☐ Anxiety relief | ☐ Hospice / palliative support |
| ☐ Quiet room / family present | ☐ Music, prayer, or familiar items |
| ☐ Spiritual support | |

■ CONVERSATION SCRIPTS

With the person: "We want to understand what kind of care you would want if there ever came a time when you could not speak for yourself. What would matter most to you?"

With family: "This conversation is not about giving up. It is about making sure we do not have to guess later."

With the doctor: "Can you help us understand whether the advance directive matches the medical situation we are facing now?"

ROB'S NOTE

Most families don't skip these talks because they don't care. They skip them because they care so much that it hurts.

But staying quiet doesn't protect anyone. It leaves the people you love standing in a hospital hallway. They have to make huge decisions while they're tired and heartbroken.

A Living Will or Advance Directive is not about choosing death. It is about being clear. It says, "Here is what matters to me."

That is a kindness — not only to the doctors, but to the family who would otherwise have to guess.

CHAPTER 21 TAKEAWAY

A Living Will explains what care a person would want if they could not speak for themselves. The next right question is: **what did Mom or Dad say they would want if the decision became hard?**

CARDINAL MOMENT



A son sat by his mother's bed. A doctor asked if they should keep her on the breathing machine. She had never written down what she wanted. She had never really said it out loud, either. So he made the kindest guess he could.

Later, it wasn't the grief that stayed with him. It was the guessing. Did he pick what she would have picked? Now there was no one left who could answer that.

A living will doesn't make the loss any smaller. But it makes sure the choice was hers. Then no one who loved her has to carry a guess for the rest of their life.

A composite drawn from years at the bedside. Names and details changed.

Financial Power of Attorney

THIS CHAPTER IN 30 SECONDS

- **What it does:** names who can manage money, bills, accounts, benefits, and property if your parent cannot.
- **What it does not do:** the money still belongs to the parent; the agent has a fiduciary duty to keep records and pay for the person's care.
- **Watch for:** confirm it is durable, and whether authority begins immediately or only after incapacity is documented.
- **The step:** have the agent ask each major institution what it requires to accept it.
- **Pause:** if Medicaid may be needed later, get advice before moving money, retitling assets, or giving gifts.

WHAT FAMILIES USUALLY THINK

Families think, "I'm her son, so the bank will talk to me," or "The Healthcare Power of Attorney covers money too." Informal help works for a while, then it doesn't. The facility may need an authorized signer. The family may agree emotionally, but the paperwork says no.

Healthcare POA vs. Financial POA

These are different documents. The Healthcare Power of Attorney covers who speaks to doctors and makes healthcare decisions; the Financial Power of Attorney covers who manages money, bills, accounts, benefits, and property. A family may need both; one person can hold both roles, or different people may suit each.

What It May Allow — and What It Does Not Mean

Depending on the document and state law, the agent may:

- pay bills; manage bank and retirement accounts; manage taxes
- handle insurance and pension paperwork; review mail
- pay caregivers; sign care agreements; work with accountants and attorneys
- apply for benefits; help with Medicaid paperwork; manage long-term care insurance claims; manage or sell property

Some powers may need to be specifically written; ask an attorney if the authority is unclear.

It does not mean the agent owns the money, can spend it on themselves, or can ignore the person's wishes; siblings keep the right to ask reasonable questions. The fiduciary duty: keep records, avoid mixing money, pay for the person's care, follow the document, avoid self-dealing, and get professional guidance.

Durable, Immediate, or Springing

Durable usually means the document stays effective even if the person becomes incapacitated. A non-durable power of attorney may stop working at incapacity, depending on state law and wording. Ask who decides incapacity and what proof is required.

Immediate vs. springing. Some documents work immediately after signing, so the agent can help while the person still has capacity. Others "spring" into effect only after incapacity is documented — protecting independence, but risking delays when institutions require specific proof.

Even with a valid document, banks may run their own review: an original or certified copy, internal forms, attorney review, an updated document, or identification for the agent.

When Memory Is Changing, and the Warning Signs

If memory is changing, this document becomes urgent. Financial mistakes happen quietly. Warning signs that help is needed:

- ☐ unpaid bills, late notices, mail piling up
- ☐ confusion about accounts; large unexplained withdrawals
- ☐ new "friends" asking for money; scam calls
- ☐ missed insurance premiums; unfiled taxes
- ☐ a parent defensive or embarrassed about money

These signs may mean the system is too heavy for one person, not that the parent is bad with money. Check documents now, before capacity is gone.

Choosing the Agent, Keeping Records, and Family Trust

The best financial agent is trustworthy, organized, good with paperwork, not financially dependent on the parent, and free of conflicts of interest. Do not choose someone because they are the oldest or the loudest. The agent should keep clear records — bills paid, care expenses, reimbursements, transfers, and benefit applications.

Money can cause conflict over spending, care, and inheritance. Avoid unnecessary secrecy: "The money belongs to Mom; my job is to use it for her care and keep clear records." Secrecy breeds suspicion.

THE ONE PLACE A POA DOES NOT WORK

The Social Security Administration does not accept a power of attorney — not even a durable one. To manage a parent's Social Security, the agent has to apply separately to become their **Representative Payee** (ssa.gov/payee, or 1-800-772-1213). The VA has its own version, called a **fiduciary**. Start these early: they take time, and the folder of paperwork you already have will not substitute for them.

Protecting Against Scams and Financial Exploitation

As thinking changes, the risk of being scammed goes up — often before anyone realizes memory is slipping. Scammers target older adults with money, home equity, and good manners; lost dollars are often the first visible sign of decline. Common patterns:

- ☐ **Impersonation** — a caller claiming to be the IRS, Medicare, Social Security, a bank "fraud department," or a utility, demanding payment or account details.
- ☐ **The grandchild in trouble** — "It's me, I'm in jail, don't tell Mom, I need money." Voice-cloning makes these convincing.
- ☐ **Fake charities** — phony veterans, police, or disaster-relief funds.
- ☐ **Tech support** — a pop-up says the computer is infected, then asks for remote access to the accounts.
- ☐ **Romance and "sweetheart" scams** — an online friend who, over weeks, becomes someone who needs money.
- ☐ **Prizes and sweepstakes** — you've won, but first you must pay a fee in gift cards or a wire — itself the tell.

Clark Howard has hammered one rule for years: **no legitimate business or government agency will ever ask to be paid in gift cards**. A caller who says "buy gift cards and read me the numbers" is a scam, every time.

GUARDRAILS THAT ACTUALLY HELP

You cannot listen to every call, but you can make fraud slower:

- **The "no decisions by phone, in the moment" rule.** No money, no gift cards, no account numbers on a call — anything real can wait for a trusted person.
- **Add a trusted contact to the accounts.** Banks and brokerages will call that person about unusual activity, without giving them control.

- **Consider a credit freeze** at all three bureaus (Equifax, Experian, TransUnion) — free, reversible, and it blocks new accounts in your parent's name.
- **Turn on account alerts** for withdrawals and large transactions, and **register at donotcall.gov** for call-blocking.
- **Set a family code word** so a panicked "it's your grandson" call can be checked.

If money has been lost, move fast: call the bank, report to the FTC at **reportfraud.ftc.gov** and to Adult Protective Services (your state's APS number is in the State Guide at the back), and keep every record. Being scammed is never something to be ashamed of.

PAUSE BEFORE MOVING MONEY

If Medicaid may be needed later, do not move money, retitle assets, give gifts, or add names to accounts without advice. A transfer can create Medicaid problems; a gift can become a penalty issue; adding someone to an account can create tax, ownership, creditor, or family problems. Ask an elder law attorney, Medicaid planner, financial advisor, accountant, or state resource first. Caregiving panic is a bad financial advisor.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — documents folder, safe, attorney files, bank records.
2. **Identify who is named** — the agent and a backup.
3. **Confirm whether it is durable.**
4. **Find out when authority begins** — immediately, or after incapacity is documented?
5. **Check whether banks will accept it** — the agent should ask each institution.
6. **Update** if it is missing, outdated, or unclear — ask an attorney.

■ **Worksheet — Financial Power of Attorney** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ CONVERSATION SCRIPTS

With the person: "We want to make sure bills, care costs, insurance, and benefits can be handled if there is ever a time when you cannot manage them yourself. Who do you trust to help with financial matters?"

With family: "This is not about taking control. It is about making sure someone has legal authority to pay bills, manage care costs, and keep records if Mom or Dad cannot."

With the bank: "We have a Financial Power of Attorney. What is your process for reviewing and accepting this document?"

ROB'S NOTE

A Financial Power of Attorney is not about greed. It is about groceries, bills, care, insurance, and keeping the lights on.

It makes sure someone can pay the care home, call the insurance company, and take care of the bank account. It also protects the person from money mix-ups or harm.

Money can stir up strong feelings in families. That's why the paperwork matters. The person in charge should be someone you trust. The records should be clean. The reason for it should stay clear.

The money belongs to the person getting care. The job is to use it for their care, safety, dignity, and needs.

CHAPTER 22 TAKEAWAY

A Financial Power of Attorney names the person who can handle money matters if your parent cannot. The next right question is: **who is legally authorized to manage bills, accounts, benefits, and care costs if this person cannot?**

CARDINAL MOMENT



A daughter couldn't pay her father's house payment. All the accounts were in his name. After his stroke, he could no longer sign anything. The teller was kind, but the bank couldn't just take her word for it.

To fix it, she had to go to guardianship court. That meant months of waiting and thousands of dollars. One signed form, done while he was still well, would have taken care of it in one quiet afternoon.

A financial power of attorney is cheap and easy to forget today. But it is worth everything the day you have to keep the lights on for a parent who can't sign anymore.

A composite drawn from years at the bedside. Names and details changed.

Medical Orders — DNR, MOST, and POLST

THIS CHAPTER IN 30 SECONDS

- **What it does:** a living will states wishes; a signed medical order — an out-of-hospital DNR, or a portable order form such as a MOST or POLST — gives instructions that care teams, including EMS, can act on; paramedics generally cannot follow a living will sitting in a folder.
- **What it is not:** a DNR is not "do not treat" — it addresses one thing, CPR; comfort, pain relief, dignity, and medical care all continue, and the order can be changed or voided at any time.
- **Watch for:** this chapter's worksheet is not a DNR, MOST, or POLST and has no legal or medical effect — only an order signed by a healthcare provider on your state's official form directs medical care.
- **How to ask:** *"Which portable medical order form does our state use, and is it appropriate for my parent now?"*
- **Where it lives:** keep the original visible — on the refrigerator, by the bed, or at the front of the chart — let it travel with the person, and review it at every care transition.

WHAT FAMILIES USUALLY THINK

Families often think a DNR means "do not treat," that signing one is giving up, that these forms are for every older adult, or that the living will already covers it. None of that is right. A DNR addresses one thing — CPR. Portable order forms are meant for people who are seriously ill or frail — someone whose death within the coming year would not surprise their clinician — not every healthy 70-year-old, who should usually stop at an advance directive. The order can be changed or voided at any time by the patient (or their surrogate, within state rules), and should be reviewed at every care transition.

What Is a DNR?

A Do Not Resuscitate (DNR) order tells healthcare providers not to perform CPR (cardiopulmonary resuscitation) if a person's heart stops or they stop breathing. CPR can include chest compressions, electric shocks, and breathing support. A DNR does not affect

other treatments unless specified — a person with a DNR still receives medical care, comfort care, and treatment for everything else.

What Is a MOST? What Is a POLST?

A **MOST** (Medical Orders for Scope of Treatment) form turns a person's preferences into signed medical orders across a range of treatments — CPR, hospitalization, antibiotics, feeding tubes, and comfort care. Some states use the MOST; many others use a **POLST** — historically "Physician Orders for Life-Sustaining Treatment," now moving to "Portable Orders for Life-Sustaining Treatment" nationally — which does the same job. Names, colors, and signature rules vary by state (in many states a nurse practitioner or physician assistant may sign).

Where the Form Lives Matters as Much as What It Says

These forms only work if they can be found in the moment. At home, the original — often a brightly colored form — is usually kept where responders look: on the refrigerator, by the bed, or at the front of the care binder. In a facility, it belongs at the front of the chart. And it should **travel with the person** — to the hospital, to rehab, to assisted living — and be reviewed at every transition and after any major change in health or goals. Have the conversation early — with the provider and with each other — so the care team has clear instructions when there is no time to talk.

WHAT TO DO THIS WEEK

1. **Ask whether a medical order is appropriate now** — these forms are for serious illness and frailty; a healthy parent usually needs only the advance directive.
2. **Find out which form your state uses** — ask the provider, or check Your State Guide at the back of this book.
3. **Have the code-status conversation with the doctor** — ask what CPR would realistically look like for this person.
4. **Get it signed and get copies** — the original stays visible with the person; copies go to the documents folder, the facility, and the decision maker.
5. **Tell the people who would call 911** — family, caregivers, and facility staff should know the order exists and where it is.
6. **Review it at every care transition** — and remember it can be changed or voided at any time.

■ **Worksheet — Thinking About Your Preferences** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK THE DOCTOR ABOUT CODE STATUS

1. Given my parent's health, what would CPR realistically look like, and what is the chance it would work?
2. Is my parent someone who should have a portable medical order (MOST/POLST) now, or is the advance directive enough?
3. Which form does our state use, who signs it, and how do we complete it?
4. Where should the original live, and who needs copies?
5. What happens if my parent changes their mind — how is the order changed or voided?
6. Will this order follow my parent to the hospital, rehab, or a facility — and who checks it at each transition?

■ CONVERSATION SCRIPT

With the doctor: "We want my parent's wishes to be followed in an emergency, not just written in a folder. Is it time for a portable medical order — in our state, I understand that may be a MOST or POLST — and can we complete it together?"

With family: "A DNR is not 'do not treat.' It is one instruction about CPR. Everything else — comfort, medicine, dignity, care — continues. We are making sure the paramedics know what Mom wants, so nobody has to guess."

ROB'S NOTE

The word DNR scares people. It can sound like a door slamming shut.

But a DNR does not mean "do not treat." It is one order about CPR. Comfort, pain relief, dignity, and care all keep going.

These orders exist so no one has to guess in the hardest moment. The paramedic, the nurse, and the family all know what the person wanted. Talk to the doctor early. Ask what CPR would really mean for this person. Then make the order match their wishes.

That is not giving up. It makes sure love has the last word, not panic.

CHAPTER 23 TAKEAWAY

A DNR, MOST, or POLST works only if it is signed, current, and findable. The next right question is: **if the heart stops tonight, is there a signed order that says what to do, and can the responders find it?**

CARDINAL MOMENT



A woman told everyone who would listen that she never wanted to be brought back with CPR. But one day she collapsed at home. The paramedics had to try everything, because there was no signed order anywhere in the house.

Her family knew exactly what she wanted. But the people who came through the door had no way to know. What she wanted lived in talks with her family. What the crew could follow had to be on paper.

Wishes live in conversations. Medical orders live on the refrigerator, signed by a doctor. A paramedic is only allowed to follow one of those two.

A composite drawn from years at the bedside. Names and details changed.

Advance Care Planning Conversations

THIS CHAPTER IN 30 SECONDS

- **What it is:** the conversation behind the documents — what matters most, who speaks, what care is wanted and not.
- **What it is not:** one conversation forever — ask again after a diagnosis, hospitalization, or change in health.
- **How to start:** with values, not machines: *"I want to understand what matters most to you if your health changes."*
- **Watch for:** wishes are not orders — a DNR, MOST, or POLST may still be needed; no one form does every job.
- **If family disagrees:** ask "What would our loved one choose if they could speak clearly today?"

Why These Conversations Matter

Families delay because it feels uncomfortable — they do not want to scare Mom, or make Dad think they are giving up. Then comes the fall, the stroke, the doctor asking about code status. A hard time to start from zero. Advance care planning lets the family understand the person's wishes before emotions are loud and decisions urgent.

WHAT FAMILIES USUALLY THINK

Families think, "We know what she wants," "The forms are enough," or "Everyone will agree." Maybe. But in a crisis, memory gets shaky — one person remembers one conversation, another something different. Someone wants everything done, someone wants comfort, someone says, "You are giving up on her." Planning does not remove grief. It reduces confusion.

What to Talk About

Cover:

- ☐ what matters most
- ☐ who should make medical decisions and who gets updates

- ☐ what care feels acceptable and what feels too burdensome, and where they would prefer to receive it
- ☐ whether they would want hospitalization, ICU care, CPR, a ventilator, or artificial nutrition and hydration — or hospice and comfort-focused care
- ☐ what spiritual or faith needs matter, and what the family should remember if decisions get hard

It does not have to happen at once. One honest question opens the door.

When to Have the Conversation

The best time is before the emergency:

- ☐ while the person can still speak for themselves
- ☐ before major surgery, and after a serious diagnosis or hospitalization
- ☐ when dementia or memory changes begin
- ☐ when falls, weakness, or chronic illness worsen, or a move to assisted living or memory care is coming
- ☐ before hospice or palliative care is needed

What someone wanted at 65 may not be what they want at 85, and living with dementia or cancer can change the answer. Ask again.

Include the people who need to know: the person receiving care, the healthcare agent and backup, the spouse, adult children, the main caregiver, the doctor, and the hospice or spiritual team if involved. The first conversation can be quiet and private, but the person named to speak should know what they are being asked to carry.

How to Start

Start gently. You need honesty, not a conference room. Try: "I want to make sure we understand what matters most to you if your health changes." Or: "If there ever came a time when you could not speak for yourself, what would you want us to know?" Or: "I know this is not easy to talk about, but I do not want us guessing later." Do not start with machines. Start with values; the medical details come later.

Wishes vs. Orders, and When It Is Hard

Wishes are not orders. A conversation guides, a legal document records wishes, a medical order instructs medical professionals. Someone can write in an advance directive that they do not want CPR and still need a DNR, MOST, or POLST, depending on the state and setting. Ask the doctor: "Do the documents we have match the medical orders needed for this situation?"

If the person refuses to talk, the family has not failed. Try smaller questions later. Instead of "Do you want CPR?" ask, "What matters most to you if you get sicker?" Instead

of "Do you want hospice?" ask, "Would you want care focused on comfort if treatment was no longer helping?" Instead of "Where do you want to die?" ask, "Where do you feel safest?" Some doors open slowly. Do not kick them down.

If family members disagree, shift from "What do I want?" to "What would our loved one choose if they could speak clearly today?" That single question lowers the temperature in many family conflicts. Stay anchored: "Let's come back to what Mom said mattered most." "This is not about what we want. It is about what she told us." The families with the least regret are the ones who knew what their loved one wanted.

WHAT TO DO THIS WEEK

1. **Choose one calm time** — not mid-crisis or mid-argument.
2. **Start with the person receiving care**, if they can participate.
3. **Ask one values question** — "What matters most to you if your health changes?" — and write down the answer.
4. **Identify the decision maker** — who speaks if the person cannot.
5. **Check which documents exist** — POA, directive, medical orders — and share the key wishes with the healthcare agent and main caregiver.

■ **Worksheet — Advance Care Planning Conversation** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE DOCTOR

1. Do the current documents match the person's medical situation, and are any medical orders needed?
2. Would a DNR, MOST, or POLST be appropriate?
3. What would CPR likely look like for this person, and what would hospitalization likely accomplish?
4. Would palliative care be helpful, and would hospice ever be appropriate?
5. What should the family understand now?

■ CONVERSATION SCRIPTS

To begin: "I want to understand what matters most to you if your health changes, so we are not guessing later."

If the person is nervous: "This does not mean anything is happening today. It just means your wishes matter enough to write them down."

With family: "We need to listen to what Mom or Dad wants, not just what each of us fears."

With the doctor: "Can you help us understand what medical decisions we should talk about now, before there is a crisis?"

ROB'S NOTE

Advance care planning is not a talk about death. It is a talk about love.

It says, "Your voice matters, even if one day you can't use it." It tells the family what to do when things get hard. It gives the healthcare agent something to hold onto besides guilt.

The goal is not to guess every medical problem ahead of time. You can't. The goal is to understand the person. What matters to them? What scares them? Who do they trust? What do they hope for? What would they want the family to remember?

Start there. The forms matter. But the talk is the heart of it.

CHAPTER 24 TAKEAWAY

Advance care planning is the conversation that tells your family what this person would want before anyone has to guess. The next right question is: **have we talked honestly about what matters most, who should speak, and what care would match this person's values?**

CARDINAL MOMENT



A father kept putting off "the talk." Not yet, not yet, he said, every time his children asked what he wanted at the end. There was always a better day for it. Then one morning, a stroke took that chance away.

They spent his last months guessing at everything he never got to tell them. Again and again, they wished they had just gotten through one hard dinner while there was still time.

The talk is never easy, and the timing is never right. Have it anyway, and have it early. Do it while the person can still tell you, in their own words, what matters to them.

A composite drawn from years at the bedside. Names and details changed.

Medication and Doctor Lists

THIS CHAPTER IN 30 SECONDS

- **What it is:** one current list of everything actually being taken — prescriptions, over-the-counter drugs, vitamins, supplements, drops, patches, insulin — plus allergies and recently stopped medications.
- **Why it matters:** it prevents confusion, delays, duplicate prescriptions, missed doses, and unsafe combinations. The test: can we hand the care team one accurate list today?
- **Bring the list, not just the bottles:** a bottle shows what was prescribed, not what is actually being taken.
- **One owner:** choose one medication-list person to keep it updated — if three people have three different lists, the family does not have a list.
- **How to ask:** *"Can we review the medication list to make sure everything is still needed, safe, and appropriate?"* Do not stop medications on your own.

Why the Medication List Matters

A medication list tells the care team what is actually being taken — not what was prescribed three years ago, not what one sibling remembers. It matters at doctor appointments, ER visits, hospital admissions, rehab and home health starts, hospice, and assisted living or memory care move-ins. Include prescriptions, over-the-counter medications, vitamins, supplements, creams, inhalers, eye drops, insulin, patches, and as-needed medications. If it goes in, on, under, or near the body for medical reasons, write it down.

WHAT FAMILIES USUALLY THINK

Families often think, "Mom knows what she takes," "The pharmacy has the list," or "We will bring the bag of bottles." But medication lists are often wrong. Doctors change medications, specialists add them, hospitals stop them, pharmacies refill old prescriptions, and patients keep taking discontinued pills. Over-the-counter drugs, supplements, eye drops, and inhalers drop out of the conversation. The medication list needs one owner. Someone has to keep it clean.

What Belongs on the List

For each medication, write:

- ☐ the name
- ☐ dose
- ☐ how often and what time
- ☐ the reason (if known)
- ☐ the prescribing doctor
- ☐ the pharmacy
- ☐ the start date (if known)
- ☐ special instructions
- ☐ whether it is daily or as-needed
- ☐ any side effects or concerns

Also include:

- ☐ allergies and medication reactions
- ☐ vitamins, supplements, and over-the-counter drugs
- ☐ pain, sleep, and anxiety medications
- ☐ blood thinners
- ☐ diabetes medications
- ☐ inhalers, eye drops, creams, patches, insulin, and oxygen
- ☐ medical marijuana or CBD if used and legal in the state
- ☐ any medication recently stopped

The stopped medications matter too — a discontinued medication sitting in the cabinet has a way of coming back.

Bottles alone are not enough. A bottle shows what was prescribed, not what is actually being taken: the dose may have changed, the medication may have been stopped, and duplicates may come from different pharmacies. Bring the bottles when useful, but bring the list every time.

When to Update, and the Medication Review

Update the list:

- ☐ after every doctor appointment, hospital visit, or rehab discharge
- ☐ when home health or hospice starts
- ☐ when a medication is added, stopped, or a dose changes
- ☐ when the pharmacy changes

- ☐ after a fall or worsening confusion, dizziness, weakness, sleepiness, or appetite change
- ☐ after a new diagnosis
- ☐ before an assisted living or memory care move

Do not wait for the annual checkup.

Ask for a medication review if the person takes many medications, has new confusion, is falling, feels dizzy, sleeps more, or has multiple prescribers. Ask: "Can we review the medication list to make sure everything is still needed, safe, and appropriate?" Do not stop medications on your own.

The Doctor List, and One Owner

The doctor list is the medication list's twin — one shows what is being taken, the other who is involved. Include the primary care provider, specialists, pharmacy, hospital preference, home health, hospice, palliative team, facility nurse, private duty agency, equipment company, dentist, eye and hearing providers, therapist, and care manager, with each name, role, phone, portal info, last visit, next appointment, and main reason involved. In a crisis, "the heart doctor" is not enough — write the name down.

Choose one medication-list person to keep the list updated and share the newest version. This does not mean one person does all the caregiving — it means one person guards the list. Choose someone organized and reachable, and share the list with the main caregiver, healthcare agent, doctor, facility, and anyone taking the parent to appointments.

WHAT TO DO THIS WEEK

1. **Gather all medications** — bottles, pill packs, inhalers, drops, creams, patches, supplements, and OTC drugs.
2. **Write down what is actually being taken** — ask how each medication is really used, not just what the bottle says.
3. **Add the pharmacy name and phone number.**
4. **List every doctor and care contact** — specialists, facility, home health, hospice, and pharmacy.
5. **Ask for a medication review** for safety, duplicates, side effects, and cost.
6. **Put the list in the documents folder** — paper and digital — and update it whenever something changes.

■ **Worksheet — Medication and Doctor List** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ APPOINTMENT QUESTIONS

1. Is this medication list accurate, and are any medications duplicated or no longer needed?
2. Could any medication increase fall risk, worsen confusion, or cause dizziness or sleepiness?
3. Are there lower-cost alternatives?
4. Are there medications that should not be taken together?
5. Should any medications be taken at a different time?
6. What should we do if a dose is missed?

HOPE'S NOTE — FROM THE PHARMACIST

When a family hands me a current, complete medication list, the whole visit changes — we catch duplicates, interactions, and doses that no longer fit the person. When they hand me a bag of bottles, I am guessing about what is actually being taken.

Bring the list. Update it every time something changes. And use one pharmacy if you can — it is the only way any pharmacist ever sees the whole picture.

ROB'S NOTE

A good list of medications can save a life.

It keeps your parent from having to guess. It keeps the doctor from missing something. It keeps the caregiver from panicking. And it keeps the family from standing in the emergency room, trying to remember if the little white pill was for blood pressure, sleep, worry, or pain.

Write it down. Keep it up to date. Bring it with you.

The list doesn't have to look nice. It just has to be right.

CHAPTER 25 TAKEAWAY

A current medication and doctor list makes every decision safer and faster. The next right question is: **can we hand the care team one accurate list today?**

CARDINAL MOMENT



An ER doctor asked a daughter what medicines her mother took. All she could say was "a white one, and a little blue one." Not knowing cost hours. And it forced a risky guess at the worst possible time.

After that night, she kept an up-to-date list in her wallet and on her phone. It had every medicine, every dose, and every doctor. The next emergency visit took minutes instead of hours, because the answer was already in her hand.

An up-to-date list of medicines and doctors is the most useful page you can carry. Update it the same day anything changes, not the day you need it.

A composite drawn from years at the bedside. Names and details changed.

Emergency Contacts and Crisis Plan

THIS CHAPTER IN 30 SECONDS

- **What it is:** one page that answers the crisis question — who do we call first?
- **Call 911** for immediate danger or a life-threatening emergency. If you are unsure, call 911.
- **Otherwise the first call may be** the doctor or nurse triage line, hospice, home health, or the facility.
- **Build the call chain:** one family point person, with a backup, updates everyone else.

Why This Matters

Contact information is usually scattered — the hospice line in a folder, a sibling out of town who has no idea who to call. Then something happens, and the family loses time.

WHAT FAMILIES USUALLY THINK

"We'll just call 911." "The facility will handle it." Sometimes 911 is the right call. Sometimes the doctor, hospice, facility nurse, or home health team should be called first, and calling 911 first can create confusion. Waiting too long creates danger. The family needs a plan — not a perfect one, a usable one.

When to Call 911

CALL 911 IMMEDIATELY

For immediate danger or a life-threatening emergency: chest pain, severe trouble breathing, signs of stroke, uncontrolled bleeding, major injury, unconsciousness, a severe allergic reaction, a suspected overdose, a serious fall with injury, a person who cannot be safely moved, immediate danger to self or others, or fire, violence, or an unsafe environment. **If you are unsure whether something is life-threatening, call 911. Do not let a checklist delay emergency help.**

When to Call the Doctor, Hospice, Home Health, or Facility

The doctor or nurse triage line for problems that are serious but not clearly life-threatening — fever, new weakness, worsening confusion, medication side effects, new swelling or pain, dehydration, a fall without obvious major injury, or whether the ER is needed. Say: "Something has changed, and I need guidance on what to do next."

Hospice, if the person is on it, for pain, shortness of breath, agitation, nausea, breathing changes, not eating or drinking, or a sense that death may be near — unless there is immediate danger requiring 911. Say: "We are on hospice and something has changed. I need the nurse or on-call team to guide us." Ask who to call after hours and post that number.

Home health for the plan of care, wounds, therapy, nursing visits, medication teaching, equipment, or safety concerns. It is not an emergency service — for immediate danger, call 911.

The facility, if the person lives in assisted living, memory care, rehab, or a nursing home. Ask who to call during the day and after hours, who the nurse and administrator are, who calls the doctor, what changes require family notification, and what changes require 911. Get it in writing.

The Family Call Chain

Choose one family point person to update everyone else, and a backup. The plan should answer: who calls the doctor or agency? Who goes to the hospital? Who stays with the spouse? Who brings the document folder? Who handles medications, pets, home, or transportation? In a crisis, roles beat opinions.

What to Keep Ready, and What to Say

The emergency page should include:

- ☐ the person's full name, date of birth, and address
- ☐ main diagnosis and allergies
- ☐ where the medication list is
- ☐ doctor names and numbers
- ☐ pharmacy
- ☐ insurance information
- ☐ the healthcare agent
- ☐ family, facility, home health, and hospice contacts
- ☐ preferred hospital
- ☐ DNR/MOST/POLST status if applicable

- ☐ funeral home if chosen
- ☐ the document folder location

Print it and store it digitally.

When you call, use the crisis call script below.

If the Caregiver Breaks Down

If the caregiver is in crisis right now, call or text 988 — the Suicide & Crisis Lifeline, answered any hour of any day. Caring for someone counts as a crisis of your own.

A caregiver crisis is still a crisis. Call for help if the caregiver:

- ☐ has not slept
- ☐ feels unsafe
- ☐ cannot lift or transfer the person
- ☐ cannot manage medications
- ☐ feels emotionally overwhelmed
- ☐ is afraid they may yell or collapse
- ☐ cannot leave the person alone
- ☐ has their own medical emergency
- ☐ is no longer able to provide safe care

Use this script: **"The caregiver cannot safely continue at this level. We need help making a safer plan today."** That is not weakness. That is honesty.

WHAT TO DO THIS WEEK

1. **Choose the family point person** and a backup.
2. **Write down the medical contacts** — doctor, specialists, pharmacy, facility, home health, hospice.
3. **Write down when to call 911.**
4. **Put the contact page somewhere visible** — refrigerator, caregiver binder, or hospital bag.
5. **Build the call chain** — who calls whom, who does what.
6. **Share the plan** with the caregiver, point person, healthcare agent, and facility.

■ **Worksheet — Emergency Contacts & Crisis Plan** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

■ CRISIS CALL SCRIPT

"My name is _____. I am calling about _____, who is _____ years old and is located at _____. The main diagnosis is _____. Something changed: _____. It started around _____. Current symptoms are _____. The medication list is at _____. I need guidance on what to do next."

ROB'S NOTE

A crisis plan won't feel like a big deal until the crisis comes. Then it's worth its weight in gold.

The number on the fridge. The hospice line by the phone. The doctor's name written clearly. The one family member already picked to lead. The medication list in the folder.

That's what helps a family move. Not because everything is easy, but because the first step is clear.

When people are scared, they don't need more noise. They need a number, a name, and the next right call.

CHAPTER 26 TAKEAWAY

An emergency contact and crisis plan puts every number a family needs in one findable place. The next right question is: **can the caregiver find the right number in thirty seconds?**

CARDINAL MOMENT



When their father fell at two in the morning, the brothers and sisters found out that none of them knew who to call first. They didn't know which hospital he should go to. And they didn't know where he had written down that he never wanted to be put on machines. So they figured it out as they went, badly, in the dark.

Later, when things were calm, they wrote one page. It had the numbers, the hospital, the medicines, and his wishes. They taped it inside the kitchen cabinet where anyone could find it. The next scare, they weren't guessing.

A crisis is the worst time to plan for a crisis. Write the one page while it's quiet. The scared version of you at 2 a.m. will thank you.

A composite drawn from years at the bedside. Names and details changed.

STAY CONNECTED

For caregiver tools, updates, and Rob's writing, visit
thecardinalspromise.com.

The Last Days Plan

Part 5 — Hospice and the Last Days. This is the most tender part of the guide. These chapters walk with a family through the final season — what the last days may look like, what comfort means, what to expect and what to do, and what happens after death. The goal is not to control death or predict its timing. It is to help the family show up — calmer, clearer, and more present — for the person they love. If someone is in immediate danger and emergency treatment is wanted, call 911.

“You matter because you are you, and you matter to the end of your life.”

— DAME CICELY SAUNDERS, FOUNDER OF THE MODERN HOSPICE MOVEMENT

THIS CHAPTER IN 30 SECONDS

- **What it is:** who to call first, what comfort means, where the documents are, who should be present, and what to do when death occurs.
- **What it is not:** predicting the moment. It is preparing the family so the room can be calmer, softer, and more focused on the person.
- **Comfort medications:** never guess — let the care team teach the family, and write down the instructions.
- **Food and fluids:** do not force them; ask the care team about comfort feeding.
- **If family disagrees:** return to what the person wanted and what keeps them most comfortable now.

Why This Plan Matters

Families often wait until the final days to ask the hardest questions: Who should be called? Who has the hospice number? Where is the DNR? What happens when death occurs? Without a plan, fear fills the room. With a plan, the family still grieves, but they are not guessing every minute. A last days plan gives everyone something steady to hold.

WHAT FAMILIES USUALLY THINK

Families often think, “We should call 911 if breathing changes,” or “If they stop eating, we are starving them.” These thoughts come from fear and love. But fear and love need guidance.

The last days plan helps turn panic into care.

Comfort Becomes the Priority

In the last days a person may sleep more, eat and drink less, weaken, and breathe differently. Ask the hospice nurse, doctor, or facility nurse what signs to watch for and what to do. Do not rely on internet panic. As the body changes, comfort becomes the main goal:

- pain relief and breathing comfort
- anxiety and restlessness support
- mouth care, repositioning, skin care
- a quiet room, dim lights, familiar music, prayer
- family presence, welcome touch, no unwanted transfers

Comfort care is not doing nothing. It is doing the right things when the body is changing.

Food, Fluids, and Breathing Changes

Food and fluids. Families struggle when a loved one stops eating or drinking, because food and water are love. But near the end of life appetite decreases and food and fluids may no longer help the person. Ask the care team whether it is safe to offer food or fluids, whether to offer small sips or mouth swabs, and what comfort feeding means. **Do not force food or fluids.** Offer comfort. Mouth care often becomes more helpful than pushing intake.

Breathing changes. When breathing changes, call the care team and ask what is expected, what may help, and when the nurse should come. If the person is on hospice, call hospice; if they are not and there is immediate danger or the family wants emergency treatment, call 911. Know ahead of time who to call first, whether you are trying to avoid hospital transfer, and whether there is a DNR/MOST/POLST. A breathing change should not be the first time the family asks these questions.

Comfort Medications

Discuss the fear of comfort medication directly with the nurse or doctor. Ask what each medication is for, when and how much to give, what to watch for, what to do if the person cannot swallow, and how to know the person is comfortable. **Never guess with comfort medications.** Let the care team teach the family, and write down the instructions.

The Room, the People, and What to Say

Who should be there. The plan should include who to call if time becomes short — spouse, children, siblings, close friends, clergy or the hospice chaplain, and anyone who needs a chance to say goodbye. Not everyone needs to be in the room at once. Sometimes the most loving room is quiet; sometimes it is full. Ask what the person would want.

What the room should feel like. A room can be cared for too. Consider music or quiet, prayer, dim lights, the TV off, familiar photos or blankets, and soft voices. Remember the person may still hear or sense presence. Speak with respect. Tell stories. Say what needs to be said. Do not argue in the room — the room is not the place for family politics.

What to say. There is no perfect script. Simple words are enough: "I love you." "Thank you." "I forgive you." "Please forgive me." "We are here." "You are not alone." "We will take care of each other." "You can rest." Not every family can say every sentence. That is okay. Say what is true. Say what brings peace.

If Family Disagrees, and When Death Occurs

If family members disagree in the last days — one wants the hospital, another comfort at home — return to the person's wishes. Ask what they said they wanted, what documents exist, and who the healthcare decision maker is. Use this script: "This is hard for all of us. Let's come back to what Mom wanted and what keeps her most comfortable now." The goal is not to win the argument. The goal is to protect the person.

When death occurs, the family should already know what to do. Ask ahead: Who do we call first — hospice, the facility, 911? Who pronounces death? Is a funeral home selected, and who calls them? Who notifies family, and who stays with the body until the funeral home arrives? What happens to medications and equipment? What spiritual or cultural wishes should be honored? Write it down before the moment comes.

WHAT TO DO THIS WEEK

1. **Ask what signs to expect** from the nurse or doctor.
2. **Confirm who to call first** — hospice, facility nurse, doctor, or 911.
3. **Review documents** — POA, advance directive, DNR/MOST/POLST.
4. **Choose the family point person** to update family and coordinate calls.
5. **Decide who should be notified** before the final hours.
6. **Ask the nurse or doctor to explain** comfort medications and the death process.

■ **Worksheet — Last Days Plan** — fill it in in **The Workbook** at the back of this book, p. 209
· printable copies at TheCardinalsPromise.com/resources

ROB'S NOTE

The last days are sacred. They are also practical.

Someone needs the number. Someone needs the medication instructions. Someone needs to know who to call. Someone needs to know where the papers are. And someone needs to tell the family.

That doesn't make the moment any less holy. It makes it more protected.

The goal isn't to control death. The goal is to care well until it comes. Quietly. Clearly. Together.

CHAPTER 27 TAKEAWAY

A last days plan prepares the family for comfort, calls, documents, visitors, and what to do when death occurs. The next right question is: **what can we put in place now so the final days are more peaceful?**

CARDINAL MOMENT



A family knew the end was close. But no one had ever said out loud where they wanted it to happen. So when the moment came, they panicked and called 911. Their mother spent her last two days in an ambulance and in the emergency room she had begged never to go back to.

What hurt them afterward wasn't that she died. It was where she died. It was a place she had asked not to be, all because there was no plan that could have fit on an index card.

The last days will come, plan or no plan. A plan is simply how you make them hers, instead of the system's.

A composite drawn from years at the bedside. Names and details changed.

What to Expect in the Last Days

THIS CHAPTER IN 30 SECONDS

- **What families may see:** more sleeping, less eating and drinking, breathing changes, cool hands or feet, restlessness, visions, sometimes a rally — every person is different. Hearing may remain: speak as if they can hear.
- **Food is love, but:** offer, do not force. The family is not starving the person.
- **Breathing changes:** noisy breathing does not always mean suffering — but still call the care team. If on hospice, call hospice.
- **Comfort medications:** use them only as directed — ask what each is for and when to call.

Why Families Need to Know

The body changes in the last days, and those changes look frightening if no one explains them. Knowing what to expect does not remove grief, but it can reduce panic and help the family stay present, call the right person, and focus on comfort.

WHAT FAMILIES USUALLY THINK

Families often think, "If they stop eating, they will starve," "If breathing changes, we should call 911," "If they cannot talk, they cannot hear us," "If we give comfort medicine, we are speeding things up," or "If they rally, they are getting better." Some come from love, some from fear, most from not knowing. Families should never be left guessing — ask the care team what is expected and what needs immediate attention.

Sleeping, Eating, and Drinking Less

More sleeping. The person may sleep much more, awake only briefly. That is hard when you want one more long talk, but sleepiness is often part of the body slowing down. Sit nearby, speak softly, avoid forcing alertness, tell them who is present, and say what needs to be said. Do not assume they cannot hear.

Eating less. The person may refuse meals, take only bites, or cough. Food is love, but near the end of life the body may not need or tolerate it, and forcing food can cause distress, choking, or aspiration. Ask the care team what is safe to offer and what comfort feeding

means. Offer, do not force; use small amounts; stop if coughing or distress occurs. The family is not starving the person. The illness is changing the body.

Drinking less. The person may stop asking for fluids, have trouble swallowing, or cough with liquids. Often mouth care becomes more helpful than forcing fluids. Ask whether sips or thickened liquids are safe. Use mouth swabs if recommended, apply lip balm, and do not force fluids.

Breathing Changes

Breathing changes are often the most frightening part for families. Breathing may become slower, faster, uneven, shallow, noisy, paused, wet-sounding, or labored-looking. (Families sometimes hear noisy, wet breathing called "the death rattle" — unsettling to hear, but usually not a sign of suffering.) Call the care team when breathing changes — if the person is on hospice, call hospice. Ask whether it is expected, whether the person appears uncomfortable, whether repositioning, comfort medicine, or oxygen would help, and what signs mean urgent help is needed. Stay calm, reposition gently if instructed, use comfort medications only as directed, and speak softly.

Skin, Circulation, and Urine

Cool hands and feet. Hands, feet, arms, or legs may become cool, and skin may look pale, gray, bluish, or mottled as circulation changes. Use light blankets if comfortable, do not use heating pads unless instructed, and tell the nurse what you see. The goal is comfort, not warming the body back to normal.

Less urine. Urine may decrease and darken as intake drops. Call the nurse if there is no urine for a long time, signs of discomfort, bladder fullness, fever, pain, agitation, or catheter concerns. Keep the person dry and clean.

Restlessness, Visions, and Pain

Restlessness or agitation. Some people become restless, pull at blankets, try to get up, call out, or seem anxious. Ask the care team whether it could be pain, anxiety, urinary retention, constipation, or medication. Stay calm, reduce noise, keep the person safe, offer reassurance, and use medications only as directed. Restlessness is not a family failure; it is a symptom to report and manage.

Confusion or visions. Some people talk to loved ones who have died, reach toward something, or speak about going home. Often correction is not needed. Listen, reassure,

avoid arguing, do not shame, and let the moment be peaceful. Sometimes the kindest response is: "You are safe. We are here with you."

Pain. Do not assume pain is unavoidable. Signs may include grimacing, moaning, restlessness, guarding, a tense body, faster breathing, or a furrowed brow. Ask what medication is ordered, when to give it, what to do if they cannot swallow, and when to call. Pain management is a central goal of comfort care.

The Rally, and Hearing

The rally. Sometimes a person has a period of unexpected alertness — they wake, talk, ask for food, or recognize people, and the family feels hopeful. It does not always mean recovery. Receive the moment, say what matters, let them rest afterward, and avoid making sudden major decisions based on one good hour. A rally can be a gift. Let it be one.

Hearing may remain. Even when a person cannot respond, they may still hear or sense presence. Speak as if they can hear: "We are here." "You are safe." "We love you." "Thank you." "You can rest." Avoid arguing in the room, and avoid talking over the person as if they are already gone. The room should protect their dignity.

When to Call the Care Team

Call when:

- ☐ breathing changes
- ☐ pain seems uncontrolled
- ☐ restlessness increases
- ☐ the person cannot swallow medications
- ☐ the family is unsure what to do
- ☐ there is a fall or bleeding
- ☐ there is fever or distress
- ☐ skin changes are concerning
- ☐ the caregiver is overwhelmed
- ☐ death seems near
- ☐ death has occurred

If on hospice, call hospice. If not on hospice and emergency treatment is wanted or needed, call 911. If unsure, call for guidance. Families should not sit alone with unanswered fear.

1. **Ask what changes to expect** from the nurse, doctor, hospice, or palliative team.
2. **Ask what symptoms require a call** — write down when and who to call.
3. **Review comfort medications** — what each is for and how to give it safely.
4. **Prepare the room** with comfort items, music, blankets, photos, or prayer items.
5. **Prepare the family** — tell key members what changes may happen.
6. **Write down the death process** — who to call, who pronounces, who calls the funeral home.

■ FIELD GUIDE — LAST DAYS SIGNS & COMFORT

A companion to the **Chapter 27 Last Days Plan** — keep the two together. Care team numbers, comfort medications, who to call first, what to say, and the after-death steps all live on that plan. This page is for what you are seeing and what helps. It does not replace medical advice — call the care team with concerns, and 911 for immediate danger if emergency treatment is wanted or needed.

Common changes to watch (check what is happening)

- | | |
|--|--|
| ☐ Sleeping more / less talking | ☐ Cool hands or feet / skin color changes |
| ☐ Less eating / less drinking | ☐ Less or darker urine |
| ☐ Trouble swallowing | ☐ Pain signs |
| ☐ More weakness or confusion | ☐ Seeing or talking to people not present |
| ☐ Restlessness or agitation | ☐ Short rally or burst of alertness |
| ☐ Breathing changes / noisy breathing | |

Eating, drinking & mouth care

Food instructions from the care team _____

Fluid instructions _____

Swallowing concerns? (yes / no / unsure) _____

Mouth care instructions _____

Supplies needed: Mouth swabs ☐ Lip balm ☐ Small spoon ☐ Thickened liquids (if ordered) ☐
Towels ☐

Breathing, pain & restlessness plan

QUESTION	WHAT THE CARE TEAM TOLD US
What breathing changes may happen, and what means discomfort?	
What position or medication may help breathing?	

How do we know if the person is in pain, and what is ordered?	
What might cause restlessness, and what should we try first?	
When should we call?	

Room comfort (check what would help)

- | | |
|---|---|
| ☐ Quiet | ☐ Familiar scent |
| ☐ Soft music | ☐ TV off |
| ☐ Prayer / scripture / readings | ☐ Limited visitors |
| ☐ Dim lights | ☐ Open visiting |
| ☐ Favorite blanket / family photos | |

If death seems near

What signs suggest death may be close? (ask the care team) _____

Who to call, who pronounces, the funeral home, and what happens with medications and equipment: see your Chapter 27 plan.

■ IF THE FAMILY IS UNSURE — SCRIPT

"We are seeing changes and we are not sure what they mean. Can you help us understand what is expected, what may be discomfort, and what we should do next?"

ROB'S NOTE

The last days can look different than families expect. That doesn't mean the family is doing something wrong. The body is changing, and the work changes too.

Less fixing. More comforting. Less pushing. More listening. Less panic. More being there.

Call the care team. Ask about what you're seeing. Write down what they tell you. Then come back to the person in the bed. Hold a hand. Speak softly. Protect the room.

The family doesn't have to understand every sign perfectly. They just need to know who to call, what comfort looks like, and that being there still matters.

CHAPTER 28 TAKEAWAY

The last days may bring changes in sleep, eating, drinking, breathing, skin, and awareness. The next right question is: **what are we seeing, who should we call, and what comfort support is needed now?**

CARDINAL MOMENT



A daughter was scared by the change in her father's breathing. The long pauses. The cooling hands. The deep sleep. She was sure he was suffering, and that she was failing him by not doing more.

The hospice nurse sat with her and gently explained what was normal and what it meant. Once she understood what she was seeing, her panic turned into presence. She stopped fighting it and simply stayed by his side.

Fear usually comes from not knowing what is normal. When you understand what the last days really look like, you can stop bracing and start being there.

A composite drawn from years at the bedside. Names and details changed.

After Death

THIS CHAPTER IN 30 SECONDS

- **The most important thing:** there is usually time to pause, sit, and say goodbye — and the right first call is rarely 911.
- **If on hospice:** call hospice first, not 911 — say "*We believe our loved one has died.*" In many states the hospice nurse can pronounce the death.
- **If not on hospice:** call the facility staff, or the doctor for an expected home death; with no plan, 911 may be necessary — keep the DNR or POLST visible.
- **Take your time in the room:** ask how much time you have; it is often more than families expect.
- **The paperwork after:** name one point person and let the rest wait.

Why This Matters

The moments after a death are a strange mix of the holy and the practical. The family is grieving, and yet calls have to be made and a funeral home has to come. Families without a plan often call 911 by reflex and are swept into a chaotic scene the person never wanted. Knowing the steps ahead of time lets the family stay in the room longer, and keeps the first hours from becoming a second trauma.

WHAT FAMILIES USUALLY THINK

Families often think they must call 911 immediately, that the body must be moved right away, or that they cannot touch or sit with their loved one. In an expected death with hospice or a plan in place, none of that is usually true. Calling 911 can bring paramedics and sometimes police, and turn a peaceful bedside into an emergency scene. Know in advance who to call.

If the Person Was on Hospice

If the person was on hospice, **call hospice first — not 911**. A nurse will come to confirm the death — in many states the hospice nurse can pronounce it, or will arrange for it — and the team will notify the physician and the funeral home, dispose of medications properly, and arrange for equipment. Keep the hospice after-hours number where it can be

found, and when you call, simply say, "We believe our loved one has died." The team takes it from there.

If the Person Was Not on Hospice

In a facility, the staff will usually know the process — they will notify the nurse and physician and help contact the funeral home. At home with an expected death and a doctor who will sign the death certificate, call that doctor or an on-call line. If the death was unexpected, or there is no plan, calling 911 may be necessary. That may bring emergency responders — and, in some areas, law enforcement — and may lead to resuscitation attempts unless a valid DNR or POLST is visible. This is why the documents in Part 4 should be easy to find.

Taking Your Time in the Room

Once the calls are made, or while you are deciding, it is okay to stay. Many families find comfort in sitting with their loved one, holding a hand, praying, playing music, or simply being present. Some bathe or dress the person, or gather others so everyone can say goodbye. Ask the hospice nurse or funeral home how much time you have — it is often more than families expect. This is a moment you do not get back. If it helps you to linger, linger.

When the family is ready, the funeral home is called to take the person into their care. If one was chosen ahead of time, this call is simple; if not, the hospice or facility can help. When they arrive, family members may stay or step away — some want to walk their loved one to the door, some cannot watch, and both are acts of love. The funeral home will explain the next steps, including when to make arrangements and what documents they will need.

The First Days of Practical Tasks

In the days after, practical tasks arrive:

- ☐ obtaining copies of the death certificate (get several; many institutions require an original)
- ☐ meeting with the funeral home
- ☐ notifying Social Security
- ☐ contacting pension and insurance companies
- ☐ notifying banks and closing or changing accounts
- ☐ canceling appointments and services
- ☐ beginning to sort belongings

None of this has to happen at once. Choose one point person, and let the rest wait. The paperwork can almost always wait a little longer than it feels like it can.

WHAT TO DO — IN THE FIRST HOURS

1. **Pause.** Unless the situation requires otherwise, there is time.
2. **Call the right first number** — hospice, the facility staff, the doctor for an expected home death, or 911 only if there is no plan or the death was unexpected.
3. **Have the DNR or POLST visible** if responders may come.
4. **Stay as long as you need.** Sit, hold a hand, gather family.
5. **Call the funeral home when ready** — hospice or the facility can help.
6. **Name one point person** for the calls and paperwork.

■ **Worksheet — After-Death Plan** — fill it in in **The Workbook** at the back of this book, p. 209 · printable copies at TheCardinalsPromise.com/resources

ROB'S NOTE

The first thing I tell families after a death is simple: you don't have to hurry.

The world will tell you to move fast. Make the calls. Sign the forms. Clear the room. But most of the time, the person you love isn't going anywhere in the next hour. And neither are you. You're allowed to stay. You're allowed to sit in the quiet. You're allowed to say the last things.

I've watched families panic and call 911 without thinking. Then they stand in a hallway while strangers work over a body that was ready to rest. It doesn't have to be that way. That's what the plan is for.

I wasn't with my father when he died. That is the reason this book exists. It's the reason this book exists. So hear me: if you're given the hour I missed, you won't need a checklist to tell you to hold a hand. The paperwork can wait. Make the one call that matters. Then go back and be a son, a daughter, or a spouse. Not a coordinator. The rest will keep.

The Weeks After

If hospice was involved, their bereavement team stays with your family — typically for about thirteen months — with calls, mailings, and grief groups at no charge. You can say yes to it months later, even if you said no at first. Grief has a long tail: the fog, the sleeplessness, the anger, the moment you forget and reach for the phone — none of it means you are going crazy. It means you loved someone. If the weight does not begin to

lift, or you simply want company inside it, a grief group or counselor is not a luxury. Ask the hospice, your doctor, or your faith community — any month, in any year.

CHAPTER 29 TAKEAWAY

After a death, there is usually more time than families think, and the right first call is rarely 911. The next right question is: **who do we call first, and what can wait until we are ready?**

CARDINAL MOMENT



In the first hour after her mother died, a daughter stood in the quiet. She had no idea what she was supposed to do. Call 911? Call the funeral home? Call no one? She felt like she was already failing at something. She didn't even know what.

The hospice nurse had left one sheet of paper. It said who to call, in what order, and what could wait until morning. That sheet let her stop managing the moment. At last, she could just grieve her mother.

In the first hours, almost everything can wait longer than you think. The few things that can't should already be written down. That way, you're free to just be their child.

A composite drawn from years at the bedside. Names and details changed.

KEEP GOING

You don't have to do this alone. Find more support — and Rob's memoir *The Cardinal's Promise* — at thecardinalspromise.com.

About the Author



Rob Brizzi is a Certified Dementia Practitioner who has spent his career in home health, hospice, senior living, and other healthcare settings — years of sitting with families in hospital hallways, facility lobbies, and living rooms as they faced the decisions in this book. He is the founder of The Cardinal's Promise, an education effort helping families navigate aging, dementia, caregiving, serious illness, and loss, and the author of the forthcoming memoir *The Cardinal's Promise*. He wrote this guide after the death of his father, Lou Brizzi — the man who taught him that showing up is most of the job.

A Note of Thanks

The guidance in this book on protecting older adults from scams and financial exploitation was motivated in part by the consumer-protection work of **Clark Howard**, whose plain rule — that no legitimate business or agency will ever ask to be paid in gift cards — has helped countless families recognize a scam before the money is gone.

Learn more, find caregiver tools, and reach the author at thecardinalspromise.com.

ALSO BY ROB BRIZZI

The Cardinal's Promise

Rob's memoir — the story of being adopted, of loss, and of learning to show up for the end of life. It's the story behind this guide. Coming soon; follow along and find free caregiver tools at thecardinalspromise.com.

Your State Guide

Almost every office in this book has a version in your state — same job, different name. This page shows you how to find each one. Programs and websites change; when in doubt, start with the Eldercare Locator (1-800-677-1116), whose entire job is connecting you to your local agencies.

WHAT THIS BOOK CALLS IT	HOW TO FIND YOUR STATE'S VERSION
State Medicaid office	Medicaid.gov → "Contact your state," or search "[your state] Medicaid long-term care." Applications run through your county or state social services office — names vary ("Social Services," "Human Services," "Job and Family Services") — or your state's online portal.
SHIP — free, unbiased Medicare counseling	shiphelp.org or 1-877-839-2675. Every state has one; many use their own name (SHIP, SHIBA, HICAP, and others).
Area Agency on Aging	Eldercare Locator, 1-800-677-1116 or eldercare.acl.gov — it routes you by ZIP code.
Adult Protective Services	Find yours through the National Adult Protective Services Association directory (napsa-now.org) or the Eldercare Locator — every state has an APS program, and many run a statewide hotline.
Portable medical order form (Chapter 23)	Ask the doctor which form your state uses — POLST in many states, MOST in others (polst.org lists every state's program and form).
Advance directive forms (Chapters 20–21)	In most states the attorney general, health department, or bar association publishes free statutory forms; an elder law attorney can confirm what your state requires (witnesses, notarization).
Veterans services (Chapter 18)	Your county Veterans Service Officer (search "[your county] veterans service officer") or VA.gov; every state also has a Department of Veterans Affairs.
Assisted living / facility inspection records	Each state's licensing agency publishes inspection reports — search "[your state] assisted living inspection reports" — and Medicare's Care Compare (medicare.gov/care-compare) rates nursing homes and home health agencies nationally.

Quick-start guides — Rob's home states. Because this work is rooted in the Southeast, this edition names the actual offices for four states. Numbers were current at press time;

confirm before relying on them, and note the state your parent lives in is what governs — not where you live.

North Carolina

WHAT YOU NEED	WHO TO REACH IN NORTH CAROLINA
Medicaid & long-term care	Apply at epass.nc.gov or your county Department of Social Services. NC Medicaid Contact Center 1-888-245-0179 .
Free Medicare counseling (SHIP)	SHIIP — Seniors' Health Insurance Information Program (NC Dept. of Insurance), 1-855-408-1212 .
Report abuse, neglect, exploitation	Your county Department of Social Services ; the NC DHHS CARE-LINE, 1-800-662-7030 , will route you.
Aging services / Area Agency on Aging	NC Division of Aging & Adult Services, ncdhhs.gov/aging , or the Eldercare Locator, 1-800-677-1116 .
Portable medical order (Ch. 23)	North Carolina uses the MOST form (Medical Orders for Scope of Treatment) — ask the doctor.
Advance directive (Ch. 20–21)	NC Health Care Power of Attorney + Advance Directive for a Natural Death; free statutory forms and the state registry at the NC Secretary of State (sosnc.gov).
Veterans (Ch. 18)	NC Dept. of Military & Veterans Affairs (milvets.nc.gov) or your county Veterans Service Officer.

Florida

WHAT YOU NEED	WHO TO REACH IN FLORIDA
Medicaid & long-term care	Apply through MyACCESS — myaccess.myflfamilies.com (the only official address ends in myflfamilies.com) or 1-866-762-2237 . Long-term care runs through the Statewide Medicaid Managed Care program.
Free Medicare counseling (SHIP)	SHINE — Serving Health Insurance Needs of Elders (Dept. of Elder Affairs), Elder Helpline 1-800-963-5337 .
Report abuse, neglect, exploitation	Florida Abuse Hotline, 1-800-962-2873 (1-800-96-ABUSE), any hour.
Aging services / Area Agency on Aging	Florida Dept. of Elder Affairs, Elder Helpline 1-800-963-5337 (elderaffairs.org).

Portable medical order (Ch. 23)	Florida uses a state DNRO — Do Not Resuscitate Order, the yellow DH Form 1896 — not POLST. Ask the doctor.
Advance directive (Ch. 20–21)	Florida Living Will + Designation of Health Care Surrogate (Fla. Stat. Ch. 765); free forms from Florida Health and the Florida Bar.
Veterans (Ch. 18)	Florida Dept. of Veterans' Affairs (floridavets.org), 1-844-693-5838.

Virginia

WHAT YOU NEED	WHO TO REACH IN VIRGINIA
Medicaid & long-term care	Virginia Medicaid — Cardinal Care — apply through Cover Virginia, 1-855-242-8282 or commonhelp.virginia.gov , or your local Dept. of Social Services.
Free Medicare counseling (SHIP)	VICAP — Virginia Insurance Counseling & Assistance Program (DARS), 1-800-552-3402 .
Report abuse, neglect, exploitation	Virginia Adult Protective Services, 1-888-832-3858 , 24 hours.
Aging services / Area Agency on Aging	Dept. for Aging & Rehabilitative Services (DARS), 1-800-552-3402 (vda.virginia.gov).
Portable medical order (Ch. 23)	Virginia uses the POST form (Physician Orders for Scope of Treatment); the National POLST form is also accepted (virginiapost.org).
Advance directive (Ch. 20–21)	Virginia Advance Directive (Code § 54.1-2984); free forms from the Virginia State Bar and virginiaadvancedirectives.org .
Veterans (Ch. 18)	Virginia Dept. of Veterans Services (dvs.virginia.gov).

South Carolina

WHAT YOU NEED	WHO TO REACH IN SOUTH CAROLINA
Medicaid & long-term care	Healthy Connections (SC Dept. of Health & Human Services) — apply at apply.scdhhs.gov ; Resource Center 1-888-549-0820 .
Free Medicare counseling (SHIP)	I-CARE — Insurance Counseling, Assistance & Referrals for Elders (SC Dept. on Aging), 1-800-868-9095 .
Report abuse, neglect, exploitation	1-888-227-3487 (1-888-CARE4US), SC Dept. of Social Services — confidential, any hour.

Aging services / Area Agency on Aging	SC Dept. on Aging, 1-800-868-9095 (aging.sc.gov) or GetCareSC.com.
Portable medical order (Ch. 23)	South Carolina uses the POST form (Physician Orders for Scope of Treatment) — ask the doctor.
Advance directive (Ch. 20–21)	South Carolina uses two documents — a Declaration of a Desire for a Natural Death (living will) and a Health Care Power of Attorney .
Veterans (Ch. 18)	SC Dept. of Veterans' Affairs (scdva.sc.gov) or your county Veterans Affairs Office.

More states are added as the Cardinal's Promise Series grows — with every program, office, and form for your state named and explained — at thecardinalspromise.com. (Virginia, it turns out, calls its Medicaid program Cardinal Care. We take that as a good sign.)

Find Your Worry

Not sure where to start? Find the worry that sounds like yours and turn to the page. The dot shows how urgent it usually is — ■ act today ■ watch closely ■ plan ahead.

When something feels off

■ Something feels off, but I can't name it.	p. 32
■ Mom keeps falling.	p. 37
■ Dad isn't steady on his feet.	p. 37
■ She's repeating herself and forgetting names.	p. 44
■ Is this normal aging, or dementia?	p. 44
■ I'm exhausted, angry, and I feel guilty.	p. 50
■ I can't do this anymore.	p. 50
■ My siblings won't help — or we can't agree.	p. 58

When you need care

■ Dad needs help at home but won't move.	p. 67
■ I need someone to help with bathing and meals.	p. 73
■ Mom can't live alone anymore.	p. 80
■ Dad wanders and isn't safe with the stove.	p. 86
■ The hospital is discharging her and says she needs rehab.	p. 91
■ Is Mom "admitted," or under "observation"?	p. 91
■ They say she's "plateaued" and therapy is ending.	p. 91
■ She's in pain but not dying — what helps?	p. 96
■ The doctor said "six months."	p. 101

When you're paying for it

■ How will we pay for any of this?	p. 107
■ What does Medicare actually cover?	p. 107
■ We're running out of money for the nursing home.	p. 113
■ Do we have to spend everything first?	p. 113
■ We'll be paying out of our own pocket.	p. 119
■ Mom has a long-term care policy — how do we use it?	p. 124
■ Her medications cost too much.	p. 129

■ Dad's a veteran — is there help?	p. 135
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When you're getting the papers in order

■ I can't find any of the paperwork.	p. 140
■ Who can make medical decisions if she can't?	p. 144
■ What treatment does she want at the end?	p. 149
■ Who handles the money and the bills?	p. 155
■ I think someone is scamming my parent.	p. 155
■ Do we need a DNR? What's a POLST?	p. 161
■ I don't know how to start the hard conversation.	p. 166
■ I can't keep track of her medicines and doctors.	p. 171
■ What do I do in an emergency — who do I call?	p. 176

When the end is near

■ The end is close — what's the plan?	p. 181
■ What will the last days actually look like?	p. 186
■ She's gone. What do I do now?	p. 192

Glossary

Plain-language meanings for the terms families run into most — with the page where each is used.

Activities of Daily Living (ADLs) p. 73

The basic things a person does each day to care for themselves — eating, bathing, dressing, using the toilet, and getting around. How much help someone needs with these usually decides what kind of care they need.

Adult Protective Services (APS) p. 155

A state agency that looks into reports that an older or disabled adult is being hurt, neglected, or taken advantage of. Call them if you think someone is being harmed.

Advance Directive p. 149

A paper that says, ahead of time, what medical care a person does or does not want if they cannot speak for themselves. It usually includes a living will and a health care power of attorney.

Anosognosia p. 44

When a person with dementia truly cannot see their own decline — the illness hides itself from them. It is not denial or stubbornness; the brain reports that everything is fine. Arguing the facts rarely works; aim for comfort and safety instead.

Anticipatory Grief p. 50

Mourning someone who is still alive — grieving who they used to be while you care for who they are now. Normal, common, and not disloyalty. Naming it is how caregivers stop mistaking it for guilt.

Area Agency on Aging p. 113

A local office that helps older adults and families find services like meals, rides, in-home help, and caregiver support. Free to call.

Assisted Living p. 80

A place where seniors live in their own room or apartment and get help with daily tasks like meals, medicine, and bathing — but not full nursing care.

Do Not Resuscitate (DNR) p. 161

A doctor's order not to do CPR (restart the heart) if the person's heart or breathing stops. It only covers CPR, not other care.

Financial Power of Attorney p. 155

A legal paper that lets a trusted person handle money, bills, and accounts for someone who cannot. "Durable" means it still works after the person becomes too sick to manage.

Extra Help p. 129

A Medicare program that helps people with low income pay for their prescription drug costs.

Guardianship p. 155

When a court names someone to make decisions for an adult who no longer can — and had no power of attorney in place. It takes months and money; good paperwork ahead of time usually avoids it.

Home Health

p. 67

Skilled care (nursing or therapy) given at home for a limited time, often after a hospital stay — though Medicare does not require one. Not the same as everyday help.

Hospice

p. 101

Comfort care for someone expected to live about six months or less. It eases pain and supports the family instead of trying to cure the illness. Usually fully covered by Medicare.

Living Will

p. 149

A paper that spells out what treatment a person wants, or does not want, near the end of life — like breathing machines or feeding tubes.

Long-Term Care

p. 124

Ongoing help with daily living for months or years — at home, in assisted living, or in a nursing home. Regular insurance and Medicare usually do not pay for it.

Medicaid

p. 113

A government program that pays for health care and long-term care for people with low income and few savings. Run by each state, with different rules.

Medicaid Look-Back

p. 113

When you apply for Medicaid to cover a nursing home, the state reviews money or property given away during a look-back period — five years in most states, though a few differ. Gifts in that window can delay coverage.

Medicare

p. 107

The federal health insurance for people 65 and older (and some younger people with disabilities). It covers doctors and hospitals but not most long-term care.

Medicare Advantage

p. 107

Part C — a private plan that delivers your Medicare benefits instead of Original Medicare. You are still in Medicare. Usually a lower monthly premium, in exchange for a network of doctors, prior authorization, and copays that add up in a bad year.

Memory Care

p. 86

A kind of assisted living made for people with dementia — secured doors and staff trained for memory loss.

Observation Status

p. 107

When a hospital keeps a patient but calls it “observation” instead of “admitted.” This can quietly leave the family paying for the rehab that follows. Always ask which status your parent is under.

Palliative Care

p. 96

Care that eases pain and stress from a serious illness. Unlike hospice, you can have it at any stage — even while still seeking a cure.

MOST / POLST / POST

p. 161

A doctor’s order on a bright form that says what emergency treatment a seriously ill person wants. It travels with them, and emergency crews are meant to follow it — but the form’s name and the rules for honoring it vary by state, so ask which form your state uses.

Health Care Power of Attorney

p. 144

A legal paper that names the person who can make medical decisions for someone who cannot speak for themselves.

Private Duty Care

p. 73

Help you pay for yourself (or with long-term care insurance) — an aide for bathing, meals, or overnight care. Medicare usually does not cover it.

QIO (Quality Improvement Organization)

p. 91

The group you can call for a fast, free appeal if a hospital or rehab tries to send your parent home too soon.

Skilled Nursing Facility (SNF)

p. 107

A place that gives short-term nursing and rehab, often right after a hospital stay. Medicare may pay for a limited time — but only if the hospital “admitted” the patient.

SHIP (State Health Insurance Assistance Program)

p. 107

Free, unbiased help in every state for understanding Medicare choices. Not selling anything.

Trust

p. 155

A legal setup that holds money or property for someone. An “irrevocable” trust cannot be easily changed and is sometimes used, well ahead of time, to protect assets — always with an attorney.

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The Workbook

Every worksheet from this guide, together in one place and in chapter order. Write directly in these pages — that is what they are for. Need a fresh copy, or one for each sibling? Print any of them free at **TheCardinalsPromise.com/resources**. Start by filling in the names, doctors, and contacts once on the first sheet; the rest of the worksheets refer back to it.

■ **WORKSHEET — FUTURE PLANNER SNAPSHOT**

Who can speak for my parent if they cannot speak for themselves?

Where are the documents, insurance cards, passwords, and medication lists?

What would my parent want if life changed quickly?

What are the three calls we should make before a crisis?

■ WORKSHEET — THE FAMILY MEETING

Bring this to the meeting and fill it out as you talk.

Date of meeting _____

Person receiving care _____

Main caregiver _____

Family point person _____

Backup point person _____

Next meeting date _____

Why are we meeting?

What changed? What is unsafe? What decision cannot wait? _____

What matters most to the person receiving care? _____

People in the meeting

NAME	RELATIONSHIP	ROLE IN CARE

Decisions & task assignments

DECISION / TASK	PERSON RESPONSIBLE	DUE DATE
Call doctor / provider		
Update medication list		
Locate / update documents		
Research care options		
Give caregiver a break		

Documents to find or update

- ☐ Healthcare Power of Attorney
- ☐ Financial Power of Attorney
- ☐ Living Will / Advance Directive

- ☐ DNR / MOST / POLST (if applicable)
- ☐ Medication List & Doctor List

☐ Insurance / Medicare / Medicaid / VA
information

☐ Funeral home information

☐ Emergency contacts

This week's three actions

1. _____

2. _____

3. _____

Family agreement

We are writing this down so one person does not have to carry everything alone. The goal is not a perfect plan. It is to make the next week safer and clearer.

Main caregiver _____

Family point person _____

Family member / helper _____

Date _____

■ WORKSHEET — HOME HEALTH EVALUATION

Use this before a provider visit or home health evaluation.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Preferred home health agency (if any) _____

Insurance _____

What changed? (check all that apply)

- | | |
|---|---|
| ☐ Fall or near fall | ☐ Medication concern |
| ☐ New weakness or trouble walking | ☐ Increased confusion |
| ☐ Trouble transferring | ☐ Shortness of breath or pain |
| ☐ Recent hospitalization or rehab stay | ☐ Difficulty bathing, dressing, or toileting |
| ☐ Recent surgery | ☐ Caregiver unable to manage safely |
| ☐ New wound | |

Current safety concerns

CONCERN	YES / NO / UNSURE	NOTES
Safe walking		
Safe transfers		
Safe bathing & toileting		
Medication safety		
Wound or skin concern		
Breathing / swallowing concern		
Confusion or memory concern		
Safe to be alone		

Current equipment in the home (check what applies)

- | | |
|-------------------------------------|---|
| ☐ Cane | ☐ Shower chair |
| ☐ Walker | ☐ Grab bars |
| ☐ Wheelchair | ☐ Raised toilet seat |

☐ Bedside commode

☐ Medical alert device

☐ Hospital bed

☐ Medication organizer

☐ Oxygen

If home health starts

Agency name & phone _____

Start of care date _____

Primary nurse / therapist _____

Visit schedule _____

DISCIPLINE	NAME	PHONE
Nurse		
Physical therapist		
Occupational therapist		
Speech therapist		
Social worker		
Home health aide		

■ WORKSHEET — PRIVATE DUTY CARE PLANNING

Use this before calling agencies or hiring help.

Person receiving care _____

Main caregiver / phone _____

Family point person / phone _____

Care schedule manager / phone _____

Current care needs

NEED	HOW OFTEN?	WHO DOES IT NOW?
Bathing / dressing / toileting		
Meals		
Laundry / cleaning		
Transportation / errands		
Medication reminders		
Companionship / supervision		
Overnight safety		
Dementia support		
Caregiver respite		

Highest-risk times of day (check all that apply)

- | | |
|------------------------------------|--|
| ☐ Morning | ☐ Weekends |
| ☐ Midday | ☐ Shower days |
| ☐ Afternoon | ☐ Appointment days |
| ☐ Evening | ☐ When caregiver is working |
| ☐ Overnight | ☐ When caregiver needs rest |

Best schedule to try first (check one)

- | | |
|---|--|
| ☐ 2–4 hours, 1–2 days per week | ☐ Weekend respite |
| ☐ Morning help several days per week | ☐ Overnight care |
| ☐ Evening help several days per week | ☐ Daily care |

☐ 12-hour shifts

☐ 24-hour care

Preferred start date _____

Preferred days & times _____

Minimum hours needed _____

Caregiver preferences

Preferred caregiver gender (if any) / language _____

Dementia experience needed? (yes / no / unsure) _____

Transfer or mobility help needed? (yes / no / unsure) _____

Transportation needed? (yes / no / unsure) _____

Pet & household notes _____

Agency comparison

QUESTION	AGENCY 1	AGENCY 2	AGENCY 3
Agency name & phone			
Hourly rate			
Minimum hours			
Weekend / overnight rate			
Background checks?			
Dementia training?			
Backup coverage?			
Care plan provided?			
Start date available			

■ WORKSHEET — ASSISTED LIVING TOUR & DECISION

Use this to compare communities and decide whether the current home plan is still safe. Tour at least three if you can.

Person receiving care _____

Main caregiver / phone _____

Family point person / phone _____

Current living situation _____

Preferred move-in timeframe / budget range _____

Is the current home plan still working? (check all that apply)

- | | |
|--|---|
| ☐ Living alone no longer feels safe | ☐ The caregiver is exhausted |
| ☐ Falls or near falls are happening | ☐ Private duty care is too expensive or hard to manage |
| ☐ Meals are inconsistent | ☐ The person is isolated or lonely |
| ☐ Medication is being missed or taken incorrectly | ☐ Driving is no longer safe |
| ☐ Bathing or hygiene is not happening regularly | ☐ A spouse caregiver cannot safely continue |

What does the person need help with?

NEED	NO HELP	SOME HELP	A LOT
Bathing / dressing / grooming			
Toileting / incontinence care			
Walking / transfers			
Meals			
Medication			
Housekeeping / laundry			
Transportation			
Nighttime support			
Memory support			

Community comparison

WHAT ACTUALLY DECIDES IT	COMMUNITY 1	COMMUNITY 2	COMMUNITY 3
Name & phone			
Base monthly cost			
Care level cost — and what makes it go up			
What is included, and what is billed on top			
Awake staff overnight? Nurse on site or on call?			
Memory care on site, if it is needed later?			

Five rows, three communities. That is enough to choose. Room type, move-in dates, pets, transportation, and whether hospice can come in all matter — but they rarely decide between two places, so ask them on the tour instead of tracking them here. The care level cost is the row families most often skip and most often regret: ask what it is today, what moves someone to the next level, and what that costs.

■ WORKSHEET — MEMORY CARE DECISION

Use this when dementia is creating safety concerns and the family is deciding whether memory care should be explored.

Person receiving care _____

Main caregiver / phone _____

Family point person / phone _____

Diagnosis (if known) _____

Primary doctor / neurologist / phone _____

Why are we considering memory care? (check all that apply)

- | | |
|--|---|
| ☐ Wandering or leaving home | ☐ Refusing care |
| ☐ Getting lost | ☐ Unsafe anger or agitation |
| ☐ Unsafe cooking | ☐ Suspicion or paranoia |
| ☐ Medication mistakes | ☐ Cannot call for help reliably |
| ☐ Unsafe driving | ☐ Caregiver exhaustion |
| ☐ Falls or near falls | ☐ Assisted living or home care is not enough |
| ☐ Nighttime confusion | |

Dementia safety snapshot

SAFETY AREA	SAFE	UNSAFE	NOTES
Medication			
Cooking			
Wandering / exits			
Bathing / toileting			
Eating & drinking			
Walking / falls			
Nighttime safety			
Ability to call for help			
Caregiver safety			

Wandering & nighttime notes

Has the person left or tried to leave home? (yes / no / unsure) _____

When does it happen, and where are they trying to go? _____

Is the person confused or unsafe at night? (yes / no / unsure) _____

Is the caregiver sleeping? (yes / no / unsure) _____

Behavior & triggers

Behaviors causing concern _____

Things that calm the person _____

Things that upset the person _____

Best time of day / hardest time of day _____

Memory care community comparison

QUESTION	COMMUNITY 1	COMMUNITY 2	COMMUNITY 3
Community name & phone			
Base monthly / care level cost			
Secure exits & outdoor space?			
Staff dementia training?			
Overnight staffing			
Nurse on site / on call			
Behavior support process			
Fall & medication process			
Family-communication process			
Care needs they cannot manage			

■ WORKSHEET — REHAB & DISCHARGE PLANNING

Bring this to the care conference and update it as discharge approaches.

Person receiving care _____

Facility & phone _____

Discharge planner / social worker _____

Expected discharge date _____

Recommended level of care after discharge _____

Before vs. now

ABILITY	BEFORE HOSPITAL	NOW
Walking (distance / help needed)		
Transfers (bed, chair, toilet)		
Bathing & dressing		
Toileting / continence		
Stairs		
Memory / confusion		
Eating & drinking		

Care conference questions (check when answered)

- ☐ What has improved, and what has not?
- ☐ What help does the person still need?
- ☐ What is the discharge goal and date?
- ☐ What setting is safest next?
- ☐ What equipment will be needed?
- ☐ Will a home health evaluation be arranged?
- ☐ Is private duty care needed?
- ☐ What training does the family need before discharge?
- ☐ What are the options if home is not safe?

Discharge plan

TASK	PERSON RESPONSIBLE	DUE DATE
------	--------------------	----------

Confirm next care setting		
Arrange equipment		
Set up home health evaluation (if appropriate)		
Arrange private duty care (if needed)		
Gather medication & doctor lists		
Schedule follow-up appointments		

■ WORKSHEET — PALLIATIVE CARE CONVERSATION

Use this to prepare for a palliative care conversation, ask better questions, and write down what matters most.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Main diagnosis or illness _____

Specialists involved _____

Why are we considering palliative care? (check all that apply)

- | | |
|--|---|
| ☐ Serious illness is harder to manage | ☐ Family members disagree about the plan |
| ☐ Pain or shortness of breath is difficult to control | ☐ Quality of life is declining |
| ☐ Fatigue or weakness is increasing | ☐ Caregiver stress is increasing |
| ☐ Appetite or weight loss is a concern | ☐ Advance care planning is needed |
| ☐ Repeated hospitalizations or ER visits | ☐ Hospice has been mentioned, but family is unsure |
| ☐ Treatment decisions are becoming confusing | |

Current symptoms (check any present)

- | | |
|--|--|
| ☐ Pain | ☐ Anxiety or depression |
| ☐ Shortness of breath | ☐ Confusion |
| ☐ Fatigue / weakness | ☐ Trouble sleeping |
| ☐ Nausea or vomiting | ☐ Difficulty swallowing |
| ☐ Poor appetite / weight loss | ☐ Swelling or wounds |

Daily life snapshot

AREA	BETTER / SAME / WORSE	NOTES
Walking / bathing / dressing		
Eating & sleeping		
Breathing		
Pain control		
Mood & memory		
Caregiver stress		

What matters most? (in the person's own words, if they can answer)

What matters most to you right now? _____

What are you hoping for? _____

What are you most afraid of? _____

What do you want to avoid if possible? _____

Who do you want involved in decisions? _____

■ WORKSHEET — HOSPICE QUESTIONS & DECISION

Use this when hospice has been mentioned or a serious illness is progressing, to understand whether hospice may be appropriate, what to ask, and who will provide care.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Main diagnosis or illness _____

Current location (home, AL, memory care, SNF, hospital) _____

Why are we considering hospice? (check all that apply)

- | | |
|---|--|
| ☐ Serious illness is progressing | ☐ Wounds not healing; difficulty swallowing |
| ☐ More hospitalizations or ER visits | ☐ Treatment is no longer helping or is too hard on the person |
| ☐ More weakness / time in bed / sleeping | ☐ Doctors or palliative team have mentioned comfort care |
| ☐ Less eating or drinking; weight loss | ☐ The person wants comfort-focused care |
| ☐ More falls or infections | |
| ☐ More confusion or shortness of breath | |

Decline snapshot (use specific examples — patterns matter)

AREA	WHAT HAS CHANGED?	WHEN DID IT START?
Eating & drinking		
Weight		
Walking & daily activities		
Sleeping & breathing		
Pain & confusion		
Falls & infections		
Hospital / ER visits		

What matters most now? (in the person's own words, if they can answer)

What matters most to you right now? _____

What are you hoping for? _____

What are you afraid of? _____

Where do you feel safest? _____

What do you want to avoid if possible? _____

Who do you want involved in decisions? _____

Who will provide hands-on care?

Hospice supports the family but does not usually provide a full-time bedside caregiver. Plan who covers each need.

CARE NEED	WHO WILL HELP?	PHONE
Daytime caregiver		
Overnight caregiver		
Medication support		
Bathing & toileting		
After-hours call person		
Backup caregiver		

Documents to have ready

- | | |
|--|--|
| ☐ Insurance cards | ☐ DNR / MOST / POLST (if applicable) |
| ☐ Medication list & doctor list | ☐ Recent hospital or rehab papers |
| ☐ Healthcare Power of Attorney | ☐ Emergency contacts |
| ☐ Living Will / Advance Directive | ☐ Funeral home information (if known) |

■ WORKSHEET — MEDICARE COVERAGE QUESTIONS

Use this to understand what Medicare may cover, what it probably will not, and what to ask before a care decision.

Person receiving care _____

Main caregiver / phone _____

Coverage type (Original Medicare / Advantage / unsure) _____

Plan name & phone _____

Part D drug plan / Medigap (if any) _____

Current care need (check all that apply)

- | | |
|---|---|
| ☐ Hospital | ☐ Doctor visit or medication |
| ☐ Rehab / skilled nursing facility | ☐ Assisted living or memory care |
| ☐ Home health | ☐ Private duty care |
| ☐ Hospice | ☐ Long-term nursing home care |
| ☐ Durable medical equipment | |

Skilled or custodial?

Is this care skilled, custodial, or both? Skilled ☐ Custodial ☐ Both ☐ Unsure ☐ — record who answered and when.

Coverage questions (record answers)

QUESTION	ANSWER	DATE
What part of Medicare applies?		
Is prior authorization required?		
Is the provider in network?		
What requirements must be met?		
What will Medicare / the plan pay?		
What will we owe (deductible/copay)?		
How long might coverage last?		
What could end coverage; can we appeal?		

Hospital status question (ask early, more than once)

Is my parent admitted as an inpatient, or under observation? Inpatient ☐ Observation / outpatient
☐ Unsure ☐ — record who answered and when.

Care Medicare usually does not pay for long-term (plan these separately)

- | | |
|--|--|
| ☐ Assisted living or memory care room and board | ☐ Daily supervision, meals, or housekeeping |
| ☐ Long-term custodial nursing home care | ☐ Companionship |
| ☐ Long blocks of private duty care | ☐ Ongoing bathing help as the only need |

How might this be paid instead?

- | | |
|--|--|
| ☐ Private pay | ☐ Family contribution |
| ☐ Long-term care insurance | ☐ State/local program |
| ☐ Medicaid (if eligible) | ☐ Unsure |
| ☐ VA benefits (if eligible) | |

■ WORKSHEET — MEDICAID PLANNING

This does not replace legal, financial, or Medicaid advice. It helps the family gather information and ask better questions.

Person receiving care _____

Main caregiver / phone _____

State of residence / county _____

Current location (home, hospital, SNF, AL, memory care) _____

What are we trying to figure out? (check all that apply)

- ☐ Will Medicaid help pay for nursing facility care?
- ☐ Will Medicaid help pay for care at home?
- ☐ Is there a Medicaid waiver?
- ☐ Will Medicaid help in assisted living or memory care?
- ☐ What happens when private funds run out?
- ☐ How does the spouse at home stay protected?
- ☐ Does estate recovery apply?
- ☐ How do we apply?

Financial snapshot (for gathering information, not deciding alone)

INCOME / ASSET	SOURCE / INSTITUTION	APPROX. VALUE
Social Security / pension		
Checking / savings		
Investment / retirement accounts		
Life insurance		
Home / real estate		
Vehicle		
Burial / funeral policy		

Documents to gather (check when found)

- ☐ Social Security & Medicare cards
- ☐ Social Security award letter
- ☐ Health insurance cards
- ☐ Pension & bank statements
- ☐ Birth & marriage certificates
- ☐ Investment & retirement statements

- | | |
|---|---|
| ☐ Life insurance policies | ☐ Trust documents |
| ☐ Property deed & mortgage | ☐ Healthcare & Financial Power of Attorney |
| ☐ Vehicle title | ☐ Level-of-care assessment |
| ☐ Funeral / burial policy | ☐ Records of any gifts or transfers |

Transfers & gifts (do not move money or property without advice)

Have any gifts or transfers been made recently? Yes ☐ No ☐ Unsure ☐ — if yes or unsure, list date, what was transferred, to whom, and value, and ask a professional whether it creates a penalty.

■ WORKSHEET — PRIVATE PAY PLANNING

For planning only. It does not replace financial, legal, tax, Medicaid, or benefits advice.

Person receiving care _____

Main caregiver / phone _____

Financial point person / phone _____

Financial Power of Attorney (if known) _____

Current care setting _____

Monthly income

SOURCE	MONTHLY AMOUNT	NOTES
Social Security		
Pension		
Retirement distribution		
Rental / investment income		
Other income		
Total monthly income		

Monthly care costs

COST	MONTHLY AMOUNT	NOTES
Private duty care		
Assisted living / memory care base rate		
Care level & medication fees		
Supplies & incontinence		
Transportation & adult day care		
Housing, utilities, maintenance		
Insurance premiums & copays		
Equipment & care manager		

Total monthly care costs		
--------------------------	--	--

Monthly gap & how long the plan lasts

Total monthly care costs _____

Total monthly income _____

Monthly gap (costs minus income) _____

Funds available for care _____

Estimated months this plan can last _____

Date to review the plan again _____

Available funds for care

ASSET / ACCOUNT	APPROX. VALUE	ACCESSIBLE?
Checking / savings		
Investment account		
Retirement account		
Home equity		
Life insurance cash value		

Other payment sources to explore (check all that may apply)

- | | |
|--|--|
| ☐ Long-term care insurance | ☐ Home sale |
| ☐ VA benefits | ☐ Reverse mortgage (with professional guidance) |
| ☐ Medicaid planning / waiver / HCBS | ☐ Life insurance options |
| ☐ Family contributions | ☐ State, local, or community support |

Family contribution plan (write it down clearly)

NAME	AMOUNT	FREQUENCY	GIFT OR LOAN?

Who pays the care bill? _____

Who tracks contributions? _____

What happens if someone cannot continue? _____

■ WORKSHEET — LONG-TERM CARE INSURANCE POLICY FINDER

Use this to find out whether a policy exists and what to do next.

Person receiving care _____

Main caregiver / phone _____

Person checking for policy / phone _____

Step 1: Is there a policy?

Policy found? Yes ☐ No ☐ Unsure ☐ — if no, stop here and move to the next payment option. If yes or unsure, continue.

Step 2: Where did we look? (check all that apply)

- | | |
|---|---|
| ☐ Important documents folder | ☐ Financial advisor files |
| ☐ Filing cabinet / safe / desk drawers | ☐ Tax files |
| ☐ Bank statements | ☐ Mail pile |
| ☐ Email | ☐ Employer or retirement benefit documents |
| ☐ Insurance folder / binder | ☐ Life insurance documents |

Step 3: Policy information

Insurance company _____

Policy number _____

Company phone _____

Agent / advisor / phone _____

Policy type: Traditional long-term care ☐ Life insurance with LTC rider ☐ Hybrid ☐ Unsure ☐

Step 4: What does it cover?

QUESTION	YES / NO / UNSURE	NOTES
Home care covered?		
Assisted living covered?		
Memory care covered?		
Nursing facility covered?		
Adult day care covered?		
Family caregiver covered?		

Licensed agency required?		
---------------------------	--	--

Step 5: Benefit details

Daily / monthly benefit amount _____

Total benefit pool _____

Elimination period & how it is counted _____

Inflation protection? (yes / no / unsure) _____

Reimbursement or cash benefit? _____

■ WORKSHEET — PRESCRIPTION ASSISTANCE

Use this when medication costs are high. It does not replace medical advice — do not stop, skip, or change medications without guidance from the prescriber or pharmacist.

Person receiving care _____

Medication point person / phone _____

Primary pharmacy / phone _____

Primary doctor / prescriber / phone _____

Current drug coverage

Coverage type: Medicare Part D ☐ Medicare Advantage w/ drug coverage ☐ Employer/retiree ☐
Medicaid ☐ VA ☐ None ☐ Unsure ☐

Plan name & phone _____

Preferred pharmacy (if known) _____

Mail-order option? (yes / no / unsure) _____

Medication cost list

MEDICATION	DOSE	PRESCRIBER	PHARMACY \$	INSURANCE \$

Three most expensive medications _____

■ WORKSHEET — VA BENEFITS SCREENING

Use this when the person may be a Veteran, surviving spouse, or dependent. It does not determine eligibility — it helps the family gather information and ask the right questions.

Person receiving care _____

Main caregiver / phone _____

VA benefits point person / phone _____

Service information

Did the person serve in the U.S. military? Yes ☐ No ☐ Unsure ☐

Branch & dates of service _____

Wartime service? (yes / no / unsure) _____

Discharge status _____

DD214 found? (yes / searching) — location _____

Current VA connection (check all that apply)

- | | |
|---|--|
| ☐ Enrolled in VA health care | ☐ Receives Aid and Attendance or Housebound |
| ☐ Has VA primary care / uses VA pharmacy | ☐ Surviving spouse receives a VA benefit |
| ☐ Has a VA disability rating | ☐ Has a VA social worker |
| ☐ Receives VA compensation or pension | ☐ Unsure |

Surviving spouse (complete if applicable)

Veteran name & date of death _____

Marriage certificate found? (yes / searching) _____

Death certificate found? (yes / searching) _____

Survivor benefits already received? (yes / no / unsure) _____

Possible VA benefits to explore (check what to explore)

- | | |
|--|--|
| ☐ VA health care enrollment | ☐ Long-term / home & community-based services |
| ☐ VA Pension / Survivors Pension | ☐ State Veterans home |
| ☐ Aid and Attendance | ☐ Caregiver support program |
| ☐ Housebound benefits | ☐ Respite |
| ☐ Service-connected disability compensation | ☐ Burial benefits |

Documents to gather (check when found)

- | | |
|--|---|
| ☐ DD214 / separation papers | ☐ Social Security number & VA claim number |
|--|---|

- ☐ VA health care & Medicare cards
- ☐ Marriage / death certificate (if survivor)
- ☐ Medical records & doctor statement
- ☐ VA Form 21-2680 (if A&A/Housebound)
- ☐ Income, asset & bank statements
- ☐ Care invoices & medical expense receipts
- ☐ Power of attorney documents

■ WORKSHEET — IMPORTANT DOCUMENTS FOLDER CHECKLIST

The goal is simple: the right person can find the right document quickly.

Person receiving care _____

Date started / last updated _____

Document point person / phone _____

Backup document person / phone _____

Folder location & who has access

Paper folder location _____

Digital folder location _____

Password manager / access instructions _____

Who knows where the folder is _____

Emergency & medical contacts

ROLE	NAME	PHONE
Emergency contact		
Primary doctor		
Specialist		
Pharmacy		
Home health / hospice		
Preferred hospital		

Insurance & benefit cards (check when copied into the folder)

- | | |
|--|---|
| ☐ Medicare / Advantage / supplement | ☐ VA card |
| ☐ Part D prescription plan | ☐ Private / dental / vision insurance |
| ☐ Medicaid | ☐ Long-term care insurance information |

Healthcare decision documents

DOCUMENT	EXISTS?	COPY IN FOLDER?
Healthcare Power of Attorney		
Living Will / Advance Directive		

DNR / MOST / POLST		
HIPAA release		

Healthcare decision maker / phone _____

Alternate decision maker / phone _____

Financial & legal documents

DOCUMENT	EXISTS?	COPY IN FOLDER?
Financial Power of Attorney		
Will		
Trust		
Property deed / vehicle title		
Bank contact list / safe deposit info		

Attorney / phone _____

Financial advisor / phone _____

Accountant / phone _____

Care, benefits & military documents (check what applies)

- | | |
|---|--|
| ☐ Hospital / rehab discharge papers | ☐ Long-term care & life insurance policies |
| ☐ Home health or hospice paperwork | ☐ Pension / Social Security / Medicaid info |
| ☐ Facility or private duty agreement | ☐ DD214 / VA claim number / VA paperwork |
| ☐ Care plan / fall log | |

End-of-life & funeral information (a kindness to the family)

Funeral home preference / prepaid plan _____

Burial or cremation wishes _____

Cemetery information _____

Clergy / church contact _____

Important people to notify _____

Passwords & accounts

Do not write passwords in an unsafe place — use a secure password manager or trusted access plan.

Password manager used? (yes / no / unsure) _____

Who has access instructions? _____

Missing documents

MISSING DOCUMENT	WHO WILL FIND IT?	DUE DATE

Review schedule

Review this folder: Every 3 months ☐ After a hospitalization ☐ After medication or doctor changes ☐ After insurance changes ☐ After a move ☐ After legal documents are updated ☐

Next review date _____

■ WORKSHEET — HEALTHCARE POWER OF ATTORNEY

This does not replace legal advice. State rules vary — ask an attorney if the document is missing, outdated, unclear, or if capacity is a concern.

Person receiving care _____

Main caregiver / phone _____

Document point person / phone _____

Document status

Does a Healthcare Power of Attorney exist? Yes ☐ No ☐ Unsure ☐

Date signed & state _____

Attorney/office that prepared it / phone _____

Original document location _____

Copy in documents folder? (yes / needs to be added) _____

Named agents

ROLE	NAME	RELATIONSHIP & PHONE	HAS COPY?
Primary agent			
Backup agent			
Second backup (if any)			

Do the named people know they are named? Yes ☐ No ☐ Unsure ☐

Copies shared with (check all that apply)

- | | |
|--|---|
| ☐ Healthcare agent & backup | ☐ Home health / hospice |
| ☐ Primary doctor / specialist | ☐ Palliative care team |
| ☐ Hospital system | ☐ Attorney |
| ☐ Assisted living / memory care | ☐ Important documents folder |
| ☐ Skilled nursing facility | |

Capacity concern

Concern about the person's ability to understand or sign documents? Yes ☐ No ☐ Unsure ☐ —
if yes or unsure, ask the doctor and attorney what happens next.

■ WORKSHEET — LIVING WILL / ADVANCE DIRECTIVE

This does not replace legal or medical advice. State rules vary — ask an attorney, doctor, or legal resource if the document is missing, outdated, unclear, or if capacity is a concern.

Person receiving care _____

Main caregiver / phone _____

Healthcare agent / phone _____

Document status

Does a Living Will / Advance Directive exist? Yes ☐ No ☐ Unsure ☐

Date signed & state _____

Attorney/office that prepared it / phone _____

Original document location _____

Copy in documents folder? (yes / needs to be added) _____

What does the document address? (check all that apply)

- | | |
|---|---|
| ☐ CPR | ☐ Hospital transfer / intensive care |
| ☐ Ventilator / breathing machine | ☐ Comfort care |
| ☐ Feeding tube | ☐ Hospice / palliative care |
| ☐ Artificial hydration | ☐ Organ donation |
| ☐ Dialysis | ☐ Spiritual wishes |
| ☐ Surgery / antibiotics | ☐ Preferred place of care |

The Wishes Conversation (ask while the person can still answer)

What matters most if you become seriously ill, and what would make life still feel meaningful?

What treatments would you want doctors to try — and what would you not want?

Would you want ICU care or a ventilator if recovery seemed unlikely?

Would you want artificial nutrition or hydration? _____

Would you want hospice or comfort-focused care if the illness progressed?

Where would you prefer to receive care, and who should be involved?

What do you want your family to remember if decisions get hard?

■ WORKSHEET — FINANCIAL POWER OF ATTORNEY

This does not replace legal, tax, Medicaid, or financial advice. State rules vary — ask a qualified professional if the document is missing, outdated, unclear, or if capacity is a concern.

Person receiving care _____

Main caregiver / phone _____

Document point person / phone _____

Document status

Does a Financial Power of Attorney exist? Yes ☐ No ☐ Unsure ☐

Date signed & state _____

Attorney/office that prepared it / phone _____

Original document location _____

Is it durable? Yes ☐ No ☐ Unsure ☐

When does authority begin? Immediately ☐ After incapacity ☐ Unsure ☐

Named agents

ROLE	NAME	RELATIONSHIP & PHONE	HAS COPY?
Primary agent			
Backup agent			

Institutions to contact (will they accept the document?)

INSTITUTION	CONTACT / PHONE	POA ACCEPTED? (Y/N/PENDING)
Bank		
Investment / retirement firm		
Insurance / long-term care insurance		
Mortgage / landlord / utilities		
Facility business office		

Financial responsibilities needing attention (check all that apply)

- | | |
|--|--|
| ☐ Bills need to be paid | ☐ Bank accounts need monitoring |
| ☐ Mail needs to be reviewed | ☐ Care bills or facility contract |

- ☐ Long-term care insurance claim to file
- ☐ Medicaid or VA application may be needed
- ☐ Taxes need filing
- ☐ Home maintenance or possible sale
- ☐ Subscriptions to cancel
- ☐ Fraud or scam risk

Monthly bills & care costs

BILL / EXPENSE	AMOUNT	DUE	PAID FROM
Mortgage / rent / utilities			
Insurance			
Assisted living / memory care			
Private duty care			
Medical bills & medications			

Record-keeping plan

Who will keep records? _____

Where will records be kept? _____

Who receives updates, and how often? _____

■ WORKSHEET — THINKING ABOUT YOUR PREFERENCES

This worksheet is not a DNR, MOST, or POLST, and it has no legal or medical effect. Only an order signed by a healthcare provider on your state's official form directs medical care. Use this page to think and talk — then bring it to that conversation. It is written in your parent's voice; hand it to them if they can complete it, or work through it together.

1. If my heart stops, I would want:

- ☐ CPR ☐ No CPR (DNR)

2. If I become seriously ill, I would prefer:

- ☐ Full treatment (all possible medical care)
☐ Limited treatment (some interventions, but not all)
☐ Comfort care only

3. I would want to be cared for:

- ☐ At home ☐ With hospice support, wherever I am
☐ In a hospital

4. I have discussed my wishes with:

- ☐ Family members ☐ Legal decision-maker
☐ Healthcare provider

5. Additional notes about my preferences:

For the family (check when done)

- ☐ Asked the provider whether a MOST/POLST/DNR is appropriate now
☐ Order signed on the state's official form
☐ Original visible with the person (refrigerator / front of chart / binder front)
☐ Copies in the documents folder and with the decision maker
☐ Family, caregivers, and facility know it exists and where it is
☐ Review date set (and at every care transition)

■ **WORKSHEET — ADVANCE CARE PLANNING CONVERSATION**

Use this to guide conversations about future care, values, wishes, decision makers, and documents. It does not replace legal or medical advice — it helps the family talk before a crisis.

Person receiving care _____

Date of conversation _____

Main caregiver / phone _____

Healthcare decision maker / agent / phone _____

Backup decision maker / phone _____

What matters most? (check any priorities mentioned)

- ☐ Staying at home
- ☐ Avoiding repeated hospital trips
- ☐ Being comfortable / avoiding pain
- ☐ Being with family
- ☐ Living as long as possible
- ☐ Faith / prayer / spiritual support
- ☐ Being able to communicate
- ☐ Maintaining independence
- ☐ Recognizing family
- ☐ Avoiding machines
- ☐ Avoiding burden on family

In the person's own words _____

What are you hoping for? _____

What are you worried about? _____

What would make life still feel meaningful — and what would feel like too much?

Care preferences

CARE QUESTION	WISHES / NOTES
Hospital transfer / ICU	
CPR	
Ventilator / breathing machine	
Feeding tube / artificial hydration	
Dialysis / surgery / antibiotics	
Palliative care / hospice	

Comfort-focused care	
----------------------	--

Preferred place of care (if illness worsens)

- | | |
|--|---|
| ☐ Home | ☐ Skilled nursing facility |
| ☐ Assisted living | ☐ Hospice facility |
| ☐ Memory care | ☐ Unsure |
| ☐ Hospital | |

Faith, spiritual & personal wishes

Faith or spiritual support desired? (yes / no / unsure) _____

Clergy / spiritual contact & phone _____

Music, prayer, rituals, readings, or personal comforts _____

Important people to include _____

Decision maker & documents

Primary decision maker / phone _____

Backup decision maker / phone _____

Does the named person know? Yes ☐ No ☐ Needs conversation ☐

Matches the Healthcare POA? Yes ☐ No ☐ Unsure ☐ No document found ☐

Documents that exist (check)

- | | |
|--|--|
| ☐ Healthcare Power of Attorney | ☐ Financial Power of Attorney |
| ☐ Living Will / Advance Directive | ☐ HIPAA release |
| ☐ DNR / MOST / POLST | ☐ Important documents folder |

■ WORKSHEET — MEDICATION AND DOCTOR LIST

Create one clear list for appointments, emergencies, hospital visits, home health, hospice, assisted living, memory care, or rehab.

Person receiving care _____

Date last updated _____

Medication-list person / phone _____

Primary pharmacy / phone _____

Allergies & medication reactions _____

Current medication list

MEDICATION	DOSE	HOW OFTEN / TIME	REASON	PRESCRIBER

As-needed medications

MEDICATION	DOSE	WHEN USED	MAX ALLOWED

Over-the-counter, vitamins & supplements

ITEM	DOSE	HOW OFTEN	REASON

Recently stopped medications

MEDICATION	DATE STOPPED	WHO STOPPED IT?	REASON

Medication concerns (check any that apply)

- | | |
|---|--|
| ☐ Missed or extra doses | ☐ Dizziness, falls, or sleepiness |
| ☐ Confusing instructions | ☐ New confusion or poor appetite |
| ☐ Duplicate medications | ☐ Nausea or constipation |
| ☐ Old bottles still in use | ☐ Trouble swallowing pills |
| ☐ Multiple pharmacies or prescribers | ☐ Refusing medications |
| ☐ High cost | |

Doctor & care team list

NAME	ROLE / SPECIALTY	PHONE	NEXT VISIT
Primary doctor			
Specialist			
Specialist			
Pharmacy			
Home health / hospice			
Facility nurse			

Medication safety plan

Who fills the pill box, and how often? _____

Who checks for missed doses? _____

Where are medications stored? _____

Are old medications removed from the home? (yes / no / needs attention)

Who updates the list after changes? _____

■ WORKSHEET — EMERGENCY CONTACTS & CRISIS PLAN

Create one clear emergency page. The goal: the right person can find the right number fast.

Person receiving care / date of birth _____

Address _____

Main diagnosis / conditions _____

Allergies _____

Medication list location _____

Preferred hospital _____

Important documents folder location _____

Family point people

ROLE	NAME	PHONE
Main family point person		
Backup point person		
Healthcare agent		
Main caregiver		
Backup caregiver		

Medical contacts

CONTACT	NAME / ORGANIZATION	PHONE	AFTER-HOURS
Primary doctor			
Specialist			
Pharmacy			
Home health			
Hospice			
Facility nurse			

Call 911 for (check to confirm the caregiver knows)

☐ Chest pain

☐ Severe trouble breathing

- ☐ Stroke signs — sudden face drooping, arm weakness, or speech trouble (note the time)
- ☐ Unconsciousness
- ☐ Uncontrolled bleeding
- ☐ Serious fall or major injury
- ☐ Severe allergic reaction
- ☐ Fire or unsafe environment
- ☐ Immediate danger to self or others

Hospice / home health

On hospice? Yes ☐ No ☐ Receiving home health? Yes ☐ No ☐

Hospice agency / main & after-hours number _____

Hospice nurse _____

Home health agency / number _____

Instructions from the team _____

Documents & medical orders (check what applies)

- ☐ Healthcare POA
- ☐ Living Will / Advance Directive
- ☐ DNR / MOST / POLST
- ☐ Medication & allergy list
- ☐ Insurance / Medicare / Medicaid / VA cards
- ☐ Long-term care insurance

Family call chain

ORDER	WHO GETS CALLED?	PHONE	WHO CALLS THEM?
1			
2			
3			

Before you call — what changed?

Awake & responsive? Yes ☐ No ☐ Unsure ☐ Fall? Y ☐ N ☐ Breathing trouble? Y ☐ N ☐

Chest pain? Y ☐ N ☐ New confusion? Y ☐ N ☐ Fever? Y ☐ N ☐ New pain? Y ☐ N ☐
 Medication change? Y ☐ N ☐

■ WORKSHEET — LAST DAYS PLAN

Use this when a serious illness is progressing or hospice is involved. It does not replace medical advice — call the care team with any concern, and 911 for immediate danger if emergency treatment is wanted or needed.

Person receiving care / current location _____

Main caregiver / phone _____

Family point person / phone _____

Healthcare decision maker / phone _____

Current care team

ROLE	NAME / AGENCY	PHONE	AFTER-HRS
Primary doctor			
Hospice agency / nurse			
Facility nurse			
Pharmacy			
Funeral home			
Clergy / spiritual			

Who do we call first? (check the plan)

If the person is on hospice with comfort-focused goals, hospice is usually the first call — day or night.

- | | |
|--|---|
| ☐ 911 for immediate danger or emergency treatment | ☐ Facility nurse |
| ☐ Hospice | ☐ Primary doctor |
| | ☐ Palliative care team |

First call number _____

Backup call number _____

Documents confirmed (check what exists, note location)

- | | |
|--|---|
| ☐ Healthcare POA | ☐ Medication list |
| ☐ Living Will / Advance Directive | ☐ Insurance cards |
| ☐ DNR / MOST / POLST | ☐ Funeral home information |

Comfort priorities (check what matters most now)

- | | |
|--|--|
| ☐ Pain relief | ☐ Prayer / spiritual support |
| ☐ Breathing comfort | ☐ Music |
| ☐ Anxiety / restlessness relief | ☐ Familiar blanket or photos |
| ☐ Quiet room | ☐ Avoiding hospital transfer if possible |
| ☐ Family nearby | ☐ Staying in current location if possible |

Symptoms to watch (ask the care team what to do)

SYMPTOM	WHAT WE WERE TOLD TO DO	WHO TO CALL
Pain		
Shortness of breath		
Anxiety / restlessness		
Breathing changes		
Trouble swallowing		
Not eating / drinking		

Comfort medications (use only as directed by the care team)

MEDICATION	WHAT IT IS FOR	DOSE / INSTRUCTIONS	WHEN TO CALL

Who taught the family how to use these, and when? _____

People to notify if time becomes short

NAME	RELATIONSHIP	PHONE	WHEN?

What to say — words the family may want to share

- | | |
|--------------------|----------------------------------|
| I love you. | We are here. |
| Thank you. | You are not alone. |
| I forgive you. | We will take care of each other. |
| Please forgive me. | You can rest. |

If death occurs (ask the care team ahead of time)

Who do we call first? / phone _____

Who pronounces death? _____

Funeral home & who calls them _____

Who updates family? _____

What happens with medications & equipment? _____

Special faith, cultural, or family wishes _____

Family role assignments

ROLE	PERSON RESPONSIBLE	PHONE
Main bedside contact		
Family updates		
Calls hospice / nurse		
Handles documents		
Calls funeral home		
Supports spouse / caregiver		

■ WORKSHEET — AFTER-DEATH PLAN

Fill in as much as you can ahead of time. This does not replace guidance from hospice, the facility, or the funeral home. In an unexpected death with no plan, call 911.

Person receiving care _____

Current location _____

Family point person / phone _____

Who to call first

First call: Hospice ☐ Facility staff ☐ Doctor (expected home death) ☐ 911 (unexpected / no plan) ☐

CONTACT	NAME / AGENCY	PHONE (INCL. AFTER-HOURS)
Hospice		
Doctor who will sign		
Facility staff		
Funeral home		
Clergy / spiritual support		

Documents ready (check what is visible / located)

- | | |
|--|--|
| ☐ DNR / POLST visible for responders | ☐ Funeral or burial wishes / prepaid plan |
| ☐ Healthcare POA & advance directive | ☐ Important documents folder |
| ☐ Insurance & Social Security information | |

Time in the room — what would bring comfort? (check any)

- | | |
|---|--|
| ☐ Sitting together | ☐ Gathering family to say goodbye |
| ☐ Holding a hand | ☐ Bathing or dressing the person |
| ☐ Prayer / scripture | ☐ A quiet, private room |
| ☐ Music | |

Faith, cultural & personal wishes at the time of death

Clergy or spiritual contact to call _____

Rituals, prayers, or readings to honor _____

Who should be present _____

Anything to avoid _____

People to notify (after the first hours)

NAME	RELATIONSHIP	PHONE	WHO CALLS?

■ First Days — Practical Checklist (none of this is urgent)

- ☐ Order several copies of the death certificate
- ☐ Meet with the funeral home to make arrangements
- ☐ Notify Social Security
- ☐ Contact pension, VA, and life insurance
- ☐ Notify banks; change or close accounts
- ☐ Cancel appointments, subscriptions, and services
- ☐ Notify employer or organizations, if applicable
- ☐ Return or arrange pickup of medical equipment
- ☐ Dispose of medications safely (hospice can help)
- ☐ Begin sorting belongings when ready

Point person leading these tasks / phone _____

Next step, and when _____

Numbers That Matter

Photograph these two pages, or tape a copy to the refrigerator. Phone numbers change — confirm before relying on them, and write your local numbers in the blanks. A printable copy of this page lives at **thecardinalspromise.com**.

WHEN	WHO	NUMBER
Immediate danger or life-threatening emergency	Emergency services	911
Mental health crisis — yours or theirs, any hour	Suicide & Crisis Lifeline	988 (call or text)
Finding local aging services anywhere in the U.S.	Eldercare Locator	1-800-677-1116
Dementia questions or a dementia crisis, 24/7	Alzheimer's Association Helpline	1-800-272-3900
Medicare questions	Medicare	1-800-MEDICARE (1-800-633-4227)
Free, unbiased Medicare counseling	State Health Insurance Assistance Program (SHIP)	1-877-839-2675 · shiphelp.org
Fast appeal when a hospital, SNF, or home health stay is being cut short	Your Medicare QIO (Quality Improvement Organization)	qioprogram.org ("Locate Your QIO") or 1-800-MEDICARE
Caring for a Veteran	VA Caregiver Support Line	1-855-260-3274
Suspected abuse, neglect, or financial exploitation	Adult Protective Services (statewide hotline in most states)	napsa-now.org or Eldercare Locator 1-800-677-1116

YOUR LOCAL NUMBERS	WRITE THEM HERE
Primary doctor	
Pharmacy	
County / state social services office (Medicaid, APS)	
Area Agency on Aging	

Hospice / home health agency	
Facility nurse / after-hours line	
Family point person	

IT'S NOT YOUR FAULT

"Take the next right step."

Rob Brizzi, Certified Dementia Practitioner
Cardinal's Promise Series · thecardinalspromise.com